

THE CARE COLLECTION • BOOK 10

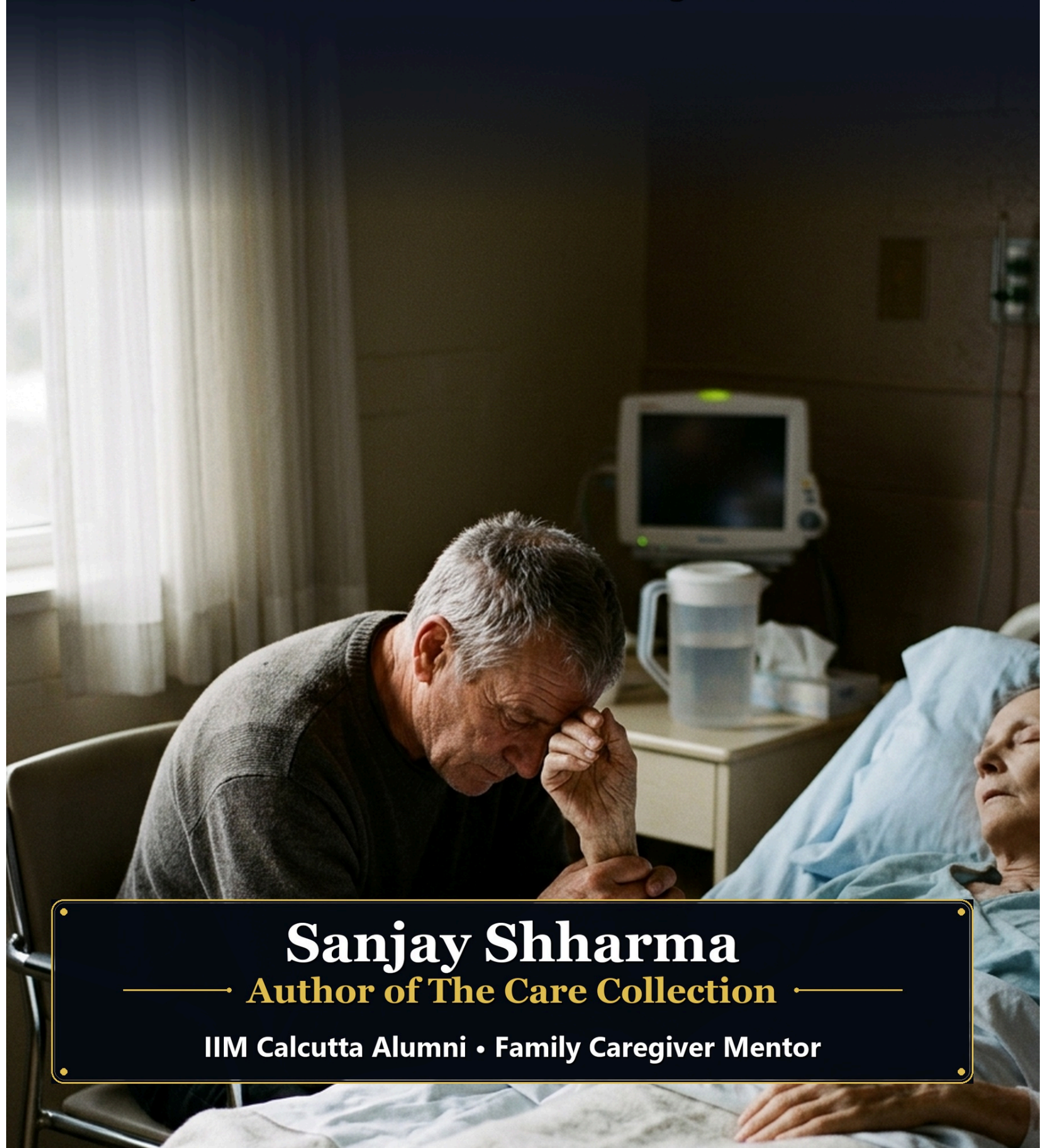


The Long Goodbye

When Your Spouse Is Dying and the Marriage Is Still Yours

Loving Your Spouse When Tomorrow Is Not Promised

A 4-Phase Framework for the Walking-Beside Years



Sanjay Shharma

— Author of The Care Collection —

IIM Calcutta Alumni • Family Caregiver Mentor

Table of Contents

The Long Goodbye

- Introduction: The Marriage Nobody Plans For
- Part I: The Day They Tell You
- Chapter 2: The First 48 Hours
- Chapter 3: What the Doctors Won't Tell You
- Chapter 4: Telling the Children (and Grandchildren, and Friends)
- Chapter 5: The New Normal
- Chapter 6: When Intimacy Changes
- Chapter 7: The Marriage Stress Test
- Chapter 8: Money, Paperwork, and the Conversations You Avoid
- Chapter 9: The Acceptance Conversation
- Chapter 10: Hospice and the Gift of Time
- Chapter 11: Saying What Needs to Be Said
- Chapter 12: The Last Good Day
- Chapter 13: The Final Days
- Chapter 14: The Last Breath
- Epilogue: The After

Published by Adorise Digital

adorisedigital.com

Introduction: The Marriage Nobody Plans For

I have not been where you are.

But I have been close. I have sat across the table from someone who has. I have held her hand while she tried to explain. I have watched her look at her own hands the way people look at hands that have just been told to stop doing what they have always done.

I have also been the one who stayed behind.

This book is for the person who has just heard the words. The ones the doctor says carefully, the way doctors say them when they have said them many times before. "It's terminal." "A few months." "We should think about comfort now." "Have you thought about hospice." "There is no further treatment that would help."

This book is for the person who is about to live inside a different kind of marriage than the one they thought they were in. A marriage with a clock in the room. A marriage where the small things suddenly become unbearable, and the unbearable things become ordinary. A marriage where the two of you are closer than you have ever been and farther than you can stand, sometimes in the same hour.

I want to be careful with this book. I want to be useful to you without being sentimental, honest without being grim. I want to be the friend who has seen a few of these marriages through, who knows the things that matter and the things that don't, who can tell you what people wish they had done and what they wish

they hadn't.

I have written this book because I have learned, slowly and against my will, that the end of a marriage is not a single event. It is a long season. It is a thousand small moments that you will not remember individually but that you will carry as a whole for the rest of your life. The way you handle this season will become part of who you are. The marriage you had at the end — the one you had when you knew it was ending — is the last marriage you will have. It deserves your attention.

I lost my wife to cancer after many years of marriage. I was her primary caregiver through the long middle. I sat with her in the quiet at the end. I held her hand when the last breath came. I have lived the first hours and the first year and the first decade of the after. I know the road you are about to walk.

I do not have all the answers. Some of the things that happen in the last months of a marriage cannot be answered. They can only be witnessed.

But I have learned a few things. I have collected them from my own marriage, from other marriages I have watched, from the people I have talked to in the years since, and from the research that exists about end of life, palliative care, anticipatory grief, and widowhood.

I want to share what I have learned.

What This Book Is

This book is a practical guide to the final months of a marriage. It is not a religious book. It is not a self-help book. It is a guidebook — the kind of book a friend might write for you if your friend had been through this and you had not.

It has four parts, one for each of the four phases of this season:

Part I: The Day They Tell You — the diagnosis, the first days, the first decisions

Part II: The Long Middle — the weeks and months of treatment, daily life, and marriage under stress

Part III: The Quiet — the acceptance, the final weeks, the saying goodbye

Part IV: The Goodbye — the last days, the last breath, the first days alone

It also has a short Epilogue for the after — the year that follows — and five bonus toolkits you can use at any stage. The toolkits are practical. They have worksheets. You do not have to use them in order. Use what you need when you need it.

What This Book Is Not

This book is not going to tell you how to feel. Grief is not a problem to be solved. Love is not a strategy. Death is not a lesson.

This book is not going to give you the right words. There are no right words. There are only the words that come out of your mouth on the day you need them, and they will be enough.

This book is not going to tell you to be strong, or to be positive, or to make every moment count. Some moments you will not want to count. Some moments you will be too tired to count. Some moments you will be too angry to count. That is normal. That is allowed.

A Note on the Word "Spouse"

I have used "spouse" throughout this book, not "husband" or "wife." The person reading this book might be the one who is dying or the one who is staying behind. The marriage at end of life does not have a gender. The work of caring does not have a gender. The grief that follows does not have a gender. The book is for both of you.

I have used "they" for the dying spouse in case studies for the same reason. In a few of the stories, you will see a name. That name is a person. That person is also you, if your spouse is named that. That person is also not you. These are real stories, lightly fictionalized, but the feelings are universal.

A Note on Timing

This book was written for a spouse who has been told there are "a few months." That can be one month. That can be three months. That can be six months. That can be a year. The framework here works for all of those timeframes.

If you have just been told, this book is for you. If you have been in this for a while, this book is for you. If you are reading this in the after, this book is for you too — the parts that don't apply, skip. The parts that do, sit with them.

A Note on Reading

You do not have to read this book in order. You do not have to read it at all if you do not want to. You do not have to finish any chapter you start. You are allowed to put the book down. You are allowed to come back to it. You are allowed to read only the parts that help and leave the rest.

You are in charge.

The marriage is still yours.

A Final Note

I want to say one thing before we begin.

You are not failing.

Whatever you are doing right now — whatever you are not doing — you are not failing. There is no right way to do this. There are people who will tell you there is. They are wrong. The right way to do this is the way that lets you and your spouse still recognize each other at the end.

If you are reading this book, you are trying. That is enough.

Let us begin.

Part I: The Day They Tell You

Chapter 1: The Word That Changes Everything

There is a moment. You will know it when it comes. The doctor says something, or you read something in a chart, or you see a scan, and the world becomes two versions of itself. The version that was true until that moment, and the version that is true after. The line between the two is the moment. It is the moment the marriage you had becomes the marriage you have.

For some of you, this moment is sudden. An accident. A stroke. A heart attack that doesn't end the way the movies do. For most of you, this moment is slow. A scan that has been showing something for a while. A biopsy that came back. A conversation with a doctor that started about one thing and ended about another.

Either way, the moment is the same. The doctor — or the test, or the body itself — has said: there is a clock. The clock has a number. The number is "a few months." Or "a year." Or less. The number is a guess, but it is a guess with gravity.

This chapter is about that moment. About what happens in the hours and days after. About how the marriage changes instantly, and about what the two of you can do in the first 72 hours to set the tone for what comes next.

Chapter at a Glance

- The 5 Things to Do in the First 72 Hours (Scorecard)
- The 3 Things Couples Get Wrong in the First 24 Hours

- Why the Second Conversation Matters More Than the First
- The 5 Things to Say — and 5 Things Not to Say — to Each Other
- The Scorecard: How Are We Doing So Far?
- Key Terms: Terminal, Palliative, Hospice, Prognosis, End-of-Life Care
- Reflection Questions for the Two of You
- Action for This Week: The First Conversation

The 5 Things to Do in the First 72 Hours

When the word lands, the first 72 hours are not a time for big decisions. They are a time for small things that set the tone. The decisions will come. The research will come. The second opinions will come. The next three to six months will come. But in the first 72 hours, the most important things are these:

1. Be together. Not on the phone with family, not in the doctor's office with the specialist, not on the internet looking for the cure. Together. In the same room. The first 24 hours are not for action. They are for being in the same room with each other while the news becomes real.

2. Sleep. Not plan. Not research. Not "have the conversation." Sleep. The news will still be there tomorrow. The clarity will be better with sleep. Many couples report that the night of the diagnosis is the night they cannot sleep, and that the day after is the day they cannot think. The way to push back on this is to take a sleeping pill if you need to. Or to ask the doctor for one. Or to just commit to going to bed even if you do not think you will sleep. The body needs rest.

The mind needs rest.

3. Tell one person. Not everyone. Not yet. One person. The one you trust most. The one who will not panic. The one who will not immediately start looking for cures. The one who will sit with you and not need to fix anything. Tell that person. Let them help you figure out who else to tell and when. Many couples in this position make the mistake of telling everyone in the first 48 hours, and they spend the next two weeks managing other people's grief instead of their own.

4. Write down what the doctor said. Not your feelings about what the doctor said. What the doctor said. The words. The number. The timeline. The recommended next steps. The names of the medications. The names of the specialists. The date of the next appointment. You will not remember this clearly tomorrow. You will remember it less clearly in a week. Write it down. Take a notebook to the appointment. Or use the voice memo on your phone. Or have your spouse write while you listen. The point is that this information is now part of your life and you will need to refer back to it.

5. Have one small ordinary moment. Eat a meal. Take a walk around the block. Watch a show you both like. Hold hands. Make the bed. Do one small thing that is part of your ordinary life. This is not about distraction. This is about reminding yourselves, in the middle of the worst moment, that the marriage is still here. The marriage is the ordinary. The marriage is the meal, the walk, the show, the bed. The illness is something that is happening to the marriage. The marriage is still the marriage.

The 3 Things Couples Get Wrong in the First 24 Hours

In my conversations with people who have lived through this, three patterns come up again and again. The first 24 hours are a vulnerable time. Couples make these mistakes not because they are weak, but because they are in shock. Knowing about them in advance can help you avoid them.

Mistake 1: Trying to be strong for the other person. This is the most common mistake. One spouse tries to hide the grief from the other. They think they are protecting the other person. They are not. They are putting a wall between the two of you at the exact moment when you need to be on the same side of the wall. The diagnosis is happening to both of you. The grief is happening to both of you. Hiding it doesn't protect them — it just means they are alone with theirs. Be weak together. Cry together. Be afraid together. You have time to be strong later. In the first 24 hours, be honest.

Mistake 2: Going to war with the disease immediately. Many spouses, especially the well one, respond to the diagnosis by becoming a general. They will research. They will get second opinions. They will find clinical trials. They will look for the cure. They will fight. The instinct is right — to do something. But the timing is wrong. In the first 24 hours, before the well spouse has fully processed the news, the spouse who is dying often feels like a project, not a person. The fighter is trying to save them. The one being saved feels like they have already been written off. Slow down. There is time. The first 24 hours are for being together, not for waging war.

Mistake 3: Trying to have the big conversation right away. Couples in this position often feel pressure to "have the conversation" — about what to do, about what the dying spouse wants, about how the rest of the time will be spent. That conversation matters. But it does not have to happen in the first 24 hours. In fact, it usually cannot happen in the first 24 hours, because both of you are still in shock. The big conversation will be better on day four or day five, when the news has settled into your bones. In the first 24 hours, you do not have to have the conversation. You only have to be in the same room.

Why the Second Conversation Matters More Than the First

There will be many conversations about what comes next. There will be the conversation about treatment, the conversation about hospice, the conversation about who to tell, the conversation about money, the conversation about the funeral you do not want to plan yet, the conversation about what the dying spouse wants to do with the time they have left.

The first conversation — the one you have in the first day or two — is usually not the most important. It is the one that happens in shock. It is the one where the well spouse is fighting and the dying spouse is processing. It is the one where neither of you really knows what you want yet.

The second conversation, a few days later, is the one that matters. By then, the news has settled. By then, the first wave of grief has passed through. By then, you have had one or two good nights of sleep (or at least rested). By then, you have talked to a few people and have started to hear yourselves think.

The second conversation is the one where you actually decide. Not in a final way. Not in a complete way. But in a real way. The second conversation is when you say: this is what we are going to do next. This is what we want for the time we have. This is how we are going to be with each other.

Plan for the second conversation. Do not try to have it in the first 24 hours. Do not try to have it in the first 72 hours. Have it when you can both sit with each other and know that you are not deciding everything, just deciding the next thing.

The 5 Things to Say — and 5 Things Not to Say — to Each Other

The instinct in the first 24 hours is to say the big things. The things you have been meaning to say. The things you have not said in 30 years of marriage. The things you thought you would have more time to say.

Some of those big things are right to say. Many of them are wrong to say in the first 24 hours, because the first 24 hours are not a time for big things. They are a time for small honest things.

The 5 Things to Say in the First 72 Hours

1. "I am here." Not "I am here for you" — that is for the caregiver, not the spouse. Just "I am here." Three words. They mean: I am not going anywhere. They mean: you are not alone. They mean: we are in this together.

2. "I don't know what to say." This is honest. It is what you are feeling. And it gives the other person permission to also not know what to say. So many couples in this position feel pressure to have the right words. The right words are: "I don't know what to

say." That is the right words.

3. "I love you." Not the big version. The small one. The one you say without thinking. The one that is true. Say it. Even if you have said it a thousand times. Say it again. It is the most important sentence in your marriage right now.

4. "What do you need right now?" Not "what do you need" in the abstract. "What do you need right now" in this hour, this afternoon, this evening. Do you need to be alone? Do you need to be held? Do you need to be distracted? Do you need to talk? Do you need to cry? The question is small. The question is specific. The answer is usually small too.

5. "We will figure this out." Not "I will fix this." Not "We will beat this." Just "we will figure this out." It is honest. You don't know yet what figuring it out will look like. But you will figure it out. You have figured out other things in your marriage. You will figure this out too.

The 5 Things NOT to Say in the First 72 Hours

1. Don't say "we will beat this" or "we will fight this" unless you mean it. Most people say it because they don't know what else to say. The person who is dying often hears it as: I am not allowed to die. I am not allowed to give up. I have to fight. If you say it, mean it. If you don't, say something else.

2. Don't say "I can't live without you." This is the most common thing spouses say in the first 24 hours. It is said with love. It is heard as pressure. The person who is dying now has to manage your grief in addition to their own. Many dying spouses report that the worst part of the first 24 hours was their spouse's grief, not

their own. Save "I can't live without you" for a few days later, when the well spouse has more support.

3. Don't say "you have to be positive" or "you have to fight." The dying spouse gets to feel whatever they feel. They get to be sad. They get to be scared. They get to be angry. They get to give up on a Tuesday and be hopeful on a Wednesday. Your job is not to manage their emotions. Your job is to be with them while they have their emotions.

4. Don't say "we don't have to talk about it right now." This sounds kind. It usually means you cannot handle the conversation. But the dying spouse often wants to talk, and the spouse who says "we don't have to" is taking that away. If you can't handle it, say "I am not ready to talk about it right now, but I want to when I can." That is honest. It opens the door. It does not shut it.

5. Don't say "call me if you need anything" to other people and then be unavailable. The first 72 hours is when other people will offer help. Take the help. Let someone bring dinner. Let someone take the dog. Let someone drive you to the appointment. This is not the time to be polite. This is the time to accept that you cannot do this alone.

The Scorecard: How Are We Doing So Far?

Use this in the first week. Both of you. Take turns answering. Be honest.

| Question | Score (1-5) |

|---|---|

| We are spending time together, not on phones or tasks | /5 |

We are both sleeping at least a few hours	/5
We have told one trusted person	/5
We have written down what the doctor said	/5
We have had one small ordinary moment	/5
We have not tried to have the big conversation in the first 24 hours	/5
We are not pretending to be strong for the other	/5
We have said "I love you" at least once	/5
We are not pressuring each other to feel a certain way	/5
We have accepted one offer of help	/5

Total: /50

Interpreting your score:

- 40-50: You are doing the first 72 hours well. The marriage is intact.
- 30-39: You are doing okay. Some patterns to watch. Use the toolkits.
- 20-29: You are struggling. Reach out to a counselor, a chaplain, a friend, a family member.
- Under 20: You need help. The marriage is at risk. The first 72 hours are not a time to white-knuckle it.

Key Terms

These are the words the doctor will use in the first 72 hours. Knowing them in advance helps you not be overwhelmed by them.

- Terminal: The illness cannot be cured. Treatment is no longer aimed at cure. This is not the same as "immediate." Terminal often means months.

- Palliative: Treatment aimed at comfort, not cure. Pain management, symptom control, quality of life. Palliative care is not the same as end-of-life care — it can start much earlier.
- Hospice: A specific kind of palliative care, usually for the last 6 months of life. Hospice is a philosophy and often a service. It can be at home, in a facility, or in a hospital.
- Prognosis: The doctor's estimate of how the illness will progress. Often given in ranges (3-6 months, 6-12 months). Doctors are usually cautious — they tend to under-estimate.
- End-of-Life Care: The umbrella term for care when someone is dying. Includes palliative, hospice, and the medical and emotional support around it.

Reflection Questions for the Two of You

Take turns. Listen more than you speak. There are no right answers. There are only your answers.

1. What is one thing you are most afraid of right now?
2. What is one thing the other person has done in the last 24 hours that has helped?
3. What is one thing the other person has done in the last 24 hours that has not helped?
4. Is there anyone you both want to be with in the next 48 hours? Anyone you both do not want to be with?
5. Is there a small ordinary thing you want to do together in the next 48 hours? (Walk, meal, show, anything.)

6. What do you each need most right now — to be held, to be alone, to be distracted, to talk, to cry, to sleep?

Action for This Week: The First Conversation

Within the next 7 days, find a time when you are both rested and alone. Not in a doctor's office. Not with family around. Not in bed at night. A Saturday morning, or an afternoon, or a quiet evening. Sit together. Make tea or coffee.

The first conversation has only one purpose. It is not to decide everything. It is to decide the next thing. Use these questions:

- What did the doctor say, in our own words?
- What is the next appointment, and what is it for?
- Who do we want to tell, and when?
- What do we want for the next week?
- What is one thing we want to do together in the next month?
- What do we each need from the other in the next week?

You do not have to answer all of these. You do not have to answer them in order. You do not have to write anything down unless you want to. The point is to be in the same room and start the next step.

The first conversation will be hard. It will be quiet. It will be the first of many. It is not the last conversation. It is the first one that is not the diagnosis itself.

After it, you will both feel a little lighter. Not happy — lighter. Like a small weight has been moved from one shoulder to a table. The weight is not gone. It is just

somewhere you can both see it.

That is enough for the first week.

A Closing Word for This Chapter

The day they tell you is a day that splits your marriage in two. There is the marriage before the words and the marriage after the words. You will be tempted to try to live in the marriage before the words. You will want to pretend the words did not land. The words did land. The marriage after the words is the marriage you have now.

The good news is this: the marriage after the words is not worse than the marriage before. It is different. It is more honest. It is more present. It is the marriage you have been building your whole life without knowing it — the marriage that knows what matters.

In the chapters that follow, we will walk through the rest of this season. The long middle. The quiet. The goodbye. The after. You do not have to read them now. You can come back when you are ready. Or you can read them in the middle of the night, when sleep is not coming. The book will be here.

For now, the first 72 hours. Be together. Sleep. Tell one person. Write it down. Have one small ordinary moment.

You are not failing. You are beginning.

Chapter 2: The First 48 Hours

The first 24 hours are about being together. The next 24 are about beginning. The first 48 hours together are the time when the news moves from being a thing that happened to you to being a thing you are doing something about. The diagnosis is in your body. Now you have to put it in your life.

This chapter is about the practical work of the first 48 hours. Not the emotional work — that will continue for months. The practical work. The phone calls. The questions to ask. The decisions to make and the decisions to defer. The people to tell and the people to wait on. The small acts of administration that, in the first 48 hours, are a way of caring for each other.

If Chapter 1 was about being, Chapter 2 is about doing. They are not in tension. The doing serves the being. The phone calls and the questions are not a way to avoid the feeling. They are a way of taking care of the person you are feeling with.

Chapter at a Glance

- The 6 Decisions to Make in the First 48 Hours (Scorecard)
- Who to Tell First, and When to Tell Them
- The 8 Questions to Ask the Doctor at the Next Appointment
- The Second Appointment: Getting a Second Opinion Without Losing a Week
- The 5 Things to Set Up in the First 48 Hours
- The 3 Things to Defer (You Don't Have to Decide Yet)

- The Scorecard: How Are We Doing on Day 2?
- Key Terms: Second Opinion, Palliative Care, Advance Directive, Healthcare Proxy
- Reflection Questions for the Two of You
- Action for This Week: The Three Lists

The 6 Decisions to Make in the First 48 Hours

The first 48 hours have six practical decisions. None of them are the big decisions. None of them are about treatment or hospice or how the rest of the time will be spent. They are the small decisions that make the next weeks possible.

Decision 1: Who is the medical point person? In most marriages, one spouse has been the one who tracks the appointments, the medications, the test results. The other has trusted that person to handle it. With a terminal diagnosis, this changes. Both of you need to be able to talk to the medical team. Pick one of you to be the primary point person for appointments, but make sure the other has full access to the information. Most medical systems have a patient portal. Get the well spouse on it. Get the login. Know how to find the test results. This is a small act of love that will matter in the long middle.

Decision 2: What is the next appointment, and what is it for? Within 48 hours, you should have clarity on what happens next. Is there a follow-up with the diagnosing doctor? Is there a referral to a specialist? Is there a test that needs to be scheduled? Write down the date, the time, the location, the doctor, and what it is for. Put it on the calendar. If the medical team does not have a follow-up scheduled, call and ask for one. You should not have to wait more than a week after

the diagnosis to have a plan.

Decision 3: What is the one question you need answered first? You will have a hundred questions. You will not be able to get them all answered in one appointment. Pick the one that is keeping you up at night. The one that is making the next day impossible. Write it down. Bring it to the next appointment. The doctor is there to answer your question, not to give you a survey of the disease. Your question. The most important one.

Decision 4: Who is the one trusted person who will be there for the next 72 hours? You told someone in the first 24 hours. Now you need to ask that person for a specific thing. Not "let me know if you need anything." A specific thing. "Can you come over tomorrow afternoon for two hours so I can nap." "Can you take the dog for the weekend." "Can you come with us to the next appointment and take notes." "Can you call me every evening for the next week." The people who love you want to help. They need you to tell them how.

Decision 5: What is the one small ordinary thing you are going to do together in the next 48 hours? You already did this in the first 24. Do it again. Eat a meal. Take a walk. Make the bed. Watch a show. Have tea. The illness is a thing that is happening. The marriage is a thing you are still doing. The ordinary is how you hold onto the marriage. Do one ordinary thing in the next 48 hours, even if it feels impossible.

Decision 6: When are you going to tell the rest of the people who need to be told? You told one person. The rest will come. In the next week, the news will need to reach the children, the parents, the siblings, the close friends, the colleagues. You do not have to tell everyone in the first 48 hours. You do have to decide when you will. Make a list. Decide who tells whom.

Pick a window — "we will tell the kids by Sunday" — and then tell them. The ambiguity of "we will tell people eventually" is worse than the act of telling.

Who to Tell First, and When to Tell Them

There is an order to telling people. The order is not always the order of your relationships. The order is: the people who can hold the news without needing to be held.

Tell first:

- The one person you trust most (already told in first 24 hours)
- The adult children (if you have them)
- The closest friend or sibling who is emotionally stable

Tell second (within the first week):

- Other family members
- Closer friends
- Anyone the dying spouse wants to know

Tell third (within the first two weeks):

- Extended family
- Work colleagues (only as needed for practical reasons)
- Wider friend network

Tell much later (or not at all):

- Acquaintances
- The "well-meaning" friends who will take the news harder than you
- The people who will want to fix it, advise you, or be angry

You are not obligated to tell everyone. You are not obligated to tell anyone except the people who need to know for practical reasons. The dying spouse gets to decide who they want to know and who they don't. The well spouse gets to decide who they want to lean on. The people you don't tell are not being left out. They are being protected from a weight they didn't ask to carry.

The 8 Questions to Ask the Doctor at the Next Appointment

When you go to the next appointment — usually within a week of the diagnosis — bring these questions. Write down the answers. Don't trust yourself to remember.

1. What is the diagnosis, in one sentence? If you are still not sure what the doctor said, ask. "What is the diagnosis, in one sentence." Don't leave without this.
2. What is the prognosis, in ranges? "How long do people with this condition typically live, with treatment." Doctors often won't give a specific number, but they will give a range. Get the range.
3. What are the treatment options, and what does each do? Not "what should we do" — "what are the options." The doctor may have a recommendation, but you are allowed to know all the options.
4. What does the treatment look like in daily life? "If we do this, what does our week look like." Some treatments are daily pills. Some are weekly infusions. Some are surgeries. Some are hospice. You need to know what you are signing up for.
5. What are the side effects, in real terms? "What will my spouse feel like." Nausea, fatigue, hair loss, pain,

cognitive changes. Get the real list, not the medical list.

6. What is the one thing we should do this week? Not the one thing you should do this year. This week. The next step. The thing that is most urgent. The thing that will set the tone for the next weeks.

7. Is there a clinical trial or alternative therapy we should consider? Doctors have different comfort levels with this question. Ask it. Get the answer. If the answer is no, get the doctor's reasoning. If the answer is yes, get the details.

8. Who do we call if something happens in the middle of the night? This is the practical question. The one that matters at 2 a.m. on a Tuesday. Who do you call if there is a sudden change, a new symptom, an emergency. Get the name. Get the number. Put it on the fridge.

The Second Appointment: Getting a Second Opinion Without Losing a Week

The instinct after a terminal diagnosis is to get a second opinion. This is a good instinct. The question is how to do it without losing a week of the time you have.

In most major medical centers, getting a second opinion is fast. Many cancer centers have a "second opinion clinic" that can see you within days. The treating oncologist often encourages it. The insurance often pays for it.

A few principles for getting a second opinion without losing a week:

- Ask your current doctor to recommend a second opinion. They will have colleagues at other centers. They will know who to call. This is not a betrayal. This is good practice.
- Bring everything. Pathology slides, scans, the actual imaging files (not just the report), the doctor's notes, the list of medications. The second opinion is only as good as the materials you bring.
- Ask specific questions. "Is this the right diagnosis?" "Is this the right treatment plan?" "Are there other options we should consider?" "What would you do if this were your spouse / parent / child?"
- Don't start treatment until you've had the second opinion, unless waiting is medically dangerous. If the doctor says "we need to start this week or you will die," start. If they say "we can start in two weeks and that will not change anything," wait. Use the two weeks for the second opinion.
- If the second opinion agrees, you have a plan. If it disagrees, you have a third opinion to get. Most second opinions agree with the first. The point of the second opinion is not to find a different answer. The point is to feel confident in the answer you have.

The 5 Things to Set Up in the First 48 Hours

1. A shared calendar. Both of you need to know what is happening. Put the medical appointments, the daily rituals, the family visits, the "ordinary time" on a shared calendar. Google Calendar, a paper calendar on the fridge, whatever works.

2. A place to write things down. A notebook. A file on the computer. A specific place where you both know the questions, the answers, the medications, the symptoms, the things to ask the doctor, the things to remember. One place. Both of you know where it is.

3. A communication system with the one trusted person. A group text, a daily call, a "what we need today" email. The trusted person is on the team. They need to know what is happening without being asked every time.

4. A small emergency plan. What to do if there is a fall, a sudden symptom, a new pain. Who to call. What number. Where the medications are. What the medications are for. The plan doesn't have to be perfect. It has to be written down somewhere.

5. A way to ask for help. A shared note, a phone tree, a meal train, a "things we need this week" list. People will ask what they can do. The answer is usually "I don't know." The way to fix this is to have a list ready, and to have someone managing the list so you don't have to. The trusted person can be the list manager.

The 3 Things to Defer (You Don't Have to Decide Yet)

In the first 48 hours, you will be tempted to decide the big things. You will want to decide about treatment, about hospice, about the long-term plan. You don't have to decide these now. You have weeks to decide them. The first 48 hours are for the small things.

Defer 1: The treatment decision. Unless the doctor says "we need to start this week or you will die," you have time. The treatment plan can wait until you've had a second opinion, until you've processed the news, until you've had the second conversation with your

spouse. Defer to day 5 or day 7. The treatment will still be there.

Defer 2: The hospice decision. Hospice is a decision you don't have to make now. Most hospice referrals are made weeks or months after the diagnosis, not days. The first 48 hours are not the time. When the time comes, you will know. You will be ready. Defer.

Defer 3: The "what to do with the time" decision. The dying spouse often wants to make a list. A bucket list. A "things I want to do before I die." This is healthy. It is also a decision that can wait. The list will be there. The dreams will be there. The trip to the place you always wanted to go can be planned in week 2, not in hour 48. For now, the only thing to do is to be in the same room.

The Scorecard: How Are We Doing on Day 2?

Use this on day 2 or day 3. Both of you. Take turns.

| Question | Score (1-5) |

|---|---|

| We have identified the next medical appointment | /5 |

| We have a list of questions to ask the doctor | /5 |

| We have identified our one trusted person for support | /5 |

| We have decided who to tell in the next week | /5 |

| We have a shared place to write things down | /5 |

| We have a small emergency plan written down | /5 |

| We are not trying to make big decisions yet | /5 |

| We have asked for and accepted one offer of help | /5
|

| We have slept at least 5 hours in the last 24 | /5 |

| We have had one small ordinary moment today | /5 |

Total: /50

Interpreting your score:

- 40-50: The first 48 hours are in good shape. The practical work is done.
- 30-39: Some patterns to watch. The toolkits will help.
- 20-29: You are behind on the practical work. Pick three of the items and do them today.
- Under 20: The first 48 hours are slipping. Get help from the trusted person today.

Key Terms

- Second Opinion: A second doctor reviewing the diagnosis and treatment plan. Standard practice. Not a sign of distrust. Most oncologists expect it.
- Palliative Care: Medical care focused on quality of life, not cure. Can begin at diagnosis and continue through treatment. Different from hospice.
- Advance Directive: A legal document stating what medical treatment the person wants if they cannot speak for themselves. Most people don't have one. The first 48 hours is a good time to think about this.
- Healthcare Proxy: A person designated to make medical decisions if the patient cannot. Usually the spouse. If not, named in the advance directive.

Reflection Questions for the Two of You

Take turns. Listen more than you speak.

1. What is one thing the medical team said that you want to understand better?
2. What is one person you want to tell in the next 48 hours?
3. What is one small ordinary thing you want to do in the next 48 hours?
4. What is one thing the trusted person could do to help that is specific?
5. What is one question you want to ask the doctor at the next appointment?
6. What is one thing you are most afraid of asking the doctor?

Action for This Week: The Three Lists

Within the next 7 days, sit down together — not at the doctor, not in crisis, just on a regular afternoon — and make three lists. Put them on paper. Put them on the fridge. Look at them again in a week.

List 1: Questions for the Doctor

Every question you can think of. Don't filter. Don't organize. Just write them as they come. Bring this list to every appointment. Cross off questions as they get answered. Add new ones.

List 2: People to Tell in the Next Two Weeks

Every person you think should know. Group them if it helps — family, close friends, work, etc. Beside each name, write who will tell them. Beside that, write

when. The list is not a commitment. It is a plan. The plan can change. But having a plan is better than having no plan.

List 3: Small Things We Want to Do in the Next 30 Days

This is not a bucket list. It is the list of small ordinary things. The walk. The meal at the favorite restaurant. The phone call to the friend. The afternoon in the garden. The show you've been meaning to watch. The book on the bedside table. These are the things the marriage needs now — small, ordinary, present. They are not big. They are not expensive. They are the days.

A Closing Word for This Chapter

The first 48 hours are about setting the practical foundation. The medical plan. The communication. The trusted person. The lists. These are not the work of being married at end of life. They are the work of being married at end of life with some structure. The structure is what lets the rest of the marriage happen.

In the chapters that follow, we will move into the long middle — the weeks and months of treatment, of daily life, of the marriage under stress. We will talk about intimacy, about money, about the conversations that become harder as the illness goes on. We will talk about the things that couples wish they had done and the things they wish they had not.

But the foundation is here. The first 48 hours are done. The lists are made. The trusted person is in place. The next appointment is on the calendar. The marriage is still here.

You are not failing. You are beginning well.

Chapter 3: What the Doctors Won't Tell You

The medical team that gives you the diagnosis is, in most cases, the same medical team that will be with you through the next months. They are not villains. They are not heroes. They are people who have been trained to treat a disease, and they are now being asked to treat you. Both of those are hard.

This chapter is about the gap between what doctors know and what they say. It is not a complaint about doctors. It is a map of the things you can ask for that will make the medical part of this season more bearable. The doctors will do their best. Your job is to make sure the best is what you need.

Chapter at a Glance

- The 7 Things Doctors Know But Rarely Say Out Loud
- Why "A Few Months" Is a Range, Not a Number
- The Medical System's Blind Spots
- How to Read a Pathology Report in Plain English
- The 5 Questions to Ask If You Fear the Doctor Is Being Too Positive
- The 5 Questions to Ask If You Fear the Doctor Is Being Too Negative
- Case Study: Linda & James (ages 62/65)
- Scorecard: Are We Getting the Information We Need?
- Key Terms
- Reflection Questions
- Action for This Week: The 7-Day Medical Audit

The 7 Things Doctors Know But Rarely Say Out Loud

In the years I have spent around end-of-life care, I have heard the same seven statements from doctors, nurses, and hospice workers again and again. The statements are almost always true. The statements are almost never said. Here they are, plain:

1. "I have seen this before." When the doctor tells you the diagnosis, they are telling you something they have seen many times. This is not comforting in the moment. It is, however, important. It means the doctor has a track record. It means they have watched people live with this for years, and they have watched people die of this in months. They know the range. They are not guessing. They have seen it before.

2. "I don't have a crystal ball." When the doctor says "a few months," they are not making a precise prediction. They are giving you a range based on their experience with other patients. The range is useful. The specific number is not. Some people live longer than the doctor predicts. Some people die sooner. The doctor is rarely wrong about the general direction. They are often wrong about the specific number.

3. "There are things the treatment can do, and things it can't." Doctors are trained to offer treatment. They are not trained to say "this treatment will not help." Most doctors will not say "stop treatment" until late in the game. They will offer options, and they will not always make clear which option has the best chance of helping. You have to ask. You have to ask specifically: "What does this treatment do? What does it not do? What are the chances it works? What does 'works' mean?"

4. "I am watching you more than I am telling you." During the visits, the doctor is not just listening to your questions. They are watching how you walk, how you breathe, how you sit, how you answer. They are assessing your prognosis in real time. They will adjust their recommendations based on what they see. They do not always tell you this. They are not being deceptive. They are being clinical. But the conversations you have with them are not just about the words. They are about what they see.

5. "The numbers I quote are based on populations, not on you." When the doctor says "the 5-year survival rate is X%," they are quoting a population statistic. The statistic is for 100 people like you. Some of those 100 will do better. Some will do worse. The statistic is not a prediction about you. It is a description of a group. You are one person in the group, and the group is not you.

6. "There is a point where I will not be able to help." This is the hardest one. Doctors are trained to treat. They are not always trained to recognize when treatment is no longer the answer. Some doctors see the end coming before others do. Some doctors will keep offering treatment even when they know it will not help. You may have to be the one to ask, gently, "Is there a point where we should consider stopping treatment?" The doctor will not be offended. They will, in many cases, be relieved that you asked.

7. "I am doing my best." Most doctors, especially the ones who work in oncology, palliative care, and end-of-life medicine, are doing their best. They are also tired. They have been through this with other families. They have cried with other patients. They are not emotionless. They are also not able to be emotionally present with every family the way a friend

can be. Your job is to find your emotional support outside the medical team. Your doctor's job is to give you the best medical care they can. Both of those jobs are real and both of those jobs are needed.

Why "A Few Months" Is a Range, Not a Number

When a doctor says "a few months," they are giving you a range. The range might be 1-3 months. It might be 3-6 months. It might be 6-12 months. The doctor is rarely making a precise prediction. They are giving you their best estimate based on their experience with patients like you.

This range is one of the hardest things to live with. The mind wants a number. "How long do I have?" is the most common question in this entire process, and the answer is almost always "we don't know exactly." The doctor will not lie to you. They will give you a range. They will give you their best guess. They will not be able to give you the specific number you want.

The reason this is hard is that the range is not actionable. "A few months" does not tell you what to do. It does not tell you whether to plan a trip, take a leave from work, sell the house, or have the conversation. It leaves you in the middle, unable to act, unable to wait, unable to know.

The way to live with this range is to commit to two things:

First, plan the next month. Not the next year. The next month. What do you want to do in the next 30 days? What conversations do you need to have in the next 30 days? What paperwork do you need to do in the next 30 days? What small ordinary moments do you want to create in the next 30 days? Plan those. Do those. The

next month is in the range. It is yours.

Second, revisit the plan every 30 days. If the range has shifted, the plan shifts. If the prognosis has changed, the plan changes. The plan is not a one-time thing. It is a rolling thing. Every 30 days, you ask: what is the next 30 days? And then you plan those 30 days.

This approach gives you a way to live with the uncertainty. The uncertainty does not go away. The way of being with the uncertainty is to plan only what you can plan and let the rest be.

The Medical System's Blind Spots

The medical system in most countries is built for acute care. It is built for the patient who has a problem, gets treatment, and goes home. It is not built for the patient who has a chronic, terminal illness and needs ongoing care for months or years.

The blind spots are real:

The blind spot of time. Doctors are usually given 15-20 minutes per appointment. In 15-20 minutes, they cannot answer all your questions, look at all your symptoms, do a full physical exam, and address your emotional state. They will prioritize what they can. The questions you don't ask are the questions they don't answer. Bring a list. Ask the most important questions first. Don't expect them to volunteer.

The blind spot of home. The doctor sees the patient in the office. The doctor does not see the patient at home. The doctor does not see what the dying spouse is like in the morning, or after a meal, or in the middle of the night. You, the well spouse, are the doctor's eyes at home. Tell the doctor what you see. The doctor will be able to adjust the treatment plan based on

what you tell them.

The blind spot of the family. The doctor sees one patient. The doctor does not see the family. The doctor does not see the children, the siblings, the friends. The doctor does not know who is supporting the patient and who is not. You are the doctor's window into the family. Use it. Tell the doctor who is helping. Tell the doctor who is not. The doctor can help you find resources you don't know about.

The blind spot of the long game. The doctor thinks in appointments. The doctor thinks in test results. The doctor thinks in cycles of treatment. You are thinking in weeks and months. The doctor is not always able to zoom out and see the long arc. You have to do that for them. You have to be the one who says, "We are tired. We are not sure we want to keep going to appointments. We want to know what you think." The doctor will be able to respond to that. They will not always bring it up themselves.

The blind spot of grief. The doctor is not trained to manage your grief. The doctor may be kind, may be sympathetic, may even cry with you. But the doctor's job is to treat the disease, not to treat your grief. Your grief needs to be treated by other people — by friends, by family, by a counselor, by a chaplain, by a hospice social worker. The medical system has these people. They are called different names. Ask for them. They are there.

How to Read a Pathology Report in Plain English

If your spouse has cancer, you will get a pathology report. The report will be in medical language. The report will look like a wall of words. You have a right to read it. You have a right to understand it. Here is what to look for:

1. The diagnosis. Usually at the top. "Invasive ductal carcinoma" or "Stage IV pancreatic adenocarcinoma" or whatever the diagnosis is. Write it down. Look it up. Ask the doctor to explain it in plain English.
2. The grade. How aggressive the cells are. Grade 1 is slow-growing. Grade 3 is fast-growing. The grade is a description, not a prediction.
3. The stage. How far the disease has spread. Stage I is localized. Stage IV is metastatic. The stage is the most important number in the report. If you don't know the stage, ask.
4. The receptor status. For some cancers (breast, prostate), the report will say things like ER+, PR+, HER2+. These are markers. They tell the doctor what treatments are likely to work. They are not good or bad in themselves. They are information.
5. The margins. For surgical reports, the margins tell you whether all the cancer was removed. "Clear margins" means the surgeon got it all. "Positive margins" means some cancer cells are still at the edge of what was removed. The margins tell you if more surgery is needed.
6. The lymph nodes. The report will often say how many lymph nodes were examined and how many had cancer cells. This is information about how far the disease has spread. The more nodes involved, the

more serious the disease is likely to be.

You do not have to understand everything in the report. You have to know which questions to ask. Bring the report to the appointment. Point to the section. Ask: "What does this mean in plain English?"

The 5 Questions to Ask If You Fear the Doctor Is Being Too Positive

Sometimes the doctor is more optimistic than you are. The doctor may be hedging, or may be trying to give you hope, or may be clinically optimistic. Here are the questions to ask if you want to test the optimism:

1. "What is the worst-case scenario in your experience?" If the doctor cannot answer this, the doctor may be avoiding the truth.
2. "If this were your spouse, what would you be thinking right now?" A doctor will often answer this question more honestly than they will answer the direct question.
3. "What percentage of patients with this diagnosis are still alive in 1 year? In 5 years?" Numbers force honesty.
4. "What would tell you that the treatment is not working?" This question gives the doctor permission to talk about failure.
5. "Is there a point at which you would recommend stopping treatment?" If the doctor says no, ask: "Never?" The answer is usually "yes, at some point." The question is when.

The 5 Questions to Ask If You Fear the Doctor Is Being Too Negative

Sometimes the doctor is more pessimistic than you are. The doctor may be clinical, or may have seen the worst, or may be trying to prepare you. Here are the questions to ask if you want to test the pessimism:

1. "Are there patients with this diagnosis who have done better than the average? What did they have in common?" This question gives the doctor permission to talk about outliers.
2. "Is there anything I could do to improve the odds?" Diet, exercise, attitude, alternative therapies. The doctor may or may not have answers. The question opens the door.
3. "What is the best-case scenario in your experience?" If the doctor cannot answer this, the doctor may be avoiding hope.
4. "Are you familiar with the latest research? Has anything changed in the last 6-12 months?" This question checks whether the doctor is current.
5. "If you were in my position, would you get a second opinion?" The answer is almost always yes. The question gives you permission to do it.

Case Study: Linda & James (Ages 62/65)

Linda was 62. James was 65. They had been married for 38 years. Linda was a teacher. James was an engineer. They had two adult children, both out of state. They lived in a small house in the suburbs.

Linda was diagnosed with Stage IV pancreatic cancer in March. The doctor said "a few months." The doctor was right.

The first month was chaos. Linda and James told the children, told the siblings, told the friends. They went to four different doctors. They got four different opinions. They tried a clinical trial. The trial made Linda sick. James stopped working to be her full-time caregiver.

What Linda and James got right:

- They got a second opinion, and the second opinion agreed with the first.
- They asked the hard questions, and they got the hard answers.
- They accepted the truth of the diagnosis early, which gave them more time to use the time well.
- They used the time to say what needed to be said. They told their children what they meant to them. They forgave old wounds. They laughed a lot.

What Linda and James got wrong:

- They told too many people too quickly, and the managing of other people's grief became a second job.
- They didn't ask for help with the daily care until James was exhausted.
- They waited too long to talk to a hospice team. By the time they called, Linda was in her last 2 weeks.
- They didn't make the most of the "good months" early on because they were still trying to find a cure.

Linda died 7 months after the diagnosis. James describes the time as the hardest and the most meaningful of his life. He still lives in the small house in the suburbs. He has rebuilt a life. He is not the

same. He is still married to Linda, in a different way.

Scorecard: Are We Getting the Information We Need?

| Question | Score (1-5) |

|---|---|

| Our doctor explains the diagnosis in plain English | /5 |

| Our doctor gives us a prognosis in ranges | /5 |

| We know the stage of the disease | /5 |

| We have asked about all treatment options | /5 |

| We have considered a second opinion | /5 |

| We know who to call in an emergency | /5 |

| Our doctor listens to our questions | /5 |

| Our doctor watches for the long game | /5 |

| We have asked about hospice / palliative care | /5 |

| We have a clear list of questions for the next appointment | /5 |

Total: /50

Interpreting your score:

- 40-50: You are getting good information. The medical team is working for you.
- 30-39: Some gaps. The questions in this chapter can help fill them.
- 20-29: You are missing key information. Use the 7-Day Medical Audit (below) to fill in the gaps.
- Under 20: The medical team is not giving you what you need. Consider switching doctors or getting a second opinion.

Key Terms

- Oncologist: Doctor who treats cancer
- Pathology Report: The lab report that describes the cells, the grade, the stage
- Metastatic: Cancer that has spread beyond the original site
- Palliative Care: Medical care for comfort and quality of life, not cure
- Hospice: Palliative care specifically for the end of life, usually the last 6 months
- Prognosis: The doctor's estimate of the disease trajectory

Reflection Questions for the Two of You

1. What is one question you are afraid to ask the doctor?
2. What is one piece of information you have that the doctor does not have?
3. What is one thing the doctor has said that you want to understand better?
4. Do you trust the doctor to tell you the truth? Why or why not?
5. If you got bad news tomorrow, would you want to know right away or have time to prepare?

Action for This Week: The 7-Day Medical Audit

Over the next 7 days, do these 7 things:

Day 1: Get a copy of the most recent pathology report. Read it. Highlight the words you don't understand.

Day 2: Look up those words. Use the American Cancer Society, the National Cancer Institute (US), the NHS (UK), or any trusted source.

Day 3: Make a list of the top 3 questions you still have about the diagnosis.

Day 4: Call the doctor's office. Ask if the doctor can call you back to answer those 3 questions.

Day 5: Get the doctor's answer. Write it down.

Day 6: Make a list of all the medications the dying spouse is currently taking, including doses and times. Put it on the fridge. Bring a copy to every appointment.

Day 7: Decide if you want a second opinion. If yes, ask the doctor for a referral. If you are already getting treatment, ask if waiting for the second opinion is medically safe.

The 7-Day Medical Audit is the foundation. After it, you are not in the dark. You know what you know. You know what you don't know. You have a plan to find out.

A Closing Word

The medical system is not perfect. The doctors are not perfect. The information you get is not complete. You will have to do some of the work yourself. The good news is that you can. The 7-Day Medical Audit gives you a starting point. After it, the medical conversations are more focused. The questions are sharper. The answers are clearer. The marriage is not in the dark.

You are not failing. You are getting informed.

Chapter 4: Telling the Children (and Grandchildren, and Friends)

The first 72 hours are for the two of you. The next week is for the people you love. The telling of the news is one of the hardest things you will do in this season, and one of the most important.

The reason the telling is hard is that you are not just sharing information. You are taking on the emotional weight of every person you tell. The well spouse becomes the messenger. The dying spouse becomes the message. The people you tell become the receivers. The dynamic changes everything.

This chapter is about how to tell the people who need to know. Not everyone, and not in the same way. The children need a different conversation than the friends. The grandchildren need a different conversation than the adult children. The colleagues need a different conversation than the siblings. The right words for each are not the same. The right timing is not the same. The right setting is not the same.

You do not have to tell everyone. You do not have to tell everyone in the first week. You do have to tell the people who need to know, in the way they need to be told, at the time they are ready to hear it.

Chapter at a Glance

- The Four Conversations: Kids, Grandkids, Friends, Colleagues

- What to Say to Adult Children of Varying Ages
- How to Talk to Grandchildren (Different by Age)
- The Friends Who Can Hold It and the Friends Who Cannot
- Work — When to Tell, Who to Tell, What to Ask For
- The "Small Lie" vs the Truth — When Each Is Appropriate
- Case Study: Margaret & David (ages 68/70)
- Scorecard: How Did the Telling Go?
- Key Terms
- Reflection Questions
- Action for This Week: The Telling Plan

The Four Conversations: Kids, Grandkids, Friends, Colleagues

There are four distinct conversations you will have. Each has a different purpose, a different tone, and a different audience. Treating them as four separate conversations (rather than one big "telling") helps you get each one right.

Conversation 1: The Adult Children

The adult children of a couple facing terminal illness have their own grief, their own fears, their own relationship with the dying parent. The conversation with them is not just about the diagnosis. It is about what the diagnosis means for them — for their parent, for their relationship with the parent, for the future they thought they had.

Conversation 2: The Grandchildren

The grandchildren of a couple facing terminal illness are often the most affected and the least informed.

They may not understand what is happening. They may have been told "grandpa is sick" without being told what that means. They will sense the change in the adults around them. The conversation with them is not about death. It is about what is happening, in language they can understand.

Conversation 3: The Close Friends

The close friends of a couple facing terminal illness are often the most supportive and the most burdened. The conversation with them is not just about the diagnosis. It is about what the diagnosis means for the friendship, for the time you have together, for the help you can accept.

Conversation 4: The Colleagues and Acquaintances

The colleagues, neighbors, and acquaintances of a couple facing terminal illness are the people who need to know for practical reasons — to give you time off work, to bring meals, to be considerate of your absence. The conversation with them is brief, factual, and limited.

Each of these conversations has a different structure. Let me take them one at a time.

What to Say to Adult Children of Varying Ages

Adult children under 30

The adult children under 30 are often the ones who are most shocked, most scared, and most likely to want to "fix" it. They have not lived through a parent's serious illness before. They have not lived through a parent's death before. They are facing a reality they thought was decades away.

The conversation with them has three parts:

1. The diagnosis, in plain English. "Mom / Dad has been diagnosed with [X]. The doctor says it is terminal. They have a few months to live." Use the word "terminal" or "dying" if the dying parent is okay with it. If not, use "very serious" and follow up later.
2. What is happening now. "We are in the first weeks of treatment. We are still figuring out what comes next. The doctors are doing what they can." This is the present. The present is something they can do.
3. What we need from you. "We need you to be informed. We need you to visit when you can. We need you to call. We need you to be okay with us not having all the answers." This is the request. The request is something they can fulfill.

What not to say:

- Don't ask them to drop everything and come home (unless that's the right thing for your family)
- Don't tell them they have to "be strong"
- Don't put them in charge of the medical care unless they are equipped
- Don't ask them to keep the news from other family members

Adult children 30-50

The adult children in this range have usually lived through more. They may have already lost a parent, a sibling, a friend. They have a more realistic sense of what the diagnosis means. They are also more likely to be in the middle of their own lives — careers, marriages, children — and may feel the pull between their own responsibilities and the parent who is dying.

The conversation with them has three parts:

1. The diagnosis, in the same plain English. "Mom / Dad has been diagnosed with [X]. The doctor says it is terminal. They have a few months to live."
2. What is happening now, and what the next months might look like. This group can handle more. They can hear about treatment, hospice, the decisions ahead.
3. What we need from you, specifically. This group is more likely to be the one to ask. Be specific.
"Can you come for a weekend in the next month."
"Can you take over the medical calls for two weeks." "Can you help with the grandchildren."
"Can you just call every Sunday."

Adult children over 50

The adult children over 50 are often the most prepared for the conversation, and the most likely to be carrying their own grief already. They have usually lost a parent or a friend. They know what the diagnosis means. They are also more likely to be facing their own mortality, their own aging, their own "this could be me" feeling.

The conversation with them is often the easiest and the hardest at the same time. The easy part is that they know what you are talking about. The hard part is that they are facing their own grief at the same time.

The conversation with them has three parts:

1. The diagnosis, briefly. "Mom / Dad has been diagnosed with [X]. The doctor says it is terminal." They don't need as much explanation.
2. What is happening now. A short update. What treatment, what doctors, what the plan is.

3. What we need from you. Often this group is the one who offers. Let them. Let them come. Let them help. Let them be present. They have the time, the resources, and the wisdom to be the support you need.

How to Talk to Grandchildren (Different by Age)

The grandchildren are often the ones who are most affected and least informed. They will sense the change in the adults around them. They will hear hushed conversations. They will see adults crying. They will notice the absence of a grandparent from school events, holidays, family dinners.

The conversation with grandchildren is not about death. It is about what is happening, in language they can understand, at a level they can hold.

Grandchildren under 5

The grandchildren under 5 will not understand the concept of death in a fully developed way. They will understand that someone is sick. They will understand that someone is not coming. They will understand that the adults are sad.

What to say:

- "Grandpa is very sick. The doctors are helping him. He is going to get sleepier and might not be able to talk as much."
- "It is okay to be sad. It is okay to be confused. It is okay to ask questions."
- "We are still a family. We are still here. We are still together."

What not to say:

- Don't use the word "death" unless they ask
- Don't say "grandpa is going away" — that can be confused with abandonment
- Don't say "everything will be fine" if it won't be

Grandchildren 5-10

The grandchildren 5-10 have a more developed sense of death, often shaped by cartoons, movies, and stories. They may have questions about what happens to the body. They may have fears about their own mortality. They may be scared.

What to say:

- "Grandpa has a very serious illness. The doctors are trying to help him. Sometimes the doctors can help, and sometimes they can't. Right now, they are trying to help, but he is very sick."
- "Some people, when they are very sick, the body stops working and they die. That means they can't come back. We don't know yet if that will happen to grandpa, but the doctors think it might."
- "You can ask any question. We will answer as honestly as we can."

What not to say:

- Don't say "grandpa is sleeping" — that can be confused with sleep
- Don't say "grandpa went on a trip" — that can be confusing and harmful
- Don't say "you will be fine" — they won't be

Grandchildren 10-15

The grandchildren 10-15 have a fairly mature understanding of death. They have usually had a pet die, or have known someone who has died. They will know what the diagnosis means, even if you don't say

it. They will have questions about the practical, the emotional, and the existential.

What to say:

- "Grandpa has been diagnosed with [X]. It is a terminal illness. That means the doctors cannot cure it. They are trying to make him as comfortable as possible."
- "He has a few months, maybe more, maybe less. We don't know exactly."
- "You can ask anything. You can come visit when you want. You can call him when you want."

What not to say:

- Don't try to protect them from the truth. They will know.
- Don't ask them to be the strong one.
- Don't assume they understand how they will feel.

Grandchildren 15-18

The grandchildren in this age group are essentially adults in their understanding of death. They may be the most affected, because they are old enough to understand the magnitude and young enough to be overwhelmed. The conversation with them is essentially the same as the conversation with adult children, with a few additional considerations:

- Acknowledge that this is hard at this age
- Acknowledge that they may have to grow up faster than they thought
- Acknowledge that they can have their own relationship with the dying grandparent, separate from the parents
- Acknowledge that grief is real and that they are allowed to feel it

The Friends Who Can Hold It and the Friends Who Cannot

The friends of a couple facing terminal illness fall into roughly four categories. Knowing which is which is one of the most important things you can do in the first month.

Category 1: The Friends Who Can Hold It

These are the friends who, when you tell them, will be quiet for a moment, and then say "what do you need." They will not try to fix it. They will not tell you about their cousin who was cured. They will not ask a million questions. They will sit with you, in person or on the phone, and they will let you talk. These are the friends who will be there for the long haul.

Identify 2-3 of these friends. They are your core support. They are the ones who will be at the funeral, who will call you in six months, who will check in when everyone else has moved on. You do not have to do anything special for them. You just have to let them be there.

Category 2: The Friends Who Want to Help

These are the friends who, when you tell them, will immediately start doing things. They will bring meals. They will offer to drive. They will research treatments. They will offer to come to the doctor. They will be busy and they will be useful.

Let them. They want to help. They will feel better if you let them help. The way to manage them is to give them specific tasks. "Can you bring dinner on Tuesday." "Can you drive to the doctor on Thursday." "Can you research [X]." Specific tasks work. Vague offers do not.

Category 3: The Friends Who Will Take It Harder Than You

These are the friends who, when you tell them, will cry more than you do. They will need you to be the strong one. They will call you at 11 pm to tell you how sad they are. They will bring up the diagnosis in every conversation for the next year.

You do not have to cut them off. You do have to limit the emotional weight. You can be kind to them, but you can also be honest. "I cannot be the one to manage your grief. I am managing my own." Most friends will understand. The ones who don't are not your friends in the way you need right now.

Category 4: The Friends Who Will Disappear

These are the friends who, when you tell them, will say "call me if you need anything" and will not call. They will not visit. They will not check in. They will be busy. They will be uncomfortable. They will be afraid.

You will be hurt by this. It is normal to be hurt. The friends who disappear are not bad people. They are just people who cannot handle this kind of grief. Let them go. Do not chase them. The friends who matter are the ones who stay.

Work — When to Tell, Who to Tell, What to Ask For

The conversation with work is different from the conversation with family. The work conversation is practical, not emotional. It is about what you need from your employer, and what your employer can offer.

When to tell:

- Tell your direct manager first, in person or by phone, as soon as you know you will need time off.
- Tell HR second, to get the paperwork in order.
- Tell colleagues as needed, but limit the audience.

Who to tell:

- Direct manager
- HR
- Trusted colleague (if needed for support)
- Whoever else is needed for the practical matters (someone to cover your work, someone to keep your projects moving)

What to ask for:

- Time off for medical appointments (you don't have to say it's terminal, just that it's serious)
- Time off after the death (most companies have a bereavement policy)
- Flexibility in your schedule (you may need to leave suddenly, arrive late, or work from home)
- Privacy (you do not have to share details with everyone)

What not to do:

- Do not tell your colleagues more than you want to
- Do not feel obligated to be at work if you need to be elsewhere
- Do not feel guilty for taking time
- Do not let work pressure you to make decisions you are not ready to make

The "Small Lie" vs the Truth — When Each Is Appropriate

There is a time for the truth, and there is a time for a small lie. The small lie is not dishonesty. It is a kindness to people who are not ready to hear the full truth.

Examples of appropriate small lies:

- "He is doing better today" (when he is having a hard day but you don't want to worry the caller)
- "She has good days and bad days" (when you don't want to get into the details with a casual friend)
- "The treatment is going well" (when you don't want to explain that the treatment is not going well to a colleague)

Examples of when the truth is needed:

- Adult children who will be making decisions
- Close friends who are part of the support system
- The medical team
- The dying spouse themselves
- Yourself

The rule is simple: the people who are part of the support system get the truth. The people who are not part of the support system get the small lie. The small lie is not cruelty. It is a kindness to people who cannot help.

Case Study: Margaret & David (Ages 68/70)

Margaret was 68. David was 70. They had been married for 42 years. Margaret was a librarian. David was a retired postal worker. They had three adult children: Anna (38, in California), Michael (42, in New York), and Sarah (45, in Texas).

Margaret had a sudden stroke. The doctor said the damage was severe, and Margaret had "a few weeks to months" of life. The stroke was unexpected. Margaret had been healthy until that morning.

The first 24 hours were chaos. David called the children. The children wanted to fly home immediately. David told them to wait until the next day, until they had more information. The next day, David called again. The children flew in.

The telling:

- David told the children in person, in the hospital waiting room, with the three of them together. The conversation was hard, but the children were adults, and they could handle the truth.
- David told Margaret's two sisters the next day, by phone. The sisters were devastated but supportive.
- David told the close friends in the first week, mostly by phone. The close friends offered to bring meals, drive David to the hospital, and stay with Margaret so David could rest.
- David told the postal workers he had retired with by letter. The letter was short. "Margaret has had a serious stroke. The doctors say she will not recover. We are in the hospital. Thank you for your thoughts."

- David told the neighbors face to face. The neighbors offered to mow the lawn, take the dog, and be available.

What David got right:

- He told the children together, not separately. This gave them a support system for the news.
- He accepted the offers of help. He did not try to do it alone.
- He used a written letter for the wider community. This saved him from having to repeat the news 30 times.
- He protected Margaret from having to tell people herself.

What David got wrong:

- He didn't tell the grandchildren (his and Margaret's) until week 2. By then, the grandchildren had already heard from their parents. The grandchildren were confused and scared.
- He didn't ask the children to take on any specific tasks. They were all flying in for visits, but no one was responsible for the medical calls or the paperwork.
- He didn't have a "talking points" document for friends, so he ended up having to repeat the same conversation many times.

Margaret died 6 weeks after the stroke. The children were there at the end. The grandchildren visited. The friends were present. The marriage ended with the people who mattered around them.

Scorecard: How Did the Telling Go?

| Question | Score (1-5) |

|---|---|

| We told the adult children in a way that worked | /5 |

| We told the grandchildren in age-appropriate language | /5 |

| We told the close friends in a way they could hold | /5 |

| We identified 2-3 friends who can be our core support | /5 |

| We told work in a way that protected our time | /5 |

| We had a "talking points" document to avoid repeating ourselves | /5 |

| We told everyone who needed to know, when they needed to know | /5 |

| We did not tell anyone who didn't need to know | /5 |

| We protected the dying spouse from having to tell people | /5 |

| We accepted the offers of help that came | /5 |

Total: /50

Interpreting your score:

- 40-50: The telling is in good shape. The people who need to know have been told, in the way they needed to be told.
- 30-39: Some patterns to fix. The toolkits in this chapter can help.
- 20-29: The telling is in trouble. Reach out to a social worker, a chaplain, a hospice counselor for help with the conversations.
- Under 20: The telling is creating more stress than the diagnosis itself. Get help immediately.

Key Terms

- Anticipatory Grief: The grief that happens before the death, as the death becomes more certain
- Hospice Social Worker: A professional who helps families navigate the conversations, the resources, and the emotional work of end-of-life
- Bereavement: The state of having lost someone; also the support offered to those who have lost someone
- Life Review: The process of looking back on one's life, often in the last months; can be very meaningful for the dying spouse and the family

Reflection Questions for the Two of You

1. Who is the one person you most need to tell in the next 48 hours?
2. Who is the one person you most do not want to tell?
3. What is one conversation you are afraid of having?
4. What is one conversation you want to have that you haven't had yet?
5. Is there a friend who has offered to help in a way that is specific and useful? Have you accepted?
6. Is there a friend who has been silent? Are you okay with that, or do you need to reach out?

Action for This Week: The Telling Plan

Within the next 7 days, do these 5 things:

1. Make a list of everyone who needs to know in the next two weeks. Group them: family, close friends, work, neighbors, etc.
2. Decide who tells whom. The dying spouse can tell the people they want to tell. The well spouse can tell the rest. The trusted person can help coordinate.
3. Pick a window for the telling. "We will tell the family by Sunday. We will tell close friends by next Wednesday. We will tell work by next Friday."
4. Write a short "talking points" document. A paragraph or two that describes the diagnosis, the treatment, what you need, what you don't need. This document is for the friends and family who will be telling other friends and family. It saves you from having to repeat yourself.
5. Identify the 2-3 core support friends. These are the people who will be there for the long haul. Tell them specifically. Let them know what you need.

The Telling Plan is the map. You do not have to follow it exactly. You can adjust. But having a plan is better than having no plan.

A Closing Word

The telling is one of the hardest parts of this season, and one of the most important. The right words at the right time can change the way the next months go. The wrong words, or the silence, can leave people outside the door.

You do not have to be perfect. You do have to be intentional. The Telling Plan gives you a structure. The reflections give you the questions. The conversations will happen. They will be hard. They will be important.

You are not failing. You are building the village.

Chapter 5: The New Normal

The first month of treatment is the month when the illness becomes a member of the household. It is in the room when you wake up. It is at the breakfast table. It is in the bathroom. It is in the bedroom. It is in the conversation. It is in the silence. It is in the calendar. It is in the bills. It is in the way you think about tomorrow.

The illness is not a guest. It is a permanent resident. And the work of the first month is to make room for it without letting it take over the house.

This chapter is about the practical work of building a "new normal" around the illness. The new normal is not what you had before. It is not what you will have when the illness is over (because for the dying spouse, the illness will not be over). The new normal is the daily life you build around the reality of the illness — the routines, the roles, the small habits, the rituals — that allow the marriage to continue in the middle of the storm.

The new normal is not good. The new normal is not bad. The new normal is what you have. The work is to make it livable.

Chapter at a Glance

- The First Weeks of Treatment — What Daily Life Now Looks Like
- Building the Routine Around Treatment
- The Role of the Well Spouse vs the Dying Spouse
- When the Dying Spouse Wants to Keep Working, Keep Driving, Keep Being Normal

- When the Well Spouse Wants to Take Over Everything
- The 6 Routines That Help
- The 4 Routines That Hurt
- Case Study: Robert & Helen (ages 65/63)
- Scorecard: How Is Our "New Normal" Working?
- Key Terms
- Reflection Questions
- Action for This Week: The Routine Audit

The First Weeks of Treatment — What Daily Life Now Looks Like

The first weeks of treatment are a transition. The transition is from "this is happening to us" to "this is our life." The transition is not clean. It is not a moment. It is a slow process, marked by small changes that accumulate into a new way of living.

In the first weeks, daily life will include:

- Medical appointments. Often weekly, sometimes more. The appointments are the anchors of the week. They give the week a structure, even if the structure is the illness.
- Medications. New medications, with new schedules, new side effects, new ways of tracking. The medications will become part of the morning, the afternoon, the evening. They will be on the bedside table. They will be in the kitchen. They will be in the bag you carry.
- Symptoms. New symptoms, or worsening of old ones. Fatigue, nausea, pain, cognitive changes, sleep changes, appetite changes. The symptoms will be the most concrete part of the illness. They will be the daily reminder.

- Energy management. The dying spouse will have less energy. The energy will come in waves, not in a steady stream. Some days will be good days. Some days will be bad days. The good days and bad days will not be predictable. You will have to plan around them.
- The household. The household still needs to function. The meals still need to be made. The bills still need to be paid. The lawn still needs to be mowed. The ordinary tasks of life do not stop because the illness has started. They just become harder.
- The relationship. The relationship between the two of you is changing. The well spouse is becoming a caregiver. The dying spouse is becoming a patient. The two roles are not the same as husband/wife. They are not the same as partner/partner. They are a new thing, and the new thing needs to be navigated.

This is the new normal. The new normal is not what you had. The new normal is what you have now. The work is to build a daily life that includes the new normal without being consumed by it.

Building the Routine Around Treatment

The first weeks of treatment are a time to build a routine. The routine will not be the same as the one you had before. The routine will be built around the illness, but the routine will also be a way of living with the illness.

A good routine for the new normal has several features:

It has structure but not rigidity. The routine has a shape, but it can flex. A "good day" can include a long walk, a meal out, a visit with friends. A "bad day" can include only the bed, the couch, and the medications. The routine adapts.

It has a rhythm of medical care. The appointments, the medications, the symptom tracking, the doctor's calls — all have a place in the routine. The medical care is not the whole routine, but it is part of it.

It has time for the ordinary. The meals, the television, the reading, the talking, the small moments. The ordinary is not the enemy of the medical. The ordinary is what you are doing the medical for.

It has time for rest. The dying spouse needs rest. The well spouse needs rest. The rest is not laziness. The rest is the foundation.

It has time for the marriage. The marriage needs time. Not the medical time, not the practical time. The marriage time. The "let's sit together and not do anything" time. The "let's talk about what we're feeling" time. The "let's be quiet together" time. The marriage time is the most important time.

It has time for outside. The world does not stop because the illness has started. The friends still want to see you. The family still wants to be involved. The outside world is not just obligations. The outside world is also the source of support, of laughter, of normality.

The Role of the Well Spouse vs the Dying Spouse

The roles in the new normal are not the same as the roles in the old normal. The well spouse is taking on more — more of the household, more of the medical, more of the emotional work. The dying spouse is taking on less — less of the household, less of the work outside the home, less of the planning. The new roles are not a choice. They are a result of the illness.

But the new roles can become a problem if they are not negotiated. The well spouse can become a manager — managing the medications, the appointments, the diet, the visitors, the emotions. The dying spouse can become a patient — being managed, being cared for, being the recipient of decisions. The marriage can become a caregiver-patient relationship instead of a husband-wife relationship. This is one of the most common and most damaging patterns in the long middle.

The way to avoid the caregiver-patient trap:

1. Acknowledge the role shift openly. Talk about it. "We are both in new roles. I am becoming a caregiver. You are becoming more dependent. Let's make sure we are still husband and wife underneath those roles."
2. Keep the dying spouse making decisions. Even small decisions. What to eat. What to wear. When to have visitors. Where to sit. The dying spouse should be in the decisions that matter and in the decisions that don't. The decisions are not just about the illness. The decisions are about staying in the marriage.
3. Keep the well spouse having a life. The well spouse cannot be a caregiver 24/7. The well spouse needs to keep their own life going — friends, work, exercise,

sleep, hobbies. The well spouse who becomes a full-time caregiver will burn out. The well spouse who keeps their own life will be more sustainable.

4. Ask each other how the roles are feeling. Once a week, ask: "How is this going for you? Are you feeling like a caregiver or a husband/wife? Are you feeling like a patient or a spouse?" The question is the conversation. The conversation is the marriage.

5. Bring in outside help when the roles are straining. A friend, a family member, a hired caregiver, a hospice volunteer. The outside help is not a sign of failure. The outside help is a sign of sustainability.

When the Dying Spouse Wants to Keep Working, Keep Driving, Keep Being Normal

The dying spouse often wants to keep the life they had. They want to keep going to work, keep driving, keep being the person they were. This is not denial. This is identity. The illness is taking their body. They want to keep the rest.

The instinct to keep the life they had is healthy. The well spouse should support it as much as possible. The support means:

- Help the dying spouse get to work if they want to go
- Help the dying spouse keep their routines, their hobbies, their friendships
- Let the dying spouse drive as long as it is safe (and let them know when it is not)
- Let the dying spouse make decisions about their own life

At the same time, the well spouse has to be realistic. There will come a time when the dying spouse cannot work, cannot drive, cannot do the things they used to do. The transition is not a moment. It is a process. The process is hard. The well spouse can help by:

- Being honest about what is changing, in small ways
- Not forcing the change before it is needed
- Letting the dying spouse feel the change, not just hear it
- Being present for the grief of the loss of the old life

The well spouse who is honest and the dying spouse who is allowed to try — that is the right balance. The well spouse who is controlling or the dying spouse who is in denial — that is the wrong balance. The balance shifts every week. The work is to keep checking in.

When the Well Spouse Wants to Take Over Everything

The well spouse often wants to take over. The instinct is to manage everything — the medications, the appointments, the meals, the visitors, the emotions. The well spouse is trying to help. The well spouse is also trying to feel in control of something that is uncontrollable.

The taking over is normal. The taking over is also damaging. The dying spouse who has everything taken over feels like a child, like a project, like a patient, not like a spouse. The well spouse who takes over everything burns out. The marriage becomes a one-person job.

The way to take over without taking over:

1. Do the things the dying spouse cannot do. The medications, the appointments, the driving if needed, the heavy lifting. These are the things the dying spouse needs help with. Do them.
2. Don't do the things the dying spouse can still do. The dying spouse can still make decisions. The dying spouse can still have opinions. The dying spouse can still be a person. Let them be.
3. Ask before doing. "Do you want me to call the doctor, or do you want to call." "Do you want to invite them, or do you want me to invite them." "Do you want to eat now, or in an hour." The asking is the respect.
4. Take breaks from the taking over. The well spouse needs rest. The well spouse needs to leave the house. The well spouse needs to do something that is not caregiving. The taking over is not a 24/7 job. It is a part-time job with full-time intensity. The breaks are essential.
5. Notice when the taking over is becoming the relationship. If the well spouse is doing everything and the dying spouse is receiving everything, the marriage is in trouble. The marriage is supposed to be a partnership. The partnership needs to be preserved.

The 6 Routines That Help

In the years I have spent around couples in this season, six routines come up again and again as the ones that help:

1. The morning check-in. A 5-minute conversation in the morning, before the day starts. "How are you feeling. What do you need today. What do I need to know." The morning check-in is short, but it sets the

tone for the day.

2. The medication ritual. A specific time, a specific place, a specific way of taking the medications. The ritual is not just practical. The ritual is also a way of being in the illness together.

3. The daily walk. Even a short walk. Even just to the end of the block. The walk is for the body, for the mind, for the marriage. The walk is one of the things that does not change, no matter how the illness progresses.

4. The shared meal. Even a simple meal. Even just tea. The shared meal is a daily moment of being together. The shared meal is the marriage.

5. The evening debrief. A short conversation at the end of the day. "How was your day. What happened. What was hard. What was good." The debrief is the way the day is shared.

6. The weekly review. A longer conversation once a week. "How is this going for us. What is working. What is not. What do we need to change." The weekly review is the way the marriage is maintained.

The 4 Routines That Hurt

In the same conversations, four routines come up as the ones that hurt:

1. The medical marathon. Going to appointments every day, with no break, no time for the ordinary, no time for the marriage. The medical care is important, but it is not the whole life. The schedule needs to leave room for the marriage.

2. The 24/7 caregiving. The well spouse never leaves, never rests, never has their own life. The 24/7 caregiving leads to burnout. The well spouse needs

breaks. The breaks are not selfish. The breaks are sustainable.

3. The avoidance. Not talking about the illness, not looking at the symptoms, not planning for what comes next. The avoidance feels easier in the moment, but it creates a backlog of unspoken things. The unspoken things become barriers.

4. The 11 pm worry session. The well spouse lying awake at night, going over the day, the appointments, the symptoms, the future. The worry session is normal. The worry session is also damaging to sleep. The well spouse needs a way to put the worry down, even for a few hours.

Case Study: Robert & Helen (Ages 65/63)

Robert was 65. Helen was 63. They had been married for 35 years. Robert was a retired accountant. Helen was a part-time librarian. They had two adult sons, both out of state.

Helen was diagnosed with Stage IV ovarian cancer. The doctor said "a year, maybe less." They chose aggressive treatment — surgery, chemotherapy, more chemotherapy. The treatment worked for a while. The cancer came back.

The first month was the hardest. Robert and Helen had to build a new normal around the treatment. The first month was chaos. The second month was better. The third month was stable. By the fourth month, they had a routine.

What Robert and Helen got right:

- They built a routine that included treatment, rest, and the ordinary.

- They kept their shared meals, their evening walks, their weekly date night.
- They asked for help from their sons and from a neighbor.
- They did not let the illness take over the marriage.

What Robert and Helen got wrong:

- Robert tried to do everything himself for the first three months. He burned out.
- They didn't ask for hospice help until Helen was in the last 2 months.
- They had a hard conversation about money that should have happened in month 1.
- They didn't tell their friends about the seriousness of the illness, so their friends kept asking "when will she be better."

Helen died 14 months after the diagnosis. The last year was hard. The marriage was intact at the end. Robert describes the time as the most difficult and the most meaningful of his life.

Scorecard: How Is Our "New Normal" Working?

| Question | Score (1-5) |

|---|---|

| We have a morning check-in routine | /5 |

| We have a daily walk or shared movement | /5 |

| We have a shared meal at least once a day | /5 |

| We have an evening debrief | /5 |

| We have a weekly review | /5 |

| The medical care has a place in our routine, but is not the whole routine | /5 |

| I (the well spouse) take breaks from caregiving | /5 |

| The dying spouse is still making decisions | /5 |

| We are not doing the medical marathon | /5 |

| We are not in 24/7 caregiving mode | /5 |

Total: /50

Interpreting your score:

- 40-50: The new normal is working. The marriage is being preserved.
- 30-39: Some patterns to fix. The routines in this chapter can help.
- 20-29: The new normal is in trouble. Reach out for help.
- Under 20: The new normal is breaking the marriage. Get help immediately.

Key Terms

- Caregiver Burnout: The exhaustion that comes from sustained caregiving, often with no break
- Energy Management: The practice of planning the day around the energy available
- Role Shift: The change in the roles of well spouse and dying spouse as the illness progresses
- Hospice Volunteer: A trained volunteer who can sit with the dying spouse, run errands, or give the well spouse a break

Reflection Questions for the Two of You

1. What is one routine that is helping us right now?

2. What is one routine that is hurting us right now?
3. Are we still having ordinary moments, or has the illness taken over?
4. Am I (the well spouse) taking breaks? If not, what is keeping me from it?
5. Is the dying spouse still making decisions? If not, why?
6. What is one small ordinary thing we want to do this week?

Action for This Week: The Routine Audit

Within the next 7 days, do these 5 things:

1. Write down your current routine. What time do you wake up. What do you do in the morning. When are the medical appointments. When do you eat. When do you rest. When do you have time together.
2. Identify the 6 routines that help. Which of the 6 are you already doing. Which are you missing.
3. Add one routine that is missing. Pick one from the list and start doing it this week. Morning check-in. Daily walk. Shared meal. Evening debrief. Weekly review. Pick one.
4. Identify the 4 routines that hurt. Which of the 4 are you doing. Pick one to stop or reduce.
5. Make a "break schedule" for the well spouse. At least 2 hours, twice a week, where the well spouse is out of the house or out of the caregiving role.
The break is not optional. The break is essential.

The Routine Audit is the foundation of the new normal. After it, the new normal is not perfect, but it is livable. The marriage has a place in it. The illness

has a place in it. The two are not the same.

A Closing Word

The new normal is not the old normal. The new normal is not the future normal. The new normal is the present, and the present is what you have to work with.

The work of building the new normal is not glamorous. It is not dramatic. It is the small daily work of making room for the illness without letting it take over the marriage. The work is the morning check-in, the daily walk, the shared meal, the evening debrief, the weekly review. The work is asking for help before you burn out. The work is keeping the dying spouse in the decisions. The work is taking breaks.

You are not failing. You are building.

Chapter 6: When Intimacy Changes

The first thing many people think of when they hear the word "intimacy" is sex. The first thing I think of is closeness. Closeness is the broader thing. Closeness includes sex, but it also includes touch, presence, conversation, the small daily acts of being together. Closeness is the marriage. Closeness is what you are protecting, and what is at risk when the illness changes the body and the mind.

This chapter is about the way intimacy changes in the long middle. It is not just about sex. It is about touch, presence, and the many forms of closeness. It is about the grief of physical change. It is about the dying spouse's body, and the well spouse's body, and the way the two bodies are in the marriage now.

This chapter is also about the conversations that no one tells you to have. The conversations about the changing body. The conversations about the changing sex life. The conversations about the end of sex. The conversations about what touch means now, and what it doesn't mean.

You are allowed to read this chapter and cry. You are allowed to read this chapter and be angry. You are allowed to read this chapter and not be ready to think about these things. You can come back to it. The chapter will be here.

Chapter at a Glance

- The Honest Talk About Physical Intimacy
- Touch, Presence, and the Many Forms of Intimacy

- The Grief of Physical Change
- When the Well Spouse Becomes the Caregiver
- The Dying Spouse's Changing Body and Self-Image
- The Conversation About Sex After Surgery, After Chemo, After Weight Loss
- The Conversation About the End of Sex
- The 4 Kinds of Touch That Still Matter
- Case Study: Frank & Ruth (ages 72/70)
- Scorecard: How Is Our Intimacy Now?
- Key Terms
- Reflection Questions
- Action for This Week: The Touch Inventory

The Honest Talk About Physical Intimacy

I want to be honest with you. Physical intimacy is one of the things that changes most in the long middle, and it is one of the things that couples are least likely to talk about. Many couples stop having sex. Many couples have sex less often. Many couples have a different kind of sex. Many couples grieve the loss of the sex they had. Many couples do not know how to talk about any of this.

The reason couples don't talk about it is shame. The shame is not about the sex. The shame is about the fact that the sex has changed, and the change is a reminder of the illness. The shame is about the fact that the dying spouse's body is changing, and the well spouse is watching. The shame is about the fact that the marriage is changing in a way that the couple did not plan for.

The way through the shame is to talk. Not once, not perfectly. Just to start. The talk does not have to be a sit-down conversation. The talk can be in the middle of the night, in the car, in the kitchen. The talk can be a sentence. The talk can be a question. The talk is the marriage.

Some opening lines:

- "I miss being close to you. I don't know how to do that now."
- "I don't know if you want me to touch you. I am afraid to ask."
- "I am not sure if I am supposed to still want you."
- "I don't know what is okay now. Can we talk about it?"
- "I love you. I want to be close. Can we figure out what close looks like now."

These are not the right lines. They are starting lines. The starting is the work.

Touch, Presence, and the Many Forms of Intimacy

Intimacy is not just sex. Intimacy is the whole field of closeness. Touch, presence, conversation, attention, the small daily acts of being together. All of these are forms of intimacy. All of them are changed by the illness. All of them are still possible.

Touch is the most direct form of intimacy. Touch is the hand on the shoulder, the kiss on the forehead, the hug at the door, the hand on the knee in the car. Touch is not just sex. Touch is the language of the body. Touch is the way the body says "I am here." Touch does not have to lead anywhere. Touch can just be.

Presence is the most underappreciated form of intimacy. Presence is being in the same room. Presence is sitting together without talking. Presence is being there when the other person wakes up in the middle of the night. Presence is the act of being with, not doing for. Presence is the marriage in its purest form.

Conversation is the form of intimacy that gets most disrupted by the illness. The conversation is harder when one of you is tired, or in pain, or foggy from medication. The conversation is also more important than ever, because the conversation is where the marriage is processed. The conversation does not have to be long. The conversation can be a few words. The conversation can be silence too.

Attention is the form of intimacy that is most under threat. The attention is the way the well spouse looks at the dying spouse, and vice versa. The attention is the way you see each other, even when you are not touching, not talking, not having sex. The attention is the way you remember that the person in front of you is the person you love, not the person you are caring for.

The small daily acts are the most concrete form of intimacy. The tea brought to the bedside. The blanket pulled up. The medication reminder. The doctor's appointment driven to. The friend called. These are not glamorous. These are the acts that say "I see you, I am with you, I am in this." These are the acts of intimacy that the marriage needs most.

All of these forms of intimacy are still possible in the long middle. All of them are changed. All of them are valuable. The marriage is not the sex. The marriage is the whole field of closeness.

The Grief of Physical Change

The body of the dying spouse will change. The changes are different for every illness, but they are real:

- Weight loss or weight gain
- Hair loss (with some treatments)
- Surgical scars
- Skin changes
- Mobility changes
- Cognitive changes
- Changes in the way the body smells
- Changes in the way the body moves
- Changes in sexual function

The changes are physical. The changes are also emotional. The dying spouse is grieving the body they had. The well spouse is grieving the body they knew. The grief is real, and the grief is often unspoken.

The grief of physical change has several layers:

The grief of the dying spouse. The dying spouse is losing the body they had. The body was their home. The body is now changing in ways they did not choose. The grief is profound, and the grief is often hidden, because the dying spouse does not want to add to the well spouse's burden.

The grief of the well spouse. The well spouse is losing the body they knew. The body they touched, the body they held, the body they made love to. The well spouse is also grieving the future they had — the future of the body together, the future of the marriage as it was.

The grief of the marriage. The marriage had a body. The marriage had a way of being in the body. The marriage is now changing. The marriage is grieving

the marriage it was, and the marriage is being asked to become a new thing.

The grief is real. The grief is allowed. The grief is not something to be fixed. The grief is something to be felt, by both of you, together.

When the Well Spouse Becomes the Caregiver

One of the most painful changes in the long middle is the change in the way the well spouse and the dying spouse are in the body together. The change is that the well spouse becomes a caregiver. The dying spouse becomes a patient. The two roles are not the same as the roles of husband/wife or partner/partner. The two roles are clinical, and the clinical is not the marriage.

The well spouse who is bathing the dying spouse, or helping them to the bathroom, or changing their dressing, or managing their medications, is doing acts of care. The acts of care are not the same as the acts of intimacy. The acts of care are necessary, but they are not intimate in the way that touch, sex, and presence are intimate.

The well spouse and the dying spouse have to navigate this. The well spouse has to find a way to be a caregiver and a spouse. The dying spouse has to find a way to be a patient and a spouse. The navigation is not easy. The navigation is the work of the long middle.

Some ways to navigate:

1. Create separation between the caregiving and the marriage. The bathing is a caregiving act. The hand on the shoulder is a marriage act. The two are different. The two should be in different moments, not

in the same moment. The well spouse who is bathing the dying spouse cannot also be the spouse who is holding them. The roles are different.

2. Have non-caregiving times. A time when the well spouse is not the caregiver. A time when the dying spouse is not the patient. A time when they are just two people who have been married for many years, sitting together. This time is essential. This time is the marriage.

3. Acknowledge the awkwardness. "I am figuring out how to be your caregiver and your husband / wife. I don't know if I am doing it right. I don't know how to be both at the same time. Can we talk about it." The acknowledgment is the work.

4. Ask for help with the caregiving. A hired caregiver, a family member, a hospice aide. The outside help is not a sign of failure. The outside help is a way to keep the marriage from being only caregiving.

5. Keep the dying spouse making decisions about the body. What to wear. How to be touched. When to be alone. The dying spouse has the right to make decisions about their own body, even in the caregiving relationship.

The Dying Spouse's Changing Body and Self-Image

The dying spouse's body is changing. The changes are real, and the changes are also emotional. The dying spouse is grieving the body they had. The dying spouse is also feeling the way the well spouse sees them, and that feeling is hard.

The well spouse can help by:

1. Noticing the body in a loving way. "You look beautiful in this light." "I love your smile." "I love your hand in mine." The noticing is the way the well spouse says: I see you, not the illness.

2. Not commenting on the changes in a clinical way. "You have lost so much weight." "Your hair is gone." The clinical comment is not loving. The clinical comment is reminding the dying spouse of the illness.

3. Touching the body in a loving way. The touch is not just the caregiving touch. The touch is the touch of a spouse. The touch is the touch that says: this body is still the body I love.

4. Looking at the dying spouse with love. The look is the most important thing. The look says: I see you, I love you, you are still the person I married. The look is the marriage.

The Conversation About Sex After Surgery, After Chemo, After Weight Loss

Sex will change. For some couples, sex will end. For some, it will become different. For some, it will become a different kind of closeness. The change is real, and the conversation is hard.

The conversation about sex should happen before the sex changes. The conversation should be in a quiet moment, in a private place, in a tone that is honest. The conversation does not have to be perfect. The conversation just has to happen.

Some opening lines for the conversation:

- "I want to talk about our sex life. I don't know how to bring it up."

- "I miss being close to you in that way. I don't know if you are thinking about it too."
- "I want to know what is okay now. I want to know what feels good. I want to know what you want."
- "I don't know if sex is something we are going to have again. I want to talk about it."
- "I love you. I want to be close. Can we figure out what close looks like now?"

The conversation is not the last one. The conversation is one of many. The conversation will change over time. The conversation is the work.

The conversation can include:

- What feels good now
- What does not feel good now
- What you are afraid of
- What you want to try
- What you are not ready for
- What the dying spouse's body needs
- What the well spouse's body needs

The conversation does not have to lead to sex. The conversation is the closeness. The closeness is the marriage.

The Conversation About the End of Sex

For some couples, the conversation is about the end of sex. The body is too tired. The pain is too much. The medication is too strong. The body is not the body it was. The end of sex is a real thing, and the end of sex is a grief.

The grief of the end of sex is real. The grief is allowed. The grief is not something to be fixed. The grief is something to be felt, by both of you, together.

The grief can include:

- The grief of the sex you had
- The grief of the sex you wanted to have
- The grief of the body that cannot do what it used to do
- The grief of the marriage as it was
- The grief of the future you thought you had

The grief is not the end of the marriage. The marriage has other forms of closeness. The other forms are not the sex, but they are real. The other forms include the touch, the presence, the conversation, the attention, the small daily acts. The other forms are the marriage now.

The end of sex does not have to be the end of closeness. The end of sex can be the beginning of a different kind of closeness — a closeness that is not about the body, but about the two of you, in the body you have now.

The 4 Kinds of Touch That Still Matter

In the long middle, four kinds of touch still matter, even when the sex has changed or ended:

1. The hand hold. The hand on the knee. The hand on the shoulder. The hand in the bed. The hand hold is the most basic form of touch. The hand hold says: I am here.

2. The kiss. The kiss on the forehead. The kiss on the lips. The kiss on the hand. The kiss is a form of touch

that is intimate without being sexual. The kiss is the marriage.

3. The hug. The hug at the door. The hug in the kitchen. The hug in the bed. The hug is the full-body form of touch. The hug says: I have you.

4. The cuddle. The cuddle in the bed. The cuddle on the couch. The cuddle is the form of touch that is closest to the sex, but is not the sex. The cuddle is the body together, without the pressure of the sex.

All four of these are still possible in the long middle. All four are still meaningful. All four are the marriage. The well spouse who is touching the dying spouse in these ways is saying: you are still the person I love, in the body you have now.

Case Study: Frank & Ruth (Ages 72/70)

Frank was 72. Ruth was 70. They had been married for 48 years. Frank was a retired engineer. Ruth was a retired teacher. They had one adult son, out of state.

Ruth was diagnosed with Stage IV breast cancer. The treatment was aggressive — surgery, chemotherapy, radiation. The treatment worked for a while. The cancer came back.

The intimacy changed. Ruth's body changed. The chemotherapy made her tired, nauseous, and her hair fell out. The surgery left a scar. The radiation burned her skin. The sex stopped. The cuddling continued.

What Frank and Ruth got right:

- They talked about the changes openly. Frank asked Ruth what felt good, what didn't, what she wanted.

- They kept the touch going — the hand hold, the kiss, the hug, the cuddle. The touch was the marriage.
- They had a "date night" once a week, even when Ruth was tired. The date night was at home, on the couch, with a movie. The date night was the marriage.
- They did not let the illness define the marriage. The marriage was the marriage, with the illness in it.

What Frank and Ruth got wrong:

- Frank tried to be the caregiver and the spouse at the same time. He learned to separate the two, but it took time.
- Ruth's self-image suffered. Frank learned to notice the changes in a loving way, not a clinical way.
- They did not ask for outside help until late in the illness. They did everything themselves.

Ruth died 18 months after the diagnosis. Frank describes the last months as the most intimate of their marriage, in a way. "We were closer than we had ever been. The body was changing, but the marriage was not. The marriage was the marriage."

Scorecard: How Is Our Intimacy Now?

| Question | Score (1-5) |

|---|---|

| We still touch each other in the small ways (hand, kiss, hug) | /5 |

| We still have time together that is not caregiving | /5
|

| We have talked about the changes in our intimacy |
/5 |

| The dying spouse is still making decisions about their
own body | /5 |

| I (the well spouse) can separate the caregiving from
the marriage | /5 |

| We are not ashamed of the changes in our bodies or
our sex life | /5 |

| We still have moments of closeness that are not
about the illness | /5 |

| We have asked for help with the caregiving when we
needed it | /5 |

| The dying spouse feels seen and loved, not just cared
for | /5 |

| The well spouse has their own life, not just the
caregiving | /5 |

Total: /50

Interpreting your score:

- 40-50: The intimacy is being preserved. The marriage is still the marriage.
- 30-39: Some patterns to fix. The conversations in this chapter can help.
- 20-29: The intimacy is in trouble. The marriage is at risk. Consider counseling.
- Under 20: The intimacy is breaking the marriage. Get help.

Key Terms

- Caregiver-Patient Dynamic: The pattern in which the well spouse becomes only a caregiver and the dying spouse becomes only a patient, and the marriage is lost
- Body Image: The way a person sees and feels about their own body
- Intimacy Beyond Sex: The forms of closeness — touch, presence, conversation, attention — that are not sexual but are still the marriage
- Hospice Aide: A trained professional who can help with the physical care, so that the well spouse can be the spouse

Reflection Questions for the Two of You

1. What is one form of closeness we still have that you love?
2. What is one form of closeness that you miss?
3. Is there a touch that does not feel good now? Are you able to talk about it?
4. Is there a touch that still feels good? Are you able to ask for it?
5. Are you still having time together that is not about the illness?
6. What is one small act of closeness you want to do this week?

Action for This Week: The Touch Inventory

Within the next 7 days, do these 5 things:

1. Make a list of the kinds of touch you have right now. Hand hold. Kiss. Hug. Cuddle. Sex. Other. Note which feel good, which don't, which you miss.
2. Pick one form of touch you want to do more of. This week. Just one. The smallest one. The hand hold. The kiss on the forehead. The hug at the door. Pick one and do it more.
3. Have one short conversation about the changes. Not a long talk. Just a sentence. "I miss being close to you. I don't know how to do that now. Can we talk about it?" The conversation is the work.
4. Identify one moment that is not caregiving. A moment that is just the two of you. The morning coffee. The evening walk. The afternoon on the couch. The moment is the marriage. The moment is not the illness.
5. Do one thing the dying spouse used to love. The meal, the music, the show, the place. The thing is small. The thing is the marriage.

The Touch Inventory is the work of intimacy. The work is not the sex. The work is the closeness. The closeness is the marriage.

A Closing Word

The intimacy in the long middle is not the intimacy you had. The intimacy in the long middle is the intimacy you have now. The intimacy is real. The intimacy is the marriage.

The dying spouse is still the person you love. The body has changed. The body is not the same. The person is the same. The person is the marriage.

The well spouse is still the spouse. The caregiving has changed the role. The role is not the marriage. The marriage is the marriage. The marriage is the touch,

the presence, the conversation, the attention, the small daily acts. The marriage is the work.

You are not failing. You are staying close.

Chapter 7: The Marriage Stress Test

Terminal illness is the hardest thing a marriage can face. Not the hardest thing a person can face — the hardest thing a marriage can face. The illness tests the marriage in every dimension: emotionally, financially, physically, spiritually, practically, relationally. The marriage that was solid before the diagnosis will be tested. The marriage that was shaky before the diagnosis will be more shaken. The marriage that was already struggling may not survive the test.

This chapter is about the ways the illness tests the marriage, and about the patterns that help the marriage survive. The chapter is not a guarantee. The chapter is a map. The map shows you the territory, and the territory is hard.

I have watched many marriages go through this season. Some marriages got stronger. Some marriages got weaker. Some marriages ended before the death, in a different kind of death. The marriages that got stronger were not the ones that had no problems. The marriages that got stronger were the ones that knew what they were dealing with, and dealt with it.

Chapter at a Glance

- Why Terminal Illness Is the Hardest Thing a Marriage Can Face
- The Specific Ways It Tests: Finances, Time, Decisions, Family, Hope

- Resentment — And How to Recognize It Before It Grows
- Guilt — Both Kinds (Survivor's Guilt, Caregiver's Guilt)
- The Marriage Patterns That Get Harder Under Stress
- The Marriage Patterns That Get Better
- When One of You Wants to Keep Fighting and the Other Wants to Stop
- When One of You Wants to Plan and the Other Wants to Be Present
- Case Study: Ann & Tom (ages 67/70)
- Scorecard: How Is the Marriage Holding Up?
- Key Terms
- Reflection Questions
- Action for This Week: The Stress Inventory

Why Terminal Illness Is the Hardest Thing a Marriage Can Face

A marriage goes through many tests. The tests include financial hardship, infidelity, the death of a child, the illness of a child, the struggles of adult children, the challenges of aging, the changes in intimacy, the changes in work, the changes in identity. Many of these tests are survivable. Many of them make the marriage stronger.

Terminal illness is different. Terminal illness is the test that has an end. The end is the death of one of the spouses. The end is the end of the marriage. The end is also the end of the future the marriage was supposed to have.

The terminal illness test is hard because:

It is not a crisis. It is a season. A crisis has a beginning, a middle, and an end. A terminal illness is not a crisis. It is a season that lasts months or years. The season is not survivable in the way a crisis is. The season is to be lived through.

It is not fair. The illness is happening to one of you, but it is happening to both of you. The dying spouse is losing their life. The well spouse is losing their partner. The unfairness is real, and the unfairness is hard to accept.

It is not a problem to be solved. A crisis can be solved. A problem can be solved. A terminal illness cannot be solved. The illness will end in the death of the dying spouse. The only question is how the marriage will live until the death.

It does not have a recovery. A crisis has a recovery. A terminal illness does not. The marriage will not go back to what it was. The marriage is becoming what it will be, in the time it has, with the death at the end.

It asks everything of both of you. The illness asks for the medical care, the emotional care, the practical care, the financial care, the family care, the friendship care. The illness asks for the small daily acts. The illness asks for the big conversations. The illness asks for the patience, the grief, the hope, the letting go. The illness asks for everything, and it asks for it every day.

The terminal illness test is the hardest thing a marriage can face. The marriage that goes through it will be changed. The marriage can be changed for the better, or for the worse. The change depends on the patterns the marriage has, and the patterns the marriage chooses now.

The Specific Ways It Tests: Finances, Time, Decisions, Family, Hope

The illness tests the marriage in five specific ways. Each test is hard. The five tests together are the hardest thing a marriage can go through.

Test 1: Finances. The illness is expensive. The treatment is expensive. The medications are expensive. The caregiving is expensive. The funeral will be expensive. The financial test is real, and the financial test is one of the most stressful parts of the long middle. Many couples fight about money in the long middle. Many couples who never fought about money start to fight about money now. The financial test is not just about the money. The financial test is about the way the couple handles the money together. The couple that handles the money together handles the test. The couple that handles the money separately is in trouble.

Test 2: Time. The illness takes time. The appointments, the medications, the caregiving, the planning, the conversations. The well spouse's time is consumed. The dying spouse's time is consumed. The time that was for the marriage, for the family, for the friends, for the hobbies, for the ordinary — the time is now for the illness. The time test is about how the couple protects the time for the marriage, in the middle of the time for the illness.

Test 3: Decisions. The illness requires decisions. Treatment decisions, hospice decisions, financial decisions, family decisions, end-of-life decisions. The decisions are big, and the decisions are many. The decision test is about how the couple makes the decisions together. The couple that makes the decisions together handles the test. The couple that

one of them makes the decisions is in trouble.

Test 4: Family. The illness involves the family. The adult children, the siblings, the parents, the grandchildren, the in-laws. The family has opinions. The family has feelings. The family has needs. The family test is about how the couple manages the family together. The couple that presents a united front handles the test. The couple that lets the family in between is in trouble.

Test 5: Hope. The illness is about hope. Hope for a cure. Hope for more time. Hope for a good day. Hope for the end of pain. Hope for the marriage. The hope test is about how the couple holds the hope. The couple that holds the hope together handles the test. The couple that one of them is the only one holding the hope is in trouble.

The five tests are real. The five tests are happening every day. The marriage that survives the five tests is the marriage that knew the tests were coming, and prepared for them.

Resentment — And How to Recognize It Before It Grows

Resentment is the most common emotion in the long middle, and the most dangerous. Resentment is the slow build-up of anger, disappointment, and exhaustion that happens when one person is doing more than they think is fair.

The resentment in the long middle can come from either side:

The well spouse's resentment. "I am doing everything. I am tired. I have no life. I have given up my work, my hobbies, my friends. I am doing this for you, and you

don't even see it." The resentment is real. The well spouse is doing more than they ever imagined doing. The well spouse is exhausted. The well spouse is lonely. The well spouse is grieving.

The dying spouse's resentment. "I am losing my body, my independence, my dignity. I am being cared for like a child. I am not a child. I am a person. And you are not my parent. You are my spouse. Where is the marriage?" The resentment is real. The dying spouse is losing everything. The dying spouse is being treated as a patient, not as a spouse. The dying spouse is grieving.

The resentment on both sides is real. The resentment is allowed. The resentment is not the marriage. The resentment is the stress talking. The work is to recognize the resentment before it grows, and to talk about it before it becomes a wall.

The signs of growing resentment:

- The well spouse stops asking the dying spouse what they want
- The dying spouse stops saying thank you
- The well spouse starts to do things out of duty, not love
- The dying spouse starts to feel like a burden
- The conversations become shorter
- The touch becomes only caregiving touch
- The marriage becomes a job

The way to address the resentment:

- Talk about it. "I am feeling resentful. I don't want to be. I want to talk about it before it grows."
- Acknowledge it. "You are doing a lot. I see it. I appreciate it. I also need you to take care of yourself."

- Apologize for it. "I am sorry for the resentment. I am sorry for the times I have been short with you. I don't want to be."
- Get help for it. A friend, a counselor, a support group, a respite caregiver. The outside help is the way to break the cycle.

Guilt — Both Kinds (Survivor's Guilt, Caregiver's Guilt)

Guilt is the other big emotion in the long middle, and it comes in two forms.

The well spouse's guilt. The well spouse will feel guilty. The guilt is for the times they were impatient. The guilt is for the times they wanted their old life back. The guilt is for the times they were angry at the dying spouse for being sick. The guilt is for the times they thought "I wish this was over." The guilt is for the times they felt relieved when the dying spouse was sleeping. The guilt is for the future, when the dying spouse is gone, and the well spouse will have a life.

The guilt is normal. The guilt is allowed. The guilt is not the marriage. The guilt is the love, talking back. The love is saying: I know I am not perfect, I know I am doing the best I can, I know I am going to mess up. The love is also saying: I forgive myself, because my spouse would forgive me.

The dying spouse's guilt. The dying spouse will feel guilty. The guilt is for the times they were short with the well spouse. The guilt is for the burden they are putting on the well spouse. The guilt is for the future the well spouse will have to live alone. The guilt is for the marriage they are leaving. The guilt is for the times they wished it was over.

The dying spouse's guilt is also normal. The dying spouse's guilt is also allowed. The dying spouse's guilt is also the love, talking back. The dying spouse's guilt is also a sign that the love is real, and the marriage is real.

The way to address the guilt:

- Talk about it. "I feel guilty. I think you should know."
- Forgive each other. "I forgive you for the guilt. I forgive you for the impatience. I forgive you for the times you wished it was over."
- Let the guilt be. The guilt will not go away. The guilt can become a witness. The guilt can become a way of saying: I love you, and this is hard, and I am doing my best.

The Marriage Patterns That Get Harder Under Stress

Some marriage patterns get harder under stress. The patterns that get harder are the ones that were already a problem before the illness.

The pattern of one person making all the decisions. In a marriage where one person has always made the decisions, the illness makes the pattern worse. The well spouse makes all the decisions. The dying spouse is left out. The dying spouse feels like a child. The well spouse feels like a parent. The marriage is lost.

The pattern of one person doing all the talking. In a marriage where one person has always done the talking, the illness makes the pattern worse. The well spouse talks to the doctors, the family, the friends. The dying spouse is talked about, not talked to. The dying spouse feels invisible. The well spouse feels

responsible. The marriage is lost.

The pattern of one person being the "strong" one. In a marriage where one person has always been the "strong" one, the illness makes the pattern worse. The well spouse has to be strong. The well spouse cannot be weak. The well spouse cannot ask for help. The well spouse cannot cry. The dying spouse cannot be the one who is strong. The marriage becomes one-sided. The marriage is lost.

The pattern of avoidance. In a marriage where one or both have always avoided the hard conversations, the illness makes the pattern worse. The hard conversations are the ones that need to happen. The avoidance becomes a wall. The wall becomes the marriage. The marriage is lost.

The Marriage Patterns That Get Better

Some marriage patterns get better under stress. The patterns that get better are the ones that were already a foundation of the marriage.

The pattern of honesty. In a marriage where the couple has always been honest with each other, the illness makes the honesty deeper. The hard conversations happen. The truth is told. The marriage is more honest than it has ever been.

The pattern of partnership. In a marriage where the couple has always been partners, the illness makes the partnership stronger. The decisions are made together. The caregiving is shared. The marriage is more equal than it has ever been.

The pattern of presence. In a marriage where the couple has always been present with each other, the

illness makes the presence deeper. The touch is more intentional. The time together is more precious. The marriage is more present than it has ever been.

The pattern of forgiveness. In a marriage where the couple has always been quick to forgive, the illness makes the forgiveness deeper. The mistakes are forgiven. The resentment is released. The marriage is more forgiving than it has ever been.

The pattern of gratitude. In a marriage where the couple has always been grateful for each other, the illness makes the gratitude deeper. The thank yous are more frequent. The small acts are more noticed. The marriage is more grateful than it has ever been.

The patterns that get better are the patterns that were already there. The illness does not create the patterns. The illness reveals them.

When One of You Wants to Keep Fighting and the Other Wants to Stop

One of the most common sources of conflict in the long middle is the difference between the spouse who wants to keep fighting and the spouse who wants to stop. The "fighter" wants to try every treatment, every clinical trial, every alternative therapy. The "accepter" wants to focus on comfort, on quality of life, on the time they have.

The two positions are both real. The fighter is fighting because they love the dying spouse and don't want to lose them. The accepter is accepting because they love the dying spouse and want to honor their life.

The two positions can coexist, but they require conversation. The conversation has to be honest. The

conversation has to be respectful. The conversation has to happen before the crisis, not in the crisis.

Some questions for the conversation:

- "What are we fighting for? More time? A cure? A specific milestone?"
- "What does the fighting look like? What is the cost of the fighting?"
- "What does the accepting look like? What is the gift of the accepting?"
- "Where is the line for each of us?"
- "What would the dying spouse want, if they could choose?"

The conversation is not about who is right. The conversation is about what the marriage wants, together.

When One of You Wants to Plan and the Other Wants to Be Present

The other common source of conflict is the difference between the planner and the present-er. The planner wants to plan the funeral, the will, the finances, the legacy. The present-er wants to be in the moment, with the dying spouse, without thinking about what comes after.

The two positions are both real. The planner is planning because they love the dying spouse and want to take care of them. The present-er is being present because they love the dying spouse and want to be with them now.

The two positions can coexist, but they require balance. The planning has to happen, but not all the time. The present has to be honored, but not at the expense of the practical.

Some ways to balance:

- The planner plans for one hour a week. The present-er is present the rest of the time.
- The planner involves the present-er in the planning. The planning is a shared task.
- The present-er honors the planning as a form of love.
- The planner honors the present as a form of love.

The balance is the marriage. The balance is the work.

Case Study: Ann & Tom (Ages 67/70)

Ann was 67. Tom was 70. They had been married for 41 years. Ann was a retired nurse. Tom was a retired teacher. They had two adult daughters, both out of state.

Ann was diagnosed with Stage IV lung cancer. The treatment was palliative from the start. The doctor said "a year, maybe less." Ann chose to focus on quality of life, not aggressive treatment.

Tom wanted to fight. Tom wanted to try every clinical trial, every alternative therapy, every new medication. Ann did not. Ann wanted to spend the time she had with Tom, with their daughters, with their friends.

The first few months were hard. Tom and Ann fought about the treatment. Tom was angry. Ann was sad. They were both scared.

What changed was the conversation. They sat down one Saturday morning and talked for three hours. They cried. They laughed. They forgave each other for the anger. They made a plan together.

The plan was:

- Ann would try one more line of treatment, lightly.
- Tom would not push for more.
- They would plan the funeral together, with their daughters.
- They would spend the time they had together, in the marriage.
- They would ask for hospice when the time came.

What Ann and Tom got right:

- They had the hard conversation, even when it was hard.
- They made a plan together, not one of them.
- They let each other grieve in their own way.
- They let the hospice team in.
- They kept the marriage the marriage, not the illness.

Ann died 11 months after the diagnosis. The last 11 months were not easy. They were the most meaningful of their marriage. Tom says: "We were closer at the end than we had ever been. The illness was hard. The marriage was the marriage."

Scorecard: How Is the Marriage Holding Up?

| Question | Score (1-5) |

|---|---|

| We are talking about the hard things, not avoiding them | /5 |

| We are not letting the resentment build | /5 |

| We are not letting the guilt take over | /5 |

| We are making decisions together | /5 |

We are not one of us being the only "strong" one	/5
We are keeping the partnership alive	/5
We are honoring each other's way of coping	/5
We are asking for help when we need it	/5
We are not letting the illness become the whole marriage	/5
We are still in love, in the way we have always been	/5

Total: /50

Interpreting your score:

- 40-50: The marriage is holding up. The patterns are good.
- 30-39: Some patterns to fix. The conversations in this chapter can help.
- 20-29: The marriage is in trouble. Get help. A counselor, a chaplain, a hospice social worker.
- Under 20: The marriage is in crisis. Get help immediately.

Key Terms

- Caregiver Burnout: The exhaustion that comes from sustained caregiving
- Survivor's Guilt: The guilt the well spouse feels for surviving, for having a future, for wanting their life back
- Anticipatory Grief: The grief that happens before the death, as the death becomes more certain
- Hospice Social Worker: A professional who helps families navigate the conversations, the resources, and the emotional work

Reflection Questions for the Two of You

1. What is one thing you are most afraid of saying to each other?
2. What is one thing you are most afraid of hearing from each other?
3. Is there resentment in the marriage right now? If so, where is it coming from?
4. Is there guilt in the marriage right now? If so, where is it coming from?
5. What is one thing you can do this week to honor the marriage?
6. What is one thing you can forgive each other for, right now, even before asking?

Action for This Week: The Stress Inventory

Within the next 7 days, do these 5 things:

1. Identify the most stressful thing right now. Financial, time, decision, family, hope. Pick one. Talk about it.
2. Name the resentment, if there is one. "I am feeling resentful because..." The naming is the work.
3. Name the guilt, if there is one. "I am feeling guilty because..." The naming is the work.
4. Forgive each other for one thing. Even before the other person asks. The forgiveness is the marriage.
5. Make a date. Not a medical appointment. A date. The marriage needs dates. The marriage needs the ordinary.

The Stress Inventory is the work of the marriage in the long middle. The work is not the illness. The work is the marriage. The marriage is the work.

A Closing Word

The marriage stress test is the hardest thing. The marriage may pass. The marriage may not. The marriage is the only thing that will go through the test with you. The marriage is the only thing that will come out the other side with you.

The patterns that get harder under stress are the ones to watch. The patterns that get better are the ones to build. The work is to build the patterns that get better, and to address the patterns that get harder. The work is the marriage.

You are not failing. You are holding on.

Chapter 8: Money, Paperwork, and the Conversations You Avoid

The hardest conversations in the long middle are not the medical ones. The medical ones have a structure, a doctor, a plan. The hardest conversations are the financial ones, the legal ones, the "what happens after" ones. The conversations that no one wants to have. The conversations that the dying spouse does not want to have because they are not ready to face what comes after. The conversations that the well spouse does not want to have because having them makes the death real.

This chapter is about those conversations. The money conversations. The paperwork conversations. The "what do you want" conversations. The conversations about the house, the accounts, the possessions, the funeral, the will, the legacy. The conversations that, if they do not happen, will be a burden to the well spouse and the family for years after the death.

I am not going to pretend these conversations are easy. They are not. They are some of the hardest you will have. But they are some of the most important. The conversations you have now will save your family from having to guess. The conversations you have now will let the dying spouse have a say. The conversations you have now will let the well spouse grieve without paperwork.

This chapter will not make the conversations easy. It will make the conversations possible.

Chapter at a Glance

- The Financial Reality: What to Expect, What to Plan For
- The Paperwork No One Wants to Do: Will, Advance Directive, Healthcare Proxy
- The Conversations: What Happens to the House, the Accounts, the Possessions
- The Dying Spouse's Role in Financial Planning
- The Well Spouse's Role in Financial Planning
- The Adult Children's Role (If Any)
- Long-Term Care Insurance, Life Insurance, Retirement Accounts
- The "Elephant in the Room" Conversations
- The 6 Documents to Have in Place by Week 4
- Case Study: Robert & Helen (ages 65/63)
- Scorecard: Are We Financially and Legally Prepared?
- Key Terms
- Reflection Questions
- Action for This Week: The Paperwork Sprint

The Financial Reality: What to Expect, What to Plan For

The financial reality of terminal illness is hard. The medical bills are real. The caregiving costs are real. The lost income is real. The funeral costs are real. The financial reality is one of the most stressful parts of the long middle, and one of the most common sources of conflict.

The costs that are likely to come up:

Medical costs not covered by insurance. Even with good insurance, there are co-pays, deductibles, out-of-network charges, and non-covered services. These can add up quickly. The well spouse should track every medical bill, every insurance statement, every Explanation of Benefits (EOB). The tracking is not just for the money. The tracking is for the peace of mind.

Medications. Some medications are expensive. Some are not covered by insurance. The dying spouse may need medications for pain, for nausea, for sleep, for mood. The medications can cost hundreds or thousands of dollars a month.

Caregiving. If the well spouse cannot do all the caregiving, they will need help. The help can come from family (free), from hired caregivers (\$20-30/hour), or from a facility (\$5,000-10,000/month). The well spouse should know the cost before they need the help.

Lost income. If the well spouse has to cut back on work, or stop working, the income loss is real. The income loss is one of the most stressful parts of the long middle for many couples. The well spouse should know what they can afford to lose, and for how long.

Funeral costs. The funeral, the burial, the cremation, the reception, the headstone. The cost of dying is real. The cost is often \$7,000-15,000. The cost is one of the things the dying spouse often wants to plan, so that the well spouse does not have to.

The aftermath. The well spouse will have to live on less. The retirement accounts, the life insurance, the Social Security. The well spouse should know what is coming, so they can plan.

The financial reality is not a punishment. The financial reality is the situation. The situation can be planned for. The planning is one of the most loving things the couple can do for each other.

The Paperwork No One Wants to Do: Will, Advance Directive, Healthcare Proxy

The paperwork is hard. The paperwork is also necessary. The paperwork protects the dying spouse, the well spouse, the family. The paperwork is one of the last gifts the dying spouse can give.

The Will. The will says who gets what when the dying spouse dies. The will protects the surviving spouse, the children, the grandchildren, the charities. The will should be in place before the illness progresses. The will should be reviewed by a lawyer, especially if the estate is complex.

If there is no will, the state will decide. The state's decision is not always what the family would have wanted. The will is the way to make sure the wishes are honored.

The Advance Directive. The advance directive is a legal document that says what medical treatment the dying spouse wants if they cannot speak for themselves. The advance directive is sometimes called a "living will." The advance directive covers things like life support, resuscitation, artificial nutrition, artificial hydration. The advance directive is the dying spouse's voice when they cannot speak.

The advance directive should be in place before the illness progresses. The advance directive should be discussed with the doctor, with the well spouse, with

the family. The advance directive is a gift to the well spouse, because it takes the burden of guessing off the well spouse.

The Healthcare Proxy. The healthcare proxy is a person who is designated to make medical decisions for the dying spouse if the dying spouse cannot. The healthcare proxy is usually the well spouse. If not, the healthcare proxy should be someone who knows the dying spouse's wishes, and who can be present in the medical decisions.

The healthcare proxy should be in writing. The healthcare proxy should be discussed with the doctor, with the well spouse, with the family. The healthcare proxy is a gift to the well spouse, because it takes the burden of the medical decisions off the family.

The Power of Attorney. The power of attorney is a legal document that allows one person to make financial or legal decisions for another. The power of attorney is important if the dying spouse cannot manage their own affairs. The power of attorney can be for the well spouse, or for a trusted family member, or for an attorney.

The power of attorney should be in place before the illness progresses. The power of attorney should be reviewed by a lawyer. The power of attorney is a gift to the well spouse, because it allows them to manage the finances without going to court.

The Living Trust. A living trust is a legal document that holds the assets for the benefit of the dying spouse during their life, and then transfers the assets to the beneficiaries after death. The living trust can avoid probate, can provide privacy, can make the transition easier. The living trust is more complex than a will, and should be set up with a lawyer.

The Beneficiary Designations. The beneficiary designations on life insurance, retirement accounts, and bank accounts are the instructions for who gets the money when the dying spouse dies. The beneficiary designations override the will. The designations should be checked and updated.

The paperwork is not fun. The paperwork is a gift. The paperwork is the dying spouse's way of taking care of the well spouse and the family, even after they are gone.

The Conversations: What Happens to the House, the Accounts, the Possessions

The conversations about what happens after are the hardest conversations. The conversations are not just about the money. The conversations are about the things. The house, the car, the furniture, the jewelry, the photos, the small things that have been part of the marriage for years.

The conversations about the things are emotional. The things carry memories. The things are part of the marriage. The things are not just the things.

The conversations should happen in stages:

Stage 1: The Big Picture. "What do you want to happen to the house. The accounts. The car. The retirement." The big picture is the structure. The big picture is what the lawyer will use to write the will.

Stage 2: The Specifics. "Do you want your wedding ring to go to Anna. Do you want the photo albums to go to Michael. Do you want the china to go to Sarah." The specifics are the things the will does not cover. The specifics are the things the family will want to

know.

Stage 3: The Small Things. "What about the books. The tools. The clothes. The garden." The small things are the things the dying spouse may not have thought about. The small things are the things the well spouse will have to deal with. The small things are easier if the dying spouse has thought about them.

Stage 4: The "What I Want You to Do" Conversation. "I want you to keep the house. I want you to travel. I want you to find someone, when you are ready. I want you to be happy." This conversation is not about the things. This conversation is about the well spouse's life after. The dying spouse gets to bless the well spouse's life. The blessing is a gift.

The four stages can happen over weeks or months. The conversations do not have to be perfect. The conversations just have to happen.

The Dying Spouse's Role in Financial Planning

The dying spouse has a role. The role is not just to be informed. The role is to be the decision maker, as much as possible, for as long as possible.

The dying spouse's role includes:

- Making the decisions about the will, the advance directive, the healthcare proxy
- Deciding who gets the specific things
- Deciding what kind of funeral, what kind of burial, what kind of memorial
- Blessing the well spouse's life after
- Communicating the decisions to the family

- Updating the documents as the situation changes

The dying spouse who is still making decisions is still in the marriage. The dying spouse who has given up the decisions is not. The role is not about the money. The role is about the agency. The agency is part of the marriage.

The Well Spouse's Role in Financial Planning

The well spouse has a role. The role is not to take over everything. The role is to support the dying spouse in making the decisions, and to be ready to take over when the dying spouse cannot.

The well spouse's role includes:

- Helping the dying spouse think through the decisions
- Asking the questions the dying spouse may not have thought of
- Being the executor of the will, the trustee of the trust, the holder of the power of attorney
- Managing the day-to-day finances
- Communicating with the lawyer, the accountant, the financial planner
- Being ready to take over when the time comes

The well spouse who is supporting the dying spouse is in the marriage. The well spouse who is taking over without the dying spouse's input is not. The role is the marriage.

The Adult Children's Role (If Any)

The adult children have a role, but the role is to support, not to take over. The role of the adult children is to:

- Listen to the dying spouse's wishes
- Help with the practical matters (research, phone calls, errands)
- Be present for the conversations
- Not push the dying spouse to make decisions they are not ready to make
- Be ready to support the well spouse after the death

The adult children who are supporting are in the family. The adult children who are taking over are not. The role is the family.

The well spouse should protect the dying spouse from the adult children who want to take over. The dying spouse has the right to make their own decisions. The dying spouse has the right to be the dying spouse, not the child being managed by their children.

Long-Term Care Insurance, Life Insurance, Retirement Accounts

The long middle is a good time to review the financial instruments. The instruments are the things that will support the well spouse and the family after the death.

Long-term care insurance. If the dying spouse has long-term care insurance, this is the time to use it. The insurance can pay for in-home care, for facility care, for hospice. The insurance is a gift.

Life insurance. If the dying spouse has life insurance, the well spouse should know the policy, the

beneficiary, the amount. The life insurance is a gift to the well spouse. The life insurance is one of the things the dying spouse planned for, often years ago, for the well spouse's life after.

Retirement accounts. The retirement accounts (401k, IRA, etc.) have beneficiary designations. The designations should be checked. The designations should be updated if needed. The accounts are part of the estate.

Bank accounts. The bank accounts should have beneficiary designations, or should be in joint names, or should be transferable on death. The accounts are part of the estate.

The house. The house is usually the biggest asset. The house should be in joint names, or should be in a trust, or should be part of the estate. The house is part of the legacy.

The financial review is not a one-time event. The financial review is a process. The process happens over weeks. The process is one of the most loving things the couple can do for each other.

The "Elephant in the Room" Conversations

Some conversations are the elephant in the room. The conversations are not about the money. The conversations are about the things no one wants to talk about. The things that are uncomfortable. The things that are scary. The things that are part of dying.

The elephant conversations include:

- "I want you to remarry, if you find someone." (or "I don't want you to remarry.")

- "I want to be cremated." (or "I want to be buried.")
- "I want you to keep the house." (or "I want you to sell the house.")
- "I want the funeral to be small." (or "I want the funeral to be big.")
- "I want to die at home." (or "I want to die in the hospital.")
- "I want to be on life support if it gives us more time." (or "I don't want to be on life support.")
- "I want to know if you are afraid." (or "I don't want to talk about being afraid.")
- "I want to say I'm sorry." (or "I want to hear you say you're sorry.")
- "I want to say I love you one more time." (or "I want to hear you say it one more time.")
- "I want to say goodbye." (or "I want to hear you say it back.")

The elephant conversations are not optional. The elephant conversations are the conversations that the dying spouse needs to have, and the well spouse needs to hear. The elephant conversations are the conversations that, if they do not happen, will be a regret for the well spouse for the rest of their life.

The way to have the elephant conversations:

- Pick a quiet moment. Not in a crisis. Not at the hospital. Not in the middle of the night.
- Use an opening line. "I want to talk about something hard. Can we do that."
- Be specific. Not "I want to talk about the funeral." "I want to talk about what kind of funeral you want, and where you want to be buried."

- Be honest. The dying spouse has the right to be honest. The well spouse has the right to be honest.
- Be brief. The conversation does not have to be a long one. The conversation is the marriage.
- Be ready for tears. The tears are part of the conversation. The tears are not weakness. The tears are the love.

The 6 Documents to Have in Place by Week 4

By the end of week 4, six documents should be in place:

1. The Will. Reviewed by a lawyer, signed, witnessed, stored in a safe place.
2. The Advance Directive. Discussed with the doctor, signed, witnessed, given to the doctor and the well spouse.
3. The Healthcare Proxy. Named in writing, discussed with the doctor, given to the doctor and the well spouse.
4. The Power of Attorney. Reviewed by a lawyer, signed, witnessed, given to the lawyer and the well spouse.
5. The Beneficiary Designations. Checked on all accounts, updated if needed, recorded in a safe place.
6. The Conversations. At least one "elephant" conversation with the well spouse. The conversations are not in writing. The conversations are in the heart.

If these six documents are in place by week 4, the financial and legal foundation is set. The foundation is not perfect. The foundation is enough. The foundation

allows the rest of the long middle to happen.

Case Study: Robert & Helen (Ages 65/63)

Robert and Helen (from Chapter 5) had a hard year of treatment for Helen's Stage IV ovarian cancer. The treatment was expensive. The caregiving was expensive. The lost income was real.

What Robert and Helen got right:

- They had a will in place before the illness progressed. The will had been updated when they retired.
- They had an advance directive in place. Helen had been a nurse; she knew the importance of the advance directive.
- They had the "elephant" conversation about the house. Helen wanted Robert to keep the house. Robert did not want to keep the house. They compromised: Robert would keep the house for two years, then sell it if he wanted to.
- They had the conversation about the funeral. Helen wanted to be cremated. She wanted a small memorial, with the family, in the garden.
- They had the conversation about the future. Helen told Robert she wanted him to be happy, whatever that looked like. Robert told Helen he would try.

What Robert and Helen got wrong:

- They did not have a power of attorney in place. When Helen could no longer manage her own affairs, Robert had to go to court to get guardianship. The court process took six weeks and cost thousands.

- They did not check the beneficiary designations on the retirement accounts. Robert discovered, after Helen's death, that one of the accounts still had Helen's mother as the beneficiary. Helen's mother had died 10 years earlier. The account went into probate.
- They did not have a financial planner. They managed the money themselves, which was fine for most of the marriage, but became a burden in the long middle.

Helen died 14 months after the diagnosis. The will, the advance directive, the elephant conversations all made the aftermath easier. The power of attorney and the beneficiary designations made the aftermath harder. The lesson is: do the paperwork early, and do it completely.

Scorecard: Are We Financially and Legally Prepared?

| Question | Score (1-5) |

|---|---|

| We have a will in place | /5 |

| We have an advance directive in place | /5 |

| We have a healthcare proxy in place | /5 |

| We have a power of attorney in place | /5 |

| We have checked the beneficiary designations | /5 |

| We have had at least one "elephant" conversation | /5 |

| We have a plan for the funeral / memorial | /5 |

| We have a plan for the financial aftermath | /5 |

| The dying spouse is still making the financial decisions | /5 |

| The well spouse knows where all the documents are | /5 |

Total: /50

Interpreting your score:

- 40-50: You are financially and legally prepared. The paperwork is in place.
- 30-39: Some documents missing. The Paperwork Sprint (below) can help.
- 20-29: Many documents missing. Get help from a lawyer, a financial planner, a hospice social worker.
- Under 20: The paperwork is in trouble. Get help immediately.

Key Terms

- Will: A legal document that says who gets what when you die
- Advance Directive (Living Will): A legal document that says what medical treatment you want if you cannot speak for yourself
- Healthcare Proxy: A person designated to make medical decisions for you if you cannot
- Power of Attorney: A legal document that allows one person to make financial or legal decisions for another
- Beneficiary Designation: The instruction on a financial account that says who gets the money when you die
- Estate: All the things you own when you die

Reflection Questions for the Two of You

1. What is one document you have been putting off?
2. What is one "elephant" conversation you have been avoiding?
3. What is one thing the dying spouse wants the well spouse to know about the finances?
4. What is one thing the dying spouse wants to bless in the well spouse's life?
5. What is one thing the well spouse is most afraid of asking the dying spouse about?
6. If the dying spouse could give the well spouse one piece of advice for the life after, what would it be?

Action for This Week: The Paperwork Sprint

Within the next 7 days, do these 6 things:

1. Find the current will. Read it. If there is no will, schedule a meeting with a lawyer.
2. Find or create the advance directive. If you need a form, your doctor or a hospice social worker can give you one.
3. Name the healthcare proxy in writing. Tell the doctor. Tell the well spouse. Tell the family.
4. Check the beneficiary designations on all accounts. Make a list. Update if needed.
5. Have one "elephant" conversation. Pick one topic. Have the conversation. The conversation is the gift.

6. Make a "where the documents are" list. Put it in a safe place. Give a copy to the well spouse.

The Paperwork Sprint is the work of the legal and financial foundation. The foundation is not fun. The foundation is the gift. The gift is the well spouse's life after.

A Closing Word

The money, the paperwork, the conversations you avoid — these are some of the hardest things in the long middle. They are also some of the most important. The dying spouse gets to bless the well spouse's life after. The well spouse gets to honor the dying spouse's wishes. The family gets to know that the dying spouse had a say. The marriage gets to end with the love still in charge.

The paperwork is not the marriage. The paperwork is what allows the marriage to be the marriage. The paperwork is the love, made practical.

You are not failing. You are taking care of each other.

Chapter 9: The Acceptance Conversation

There comes a moment in every terminal illness when the focus shifts. The shift is from fighting the illness to living with the illness. The shift is from treatment to comfort. The shift is from "we are going to beat this" to "we are going to use the time we have."

The shift is not a moment in time. It is a process. The process takes days or weeks. The process involves the dying spouse, the well spouse, the doctor, the family. The process is one of the most important things that will happen in this season. The shift is the acceptance. The acceptance is the doorway to the quiet.

This chapter is about the shift. The shift is hard. The shift is also necessary. The shift is not giving up. The shift is not losing hope. The shift is changing what the hope is for. The hope is no longer for a cure. The hope is for the time that is left. The hope is for the comfort. The hope is for the marriage at the end.

The acceptance is a grief. The acceptance is also a relief. The grief and the relief are both real. The grief and the relief are both allowed.

Chapter at a Glance

- The Shift from "Fighting the Illness" to "Living with the Illness"
- The Shift from "Treatment" to "Comfort"
- How to Know When the Time Has Come
- The Conversation Between the Two of You
- The Conversation with the Doctor
- The Conversation with the Family

- The Grief of Acceptance (It's a Real Grief)
- The Relief of Acceptance (Also Real)
- Case Study: Ann & Tom (ages 67/70)
- Scorecard: Are We Ready for the Next Phase?
- Key Terms
- Reflection Questions
- Action for This Week: The Acceptance Conversation

The Shift from "Fighting the Illness" to "Living with the Illness"

The first phase of the long middle is fighting. The fighting is the treatment. The fighting is the hope for a cure, or a remission, or a longer life. The fighting is the energy of the first months. The fighting is what gets you through the appointments, the medications, the side effects.

The fighting is real. The fighting is necessary. The fighting is also not the whole story. The fighting has a shelf life. The fighting cannot last forever. At some point, the body stops responding to the treatment. At some point, the treatment starts to do more harm than good. At some point, the well spouse and the dying spouse both know that the fighting is no longer working.

The shift from fighting to living is not a moment of giving up. The shift is a moment of choosing differently. The shift is the moment when the dying spouse says, "I want to stop the treatment. I want to enjoy the time I have." The shift is the moment when the well spouse hears this, and instead of fighting it, says, "I will help you enjoy the time you have."

The shift is not a failure. The shift is a different kind of courage. The courage to stop fighting. The courage to live with the illness. The courage to make the most of the time that is left.

The shift is also not a sudden moment. The shift is a slow process. The shift happens over days or weeks. The shift involves small signs — the treatment side effects getting worse, the symptoms getting worse, the body getting weaker, the energy getting lower. The shift involves small conversations — "I'm tired of the treatment." "I want to feel better." "I want to do the things I want to do." The shift is the accumulation of the signs and the conversations.

The shift is one of the most important moments in this season. The shift is the moment the marriage changes from fighting the illness to living with the illness. The shift is the moment the dying spouse takes back their agency. The shift is the moment the marriage becomes about the time that is left.

The Shift from "Treatment" to "Comfort"

The shift from treatment to comfort is related to the shift from fighting to living. The two shifts are not exactly the same. The shift from treatment to comfort is the medical shift. The shift from fighting to living is the emotional shift. The two shifts usually happen together, but they are not always exactly synchronized.

The shift from treatment to comfort means:

- Stopping treatments that are not working
- Starting treatments that focus on comfort (pain management, symptom control)

- Stopping appointments that are not necessary
- Starting appointments that focus on comfort (palliative care, hospice)
- Letting the dying spouse eat what they want, sleep when they want, do what they want
- Letting the well spouse be the spouse, not the project manager

The shift from treatment to comfort does not mean giving up. The shift from treatment to comfort means changing the goal. The goal is no longer to extend life at all costs. The goal is to make the life that is left as good as possible.

The shift from treatment to comfort is also not a medical decision alone. The shift is a marriage decision. The shift is a family decision. The shift involves the dying spouse, the well spouse, the doctor, and the family. The shift is one of the most important decisions the couple will make together.

How to Know When the Time Has Come

The shift from treatment to comfort does not have a precise moment. The shift has signs. The signs are different for every couple, but the signs are usually clear in hindsight.

Sign 1: The treatment is no longer working. The scans show progression. The tumor markers are rising. The symptoms are getting worse. The doctor says "the treatment is not working." The treatment is no longer extending life. The treatment is now causing more harm than good.

Sign 2: The dying spouse is tired of the treatment. The dying spouse says "I'm done." The dying spouse stops

going to the appointments. The dying spouse stops taking the medications. The dying spouse says "I want to feel better, not worse." The treatment has become a burden, not a hope.

Sign 3: The well spouse is tired of the treatment. The well spouse is exhausted. The well spouse is doing all the caregiving, all the appointments, all the medications, all the emotions. The well spouse says "I can't do this anymore." The well spouse is not the dying spouse, but the well spouse is also suffering.

Sign 4: The focus is shifting. The conversations are about "what I want to do" instead of "what the treatment will do." The dying spouse is talking about the trip they want to take, the people they want to see, the things they want to say. The focus has shifted from the disease to the life.

Sign 5: The doctor is suggesting it. The doctor says "we should think about hospice." The doctor says "the treatment is not helping anymore." The doctor says "we should focus on comfort." The doctor has seen this before. The doctor knows the signs.

The signs are not always clear in the moment. The signs are usually clearer in hindsight. The shift is usually a slow process, not a moment. The process is the work.

The Conversation Between the Two of You

The conversation about the shift is the hardest conversation. The conversation is not about the treatment. The conversation is about the life. The conversation is about what the dying spouse wants, and what the well spouse is willing to give, and what the marriage will be in the time that is left.

The conversation should happen in a quiet moment. Not in the hospital. Not in the middle of a crisis. Not in the middle of the night. A Saturday afternoon. A morning with coffee. A walk in the garden. The conversation should be in a place that is safe, and in a time that is not rushed.

The conversation should be initiated by the dying spouse, if possible. The dying spouse knows when they are ready. The dying spouse is the one who has the right to make this decision. The well spouse can ask, but the dying spouse should be the one to say it.

The conversation can include:

- "I am tired of the treatment."
- "I want to focus on the time I have."
- "I want to do the things I want to do."
- "I want to be comfortable."
- "I want to spend the time with you, not in the hospital."
- "I want to talk about what comes next."
- "I want to plan the funeral."
- "I want to say goodbye to the people I love."
- "I want to live the time I have."

The conversation is not the end. The conversation is the beginning. The conversation is the shift. The conversation is the marriage, taking back the agency.

The Conversation with the Doctor

The conversation with the doctor is the medical version of the conversation. The conversation is about the treatment, the prognosis, the options. The conversation is about the shift from treatment to comfort.

The conversation with the doctor should happen after the conversation between the two of you. The well spouse should be there, if the dying spouse wants them there. The conversation should be honest, and the doctor should be asked to be honest.

Some questions for the doctor:

- "Is the treatment still working?"
- "What would happen if we stopped the treatment?"
- "What would you recommend?"
- "What does hospice offer?"
- "What would the last weeks look like if we stopped the treatment?"
- "What would the last weeks look like if we continued?"
- "What is the prognosis now, in ranges?"

The doctor is not the enemy. The doctor is the guide. The doctor has seen this before. The doctor can help the couple understand the options. The doctor can also be the one to say "it is time" if the couple is not ready to say it themselves.

The Conversation with the Family

The conversation with the family is the social version of the conversation. The conversation is about the shift, and about what the family can do to support it.

The conversation with the family should happen after the conversation between the two of you, and ideally after the conversation with the doctor. The family needs to know what is happening, so they can support the shift, not fight it.

Some opening lines for the family conversation:

- "We have made a decision about the treatment."
- "We are going to stop the aggressive treatment."
- "We are going to focus on comfort, on quality of life."
- "We have talked to the doctor. The doctor agrees."
- "We want to spend the time we have together, with the family."
- "We need your support for this decision."

The family may not agree. The family may want to keep fighting. The family may want to try another clinical trial. The family may have their own grief. The well spouse has to protect the dying spouse from the family's grief. The dying spouse has the right to make this decision. The family has the right to grieve. The two are different.

The Grief of Acceptance (It's a Real Grief)

The acceptance is a grief. The grief is real. The grief is allowed.

The grief is for the life the dying spouse thought they would have. The grief is for the future that is no longer possible. The grief is for the marriage as it was. The grief is for the things they will not do. The grief is for the people they will not see grow up. The grief is for the trips they will not take. The grief is for the books they will not finish. The grief is for the conversations they will not have. The grief is for the years they will not have.

The grief is not depression. The grief is the love, mourning the future. The grief is the natural response to a real loss. The grief is part of the acceptance. The

grief is not something to be fixed. The grief is something to be felt.

The way to be with the grief:

- Acknowledge it. "I am grieving."
- Name it. "I am grieving the future I thought I would have."
- Share it. "I want to talk about it."
- Honor it. "I want to feel this, not run from it."
- Hold it. "I want to sit with this for a while."

The grief is not the end. The grief is part of the acceptance. The acceptance is the doorway to the quiet. The quiet is the next phase.

The Relief of Acceptance (Also Real)

The acceptance is also a relief. The relief is real. The relief is allowed.

The relief is for the end of the fighting. The relief is for the end of the appointments. The relief is for the end of the side effects. The relief is for the end of the hope that has been a burden. The relief is for the freedom to live the time that is left. The relief is for the return to the marriage, away from the treatment.

The relief is not joy. The relief is the lifting of a weight. The relief is the space that is made when the fighting is put down. The relief is the time and the energy that is freed up for the things that matter.

The well spouse may also feel the relief. The well spouse has been carrying the fighting too. The well spouse has been driving to the appointments, managing the medications, managing the emotions, managing the family. The well spouse is also tired. The well spouse may also feel the relief.

The relief and the grief can coexist. They often do. The relief is for the end of the fighting. The grief is for the life that is lost. The two are not in conflict. The two are part of the acceptance.

Case Study: Ann & Tom (Ages 67/70)

Ann and Tom (from Chapter 7) faced the shift in month 6 of Ann's illness. The treatment was no longer working. The scans showed progression. The doctor said "the treatment is not helping." Ann was tired. Tom was tired.

The conversation:

- Ann brought it up first. One evening, after dinner, she said "I'm done with the chemo. I want to feel better." Tom was quiet for a long time. Then he said "I hear you. I want you to feel better. Let's talk to the doctor."
- They talked to the doctor. The doctor agreed. The doctor said "hospice is the right next step."
- They told the daughters. The daughters were sad, but they supported the decision. The daughters said "we want you to be comfortable, mom."
- Ann and Tom called hospice. The hospice team came to the house. The hospice team talked to Ann and Tom about what hospice offered — pain management, support, equipment, respite care.
- Ann stopped the chemo. Ann started the hospice. Ann felt better within a week. The nausea went away. The fatigue got better. Ann was able to do the things she wanted to do.

The grief:

- Ann grieved the future. She cried about the trip they were not going to take. She cried about the grandchildren she was not going to see grow up. Tom cried with her. They sat on the couch and cried together.

The relief:

- Ann felt the relief. "I feel like myself again." Tom felt the relief. "I have my wife back."
- They started doing the things they wanted to do. They had a small dinner party for the close friends. They took a short trip to the coast. They sat in the garden.

Ann died 5 months after the shift. The last 5 months were the best of the long middle. The shift made the last months livable. The shift made the marriage the marriage.

Scorecard: Are We Ready for the Next Phase?

| Question | Score (1-5) |

|---|---|

| The treatment is no longer working | /5 |

| The dying spouse is tired of the treatment | /5 |

| We have had the conversation between the two of us
| /5 |

| We have talked to the doctor about the shift | /5 |

| We have talked to the family about the shift | /5 |

| The dying spouse is making this decision, not the family | /5 |

| We have allowed the grief of acceptance | /5 |

| We have allowed the relief of acceptance | /5 |

| We have looked into hospice | /5 |

| We are ready to live the time we have | /5 |

Total: /50

Interpreting your score:

- 40-50: You are ready for the next phase. The shift is happening.
- 30-39: Some parts of the shift are happening. The conversations in this chapter can help.
- 20-29: The shift is in process. Get help from a counselor, a chaplain, a hospice social worker.
- Under 20: The shift is in trouble. The fighting is taking over. Get help.

Key Terms

- Acceptance: The shift from fighting the illness to living with the illness
- Palliative Care: Medical care for comfort and quality of life, not cure
- Hospice: Palliative care specifically for the end of life, usually the last 6 months
- Anticipatory Grief: The grief that happens before the death, as the death becomes more certain
- Healthcare Proxy: The person designated to make medical decisions when the dying spouse cannot

Reflection Questions for the Two of You

1. Is the treatment still working? In your honest assessment.
2. Is the dying spouse tired of the treatment? In their own words.

3. Have you had the conversation about the shift? If not, what is keeping you from it?
4. What is one thing the dying spouse wants to do in the time they have?
5. What is one thing the well spouse needs to let go of, to make room for the time?
6. What would living, instead of fighting, look like for the two of you?

Action for This Week: The Acceptance Conversation

Within the next 7 days, do these 5 things:

1. Have the conversation between the two of you. Pick a quiet moment. Use an opening line. "I want to talk about the treatment. I want to talk about what comes next."
2. Ask the doctor the hard questions. "Is the treatment working? What would happen if we stopped? What would you recommend?"
3. Look into hospice. Call a local hospice. Ask for an informational visit. The visit is not a commitment. The visit is information.
4. Tell the family, when you are ready. The family will have their own grief. The family also needs to know.
5. Make a list of the things the dying spouse wants to do. Not a bucket list. A list of small, meaningful, possible things. The list is the shift. The list is the life.

The Acceptance Conversation is the work of the shift. The work is not the illness. The work is the marriage. The marriage is the work.

A Closing Word

The acceptance is a grief. The acceptance is a relief. The acceptance is also a gift. The gift is the time that is left. The gift is the freedom to live the time. The gift is the marriage, in the time.

The fighting was necessary. The fighting is also not the whole story. The acceptance is the next chapter. The acceptance is the doorway to the quiet. The quiet is the time of the last months, the time of the last weeks, the time of the last days. The quiet is not empty. The quiet is full of the marriage, in the time that is left.

You are not failing. You are accepting.

Chapter 10: Hospice and the Gift of Time

The word "hospice" scares people. The fear is reasonable. The fear is the fear of giving up. The fear is the fear of the end. The fear is the fear of the unknown. The fear is the fear of being alone. The fear is the fear of dying.

This chapter is about hospice. Hospice is not giving up. Hospice is not the end. Hospice is not the unknown. Hospice is a gift. The gift is time. The gift is comfort. The gift is support. The gift is the marriage, in the time that is left.

I have watched many couples go through hospice. The couples who went into hospice with fear, and then found relief. The couples who went into hospice thinking they would be alone, and then found a team. The couples who went into hospice thinking they would lose the marriage, and then found the marriage stronger.

This chapter is not a sales pitch for hospice. This chapter is information. The information is: hospice is one of the best things that can happen in this season. The information is: hospice is not giving up. The information is: hospice is the gift of time.

Chapter at a Glance

- What Hospice Is (And What It Isn't)
- The Four Kinds of Hospice Care: Home, Facility, Hospital, Residential
- How to Choose a Hospice
- The Hospice Team: Who Comes to the House, When, For What

- The Dying Spouse's Role in Hospice
- The Well Spouse's Role in Hospice
- The Family and Friends' Role in Hospice
- The Myth of "Giving Up" — What Hospice Really Is
- The Financial Reality of Hospice
- The Gifts of Hospice: Pain Control, Support, Time
- Case Study: Ann & Tom (ages 67/70)
- Scorecard: Is Hospice Right for Us Now?
- Key Terms
- Reflection Questions
- Action for This Week: The Hospice Conversation

What Hospice Is (And What It Isn't)

Hospice is a philosophy of care. The philosophy is: when the illness can no longer be cured, the care should focus on comfort, on quality of life, on the time that is left. The philosophy is not about dying. The philosophy is about living, in the time that is left.

Hospice is also a service. The service provides:

- Pain management (medications, equipment, techniques)
- Symptom control (nausea, fatigue, breathlessness, anxiety)
- Medical equipment (hospital bed, wheelchair, oxygen, commode)
- Nursing care (visits from a nurse, usually 1-3 times a week)
- Aide services (help with bathing, dressing, personal care)

- Social work (help with the emotional, the family, the resources)
- Spiritual support (chaplain, rabbi, imam, priest, or the spiritual care of your choice)
- Volunteer support (a trained volunteer who can sit with the dying spouse, run errands, give the well spouse a break)
- Bereavement support (support for the family after the death, for up to 13 months)

Hospice is not:

- Giving up
- A place to die
- Hastening death
- A substitute for the family
- A loss of the marriage
- A loss of the dying spouse's agency
- A loss of hope

Hospice is:

- A philosophy of comfort
- A service that comes to you
- A team that supports the family
- A way to make the time that is left the best it can be
- A way to keep the marriage the marriage

The Four Kinds of Hospice Care: Home, Facility, Hospital, Residential

Hospice care comes in four main forms. The form depends on the needs of the dying spouse, the resources of the family, and the preferences of the couple.

1. Home Hospice. Home hospice is the most common form. The hospice team comes to the house. The well spouse is the primary caregiver. The hospice team provides the support — the nurse, the aide, the social worker, the chaplain, the volunteer. The well spouse has the team to call, 24/7, for any question or concern.

Home hospice works well when:

- The dying spouse wants to be at home
- The home is set up for the care (bed, equipment, accessibility)
- The well spouse is willing and able to be the primary caregiver
- There is a network of family and friends to help
- The dying spouse's symptoms can be managed at home

Home hospice does not work well when:

- The dying spouse's symptoms are too complex to be managed at home
- The well spouse cannot be the primary caregiver (work, health, distance)
- The home is not set up for the care
- The dying spouse does not want to be at home

2. Facility Hospice. Facility hospice is hospice care in a nursing home or assisted living facility. The facility provides the caregiving. The hospice team comes to the facility. The dying spouse is in a care environment, with 24-hour staff.

Facility hospice works well when:

- The dying spouse's symptoms are too complex to be managed at home
- The well spouse cannot be the primary caregiver
- The dying spouse is already in a facility

- The home is not set up for the care

3. Hospital Hospice. Hospital hospice is hospice care in a hospital. Hospital hospice is usually for short-term stays, for symptom management, for respite, or for the final days. The hospice team coordinates with the hospital staff.

Hospital hospice works well when:

- The dying spouse needs complex symptom management
- The dying spouse needs a short-term stay for a specific reason
- The family needs a respite from caregiving

4. Residential Hospice. Residential hospice is a facility that is specifically designed for hospice care. The facility has a home-like environment, with private rooms, family areas, and 24-hour staff. Residential hospice is often for the final weeks or days.

Residential hospice works well when:

- The dying spouse cannot be at home
- The family cannot provide the care at home
- The dying spouse wants a home-like environment
- The family needs the support of the residential hospice team

The form of hospice is a decision the couple makes together, with the doctor, with the hospice team. The decision can change as the situation changes. The couple is not locked in. The couple can move from home to facility, from home to residential, from facility to home. The hospice is flexible.

How to Choose a Hospice

Choosing a hospice is one of the most important decisions the couple will make. The hospice will be the team that supports the marriage in the last months. The hospice should be a good fit.

Some questions to ask when choosing a hospice:

- "How long have you been in operation?"
- "Are you Medicare-certified?" (Most insurance, including Medicare and Medicaid, covers hospice. The hospice should be certified.)
- "What services are included?"
- "How often will the nurse visit?"
- "How often will the aide visit?"
- "Who is on call, 24/7?"
- "What is your response time if we call in the middle of the night?"
- "What happens if the symptoms get worse?"
- "Can we keep our current doctor?"
- "What is the bereavement support?"
- "Do you have a volunteer program?"
- "What does the spiritual support look like?"
- "Can you provide the equipment we need?"

Some questions to ask of yourself and the dying spouse:

- "Did we feel comfortable with the team?"
- "Did the team listen to us?"
- "Did the team respect the dying spouse's wishes?"
- "Did the team answer our questions?"
- "Did the team give us hope, or did they take it away?"

The right hospice is the one that fits the couple. The couple should interview more than one hospice, if possible. The interview is information. The information is the right decision.

The Hospice Team: Who Comes to the House, When, For What

The hospice team is a group of professionals who support the couple. The team is usually:

The Nurse. The nurse is the medical leader of the team. The nurse visits 1-3 times a week, depending on the needs. The nurse manages the pain, the symptoms, the medications. The nurse is the one to call if there is a medical concern. The nurse is also the one to call if the well spouse is exhausted. The nurse is the most important relationship in the hospice team.

The Aide. The aide is the person who helps with the personal care — bathing, dressing, oral care, skin care. The aide visits 2-5 times a week, depending on the needs. The aide is often the most present person in the house, after the well spouse.

The Social Worker. The social worker is the person who helps with the emotional, the family, the resources. The social worker visits 1-2 times a month, depending on the needs. The social worker is the one to call if there is a family conflict, a financial concern, a resource need.

The Chaplain (or Spiritual Care). The chaplain is the person who supports the spiritual. The chaplain visits as needed. The chaplain is the one to call if the dying spouse wants to talk about the meaning, the purpose, the afterlife, the regrets. The chaplain is not there to convert. The chaplain is there to listen.

The Volunteer. The volunteer is a trained person who can sit with the dying spouse, run errands, give the well spouse a break. The volunteer visits as needed. The volunteer is one of the most underappreciated members of the team.

The Bereavement Coordinator. The bereavement coordinator is the person who supports the family after the death. The bereavement coordinator is the one who will call in the weeks and months after, who will send the sympathy card, who will offer the support group.

The hospice team is the gift. The team is not a substitute for the family. The team is the support for the family. The well spouse is still the primary caregiver. The dying spouse is still the person. The team is the team.

The Dying Spouse's Role in Hospice

The dying spouse has a role. The role is to live the time that is left. The role is to be the person, not the patient. The role is to make the decisions, as much as possible, for as long as possible.

The dying spouse's role includes:

- Making the decisions about the care (what to take, what not to take, who to see, what to do)
- Communicating with the hospice team (what feels good, what doesn't, what is changing)
- Being present with the well spouse, with the family, with the friends
- Doing the things that are still possible (the small ordinary things, the meaningful things)
- Letting the team help
- Letting the family help

- Letting the well spouse help

The dying spouse who is still in the role is still in the marriage. The dying spouse who has given up the role is not. The role is not the illness. The role is the marriage. The marriage is the role.

The Well Spouse's Role in Hospice

The well spouse has a role. The role is to be the primary caregiver, with the support of the team. The role is also to be the spouse. The role is not the illness. The role is the marriage. The marriage is the role.

The well spouse's role includes:

- Being the primary caregiver (the medications, the appointments, the personal care, the household)
- Communicating with the hospice team (the symptoms, the changes, the concerns)
- Being present with the dying spouse
- Taking breaks (the team can help)
- Asking for help (the family, the friends, the team)
- Letting the team help
- Being the spouse, not just the caregiver

The well spouse who is in both roles is in the marriage. The well spouse who is only the caregiver is not. The role is the marriage. The marriage is the role.

The Family and Friends' Role in Hospice

The family and friends have a role. The role is to support the couple. The role is not to take over. The role is not to add stress. The role is to be present, to help, to love.

The family and friends' role includes:

- Visiting, when the dying spouse is up for it
- Calling, when the well spouse needs to talk
- Bringing meals, when the household is overwhelmed
- Running errands, when the well spouse cannot leave
- Sitting with the dying spouse, so the well spouse can rest
- Being present for the conversations
- Not pushing their own grief onto the couple

The family and friends who are in the role are in the marriage. The family and friends who are taking over are not. The role is the support. The support is the role.

The Myth of "Giving Up" — What Hospice Really Is

The myth is that hospice is giving up. The myth is wrong. Hospice is not giving up. Hospice is choosing differently.

Hospice is choosing comfort over cure. Hospice is choosing quality of life over quantity of life. Hospice is choosing to live the time that is left, instead of fighting for time that may not come.

Hospice is not the end. Hospice is the last chapter. The last chapter can be the best chapter. The last chapter can be the most meaningful chapter. The last chapter can be the chapter where the marriage is most present, most honest, most loving.

The myth of giving up is based on the assumption that life is only worth living if it can be extended. The myth

is wrong. Life is worth living at every stage. The last stage is no less valuable than the first stage. The last stage is the stage the couple has now. The last stage is the stage hospice supports.

The Financial Reality of Hospice

Hospice is covered by Medicare, by Medicaid in most states, and by most private insurance. The coverage is for the hospice services — the nurse, the aide, the social worker, the chaplain, the volunteer, the medical equipment, the medications related to the terminal illness.

The coverage is not for:

- Room and board (in home hospice, the couple pays for the home; in residential hospice, the couple may pay for the room and board)
- Treatment for the terminal illness (hospice does not cover treatment aimed at cure)
- Treatment for unrelated conditions (hospice may cover some, depending on the situation)

The financial reality of hospice is: most of the cost is covered. The couple should ask the hospice about the costs. The couple should ask the insurance about the coverage. The couple should not let the financial concerns stop them from getting the hospice they need.

The Gifts of Hospice: Pain Control, Support, Time

The gifts of hospice are real. The gifts are the reason hospice is one of the best things that can happen in this season.

Gift 1: Pain Control. The hospice team is expert at pain control. The team knows the medications, the dosages, the combinations. The team can manage pain that the well spouse cannot manage alone. The pain control is a gift. The pain control is the comfort. The comfort is the life.

Gift 2: Symptom Control. The hospice team is expert at symptom control. Nausea, fatigue, breathlessness, anxiety, constipation, insomnia. The team can manage these symptoms. The management is the comfort. The comfort is the life.

Gift 3: Support. The hospice team supports the well spouse. The team listens, advises, helps. The team gives the well spouse a break. The team is there for the well spouse, not just the dying spouse. The support is the marriage. The marriage is the support.

Gift 4: Time. The hospice team gives the couple time. The team handles the medical. The team handles the equipment. The team handles the questions. The couple is free to be the couple. The couple is free to live the time. The time is the gift. The time is the marriage.

Gift 5: Bereavement. The hospice team supports the family after the death. The team calls, sends cards, offers support groups, connects the family to resources. The bereavement support is the gift that keeps on giving. The bereavement support is the marriage, after.

The gifts of hospice are real. The gifts are the reason hospice is one of the best things that can happen in this season.

Case Study: Ann & Tom (Ages 67/70)

Ann and Tom (from Chapter 9) chose home hospice. The hospice team came to the house. The team brought a hospital bed, a wheelchair, oxygen, medications, supplies. The team was there, 24/7, for any question or concern.

What Ann and Tom got right:

- They called hospice early. They did not wait until the last days. They had months of hospice support.
- They used the team. The nurse came 2-3 times a week. The aide came 3 times a week. The social worker came twice a month. The chaplain came once a month. The volunteer came once a week.
- They took breaks. The volunteer sat with Ann so Tom could go for a walk. Tom took Wednesdays off. Tom had coffee with a friend every Sunday.
- They had the conversations. Ann told Tom what she wanted. Ann told the team what she wanted. Ann had the death she wanted, at home, with Tom, with the team.
- They used the bereavement support. Tom is still in the support group, 6 months after Ann's death. Tom says "the group is the only place I can talk about Ann without people changing the subject."

Ann died 5 months after starting hospice. The last 5 months were the best of the long middle. The hospice made the last 5 months livable. The hospice made the marriage the marriage.

Scorecard: Is Hospice Right for Us Now?

| Question | Score (1-5) |

|---|---|

| The treatment is no longer working | /5 |

| We have had the conversation about the shift | /5 |

| We have looked into hospice | /5 |

| We have interviewed at least one hospice | /5 |

| The dying spouse wants to be at home (or in a hospice setting) | /5 |

| The well spouse is willing and able to be the primary caregiver | /5 |

| We have a network of family and friends to help | /5 |

| We understand that hospice is not giving up | /5 |

| We have asked the doctor for a hospice referral | /5 |

| We are ready to live the time we have | /5 |

Total: /50

Interpreting your score:

- 40-50: Hospice is right for you now. Make the call.
- 30-39: Some parts of the picture are not clear yet. The conversations in this chapter can help.
- 20-29: Hospice may be right soon. Get more information. Talk to a hospice social worker.
- Under 20: The shift has not happened yet. Focus on Chapter 9 first.

Key Terms

- Hospice: A philosophy of care and a service that supports people who are dying
- Palliative Care: Medical care for comfort and quality of life, not cure

- Home Hospice: Hospice care that comes to the house
- Bereavement: The state of having lost someone; also the support offered to those who have lost someone
- Respite Care: Short-term care that gives the well spouse a break

Reflection Questions for the Two of You

1. What is one thing you are most afraid of about hospice?
2. What is one thing you are most hopeful for about hospice?
3. Does the dying spouse want to be at home, in a facility, or in a residential hospice?
4. Is the well spouse willing and able to be the primary caregiver?
5. What is one question you most want to ask the hospice team?
6. If you started hospice, what would the first week look like?

Action for This Week: The Hospice Conversation

Within the next 7 days, do these 5 things:

1. Have the conversation between the two of you. "What do we want for the time we have. Where do we want to be. Who do we want to be with."
2. Ask the doctor for a hospice referral. Most doctors will refer when the time is right. If the doctor is not ready, ask why. If the couple is ready,

the doctor will refer.

3. Interview at least one hospice. Use the questions above. Get a feel for the team. The interview is information.

4. Look at the home, the facility, the residential hospice. See the space. See the equipment. See the team. The seeing is the decision.

5. Make a list of what the dying spouse wants to do in the time they have. The list is the life. The life is the gift. The gift is the hospice.

The Hospice Conversation is the work of the shift. The shift is the doorway to the quiet. The quiet is the next chapter.

A Closing Word

Hospice is a gift. The gift is the time. The gift is the comfort. The gift is the support. The gift is the marriage, in the time that is left.

The myth of giving up is wrong. The myth is replaced by the truth: hospice is choosing to live the time that is left, with comfort, with support, with the marriage as the center.

The team is the gift. The team is the support. The team is the way the marriage gets to be the marriage, in the last months.

You are not failing. You are choosing to live the time.

Chapter 11: Saying What Needs to Be Said

There are things that need to be said. The things are not the small things. The things are the big things. The things are the things you have been meaning to say for years. The things are the things you thought you had more time to say. The things are the things that, if you do not say them now, you will carry as a regret for the rest of your life.

The things are not easy to say. The things are not easy to hear. The things are not easy to live with. The saying of them is the work. The work is the marriage. The marriage is the work.

This chapter is about the four things that almost everyone needs to hear at the end of life. The four things are not the only things. The four things are the foundation. The foundation is the marriage. The marriage is the work.

The four things are: I love you. Thank you. I'm sorry. I forgive you. The four things are the architecture of the marriage. The four things are the way the marriage gets to end with the love still in charge.

Chapter at a Glance

- The Four Things Everyone Needs to Hear
- The Things You Wish You Had Said
- The Things the Dying Spouse Wants to Say
- The Unfinished Business
- The Forgiveness Conversations
- The Thank You Conversations
- The I Love You Conversations

- The Goodbye (But Not THE Goodbye)
- The Legacy Conversations
- How to Start the Hard Conversations
- What If the Dying Spouse Can't Talk Anymore?
- Case Study: Linda & James (ages 62/65)
- Scorecard: Have We Said What Needs to Be Said?
- Key Terms
- Reflection Questions
- Action for This Week: The Four Things

The Four Things Everyone Needs to Hear

The four things are simple. The four things are not easy. The four things are the things that, when they are said, release the marriage from the things that have been unsaid.

1. I love you. Not the small version. The big version. The version that says "I have loved you for the years we have had. I will love you for the years I have after. I love you in the way that does not end at death."

2. Thank you. Not for the big things. For the small things. For the way you made the coffee. For the way you held my hand. For the way you showed up. For the way you loved me. For the years. For the life. For the marriage.

3. I'm sorry. For the times I was not the spouse I should have been. For the times I was short with you. For the times I was angry. For the times I was absent. For the times I was not present. For the times I hurt you, knowingly or not.

4. I forgive you. For the times you hurt me. For the times you were not the spouse you should have been. For the times you were short with me. For the times you were angry. For the times you were absent. For the times you hurt me, knowingly or not. I forgive you. I am not carrying it anymore. I am letting it go.

The four things are not just for the dying spouse to say. The four things are for both of you. The four things are the way the marriage gets to be complete. The four things are the way the marriage ends with the love still in charge.

The Things You Wish You Had Said

The things you wish you had said are the things that are still unsaid. The things are usually small. The things are usually big. The things are usually the things you did not say because you were tired, or busy, or distracted, or afraid. The things are the things you are carrying. The things are the things that, if you do not say them now, you will carry for the rest of your life.

Some examples:

- "I should have told you more often."
- "I should have listened more."
- "I should have been more patient."
- "I should have been more present."
- "I should have said yes more often."
- "I should have said no more often."
- "I should have taken the trip with you."
- "I should have gone to the appointment with you."
- "I should have been there when you needed me."
- "I should have been softer."

- "I should have been braver."

The things you wish you had said are not failures. The things are the human parts of the marriage. The things are the things that make the marriage real. The saying of them now is not a confession. The saying of them now is a release. The release is the marriage.

The Things the Dying Spouse Wants to Say

The dying spouse has things they want to say. The things are usually the things they have been carrying. The things are usually the things they have not been able to say. The things are usually the things that are blocking the peace.

Some examples:

- "I am afraid of dying."
- "I am afraid of leaving you."
- "I am afraid of being forgotten."
- "I am sorry for the times I was not present."
- "I am sorry for the times I was angry."
- "I am sorry for the times I was hard to love."
- "I want you to be happy after I am gone."
- "I want you to find someone, if you want to."
- "I want to thank you for the years."
- "I want to thank you for the marriage."
- "I want to tell you what you meant to me."
- "I want to tell you what the marriage meant to me."
- "I want you to know that I love you."

The dying spouse who gets to say these things gets to die with the love still in charge. The dying spouse who does not get to say these things carries them to the

end. The carrying is not peace. The saying is the peace.

The Unfinished Business

The unfinished business is the things that have not been resolved. The unfinished business is the conflicts, the resentments, the wounds, the regrets, the unspoken things. The unfinished business is the weight that the marriage is carrying.

The unfinished business needs to be addressed. The unfinished business does not have to be resolved completely. The unfinished business has to be named. The naming is the work.

The unfinished business includes:

- Conflicts that were not resolved
- Resentments that were not released
- Wounds that were not healed
- Regrets that were not grieved
- Unspoken things that were not said
- Secrets that were not shared

The way to address the unfinished business:

- Name it. "There is something I have been carrying. I want to talk about it."
- Own it. "I have been angry. I want to tell you what I have been angry about."
- Listen. "I want to hear what you have been carrying."
- Forgive. "I forgive you for this. I want you to forgive me for this."
- Release. "I am letting this go. I don't want to carry it anymore."

The unfinished business does not have to be resolved in one conversation. The unfinished business can be addressed over days or weeks. The unfinished business is the work of the long middle. The work is the marriage.

The Forgiveness Conversations

The forgiveness conversations are the most important conversations in this chapter. The forgiveness conversations are the way the marriage gets to be released from the things that have been carrying it.

The forgiveness conversation is not about the specific thing. The forgiveness conversation is about the marriage. The forgiveness conversation is about saying "I am not carrying this anymore. I am letting it go. I am choosing to forgive."

The forgiveness conversation can be:

- "I forgive you for the time you..."
- "I forgive you for the way you..."
- "I forgive you for the years when..."
- "I forgive you for the things you could not control."
- "I forgive you for being human."
- "I forgive you, and I want you to forgive me."

The forgiveness conversation is not about being right. The forgiveness conversation is about being free. The freedom is the marriage. The marriage is the freedom.

The forgiveness conversation is not about forgetting. The forgiveness conversation is about releasing. The release is not forgetting. The release is choosing to carry it differently. The release is choosing to let it go. The release is the marriage.

The Thank You Conversations

The thank you conversations are the way the marriage gets to honor what has been given. The thank you conversations are the way the marriage gets to acknowledge the years, the small things, the big things, the life.

The thank you conversation can be:

- "Thank you for the years."
- "Thank you for the way you made the coffee."
- "Thank you for the way you held my hand."
- "Thank you for the way you showed up."
- "Thank you for the way you loved me."
- "Thank you for the children."
- "Thank you for the home."
- "Thank you for the trips."
- "Thank you for the quiet evenings."
- "Thank you for being you."

The thank you conversations are not about the things. The thank you conversations are about the marriage. The thank you conversations are the way the marriage gets to be honored. The honor is the marriage.

The I Love You Conversations

The I love you conversations are the way the marriage gets to be the marriage. The I love you conversations are not just the small ones, said without thinking. The I love you conversations are the big ones, said with intention, said with presence, said with the knowledge that the time is limited.

The I love you conversation can be:

- "I love you in the way that does not end at death."
- "I love you in the way that has been with me for the years."
- "I love you in the way that will be with me for the years to come."
- "I love you in the way that I have not always said, but have always felt."
- "I love you in the way that I will carry with me."
- "I love you in the way that I will remember."
- "I love you in the way that I will live."

The I love you conversations are the most important conversations in this chapter. The I love you conversations are the way the marriage gets to be complete. The completeness is the marriage.

The Goodbye (But Not THE Goodbye)

There are two goodbyes. The first goodbye is the one you have been practicing. The first goodbye is the one you say when you leave the house. The first goodbye is the one you say when you go to the store. The first goodbye is the one you say at the end of a visit. The first goodbye is the one you say with the assumption that you will see each other again.

The second goodbye is the one that is different. The second goodbye is the one you say when you are not going to see each other again. The second goodbye is the one you say at the end. The second goodbye is the one you say with the knowledge that this is the last time.

The first goodbye is easy. The second goodbye is hard. The second goodbye is the one that this chapter is preparing you for. The second goodbye is the one that,

if it is said with the love still in charge, will be a gift for the rest of your life.

The second goodbye is not the focus of this chapter. The second goodbye is the focus of Chapter 14. This chapter is preparing you for the second goodbye, by making sure the four things have been said. The four things are the way the second goodbye gets to be said with the love still in charge.

The Legacy Conversations

The legacy conversations are the way the dying spouse gets to pass on the things that matter. The legacy conversations are the way the marriage gets to continue, in the things the dying spouse passes on.

The legacy conversations can be:

- "I want you to know the stories of our marriage."
- "I want you to know the things I have learned."
- "I want you to know the things I believe."
- "I want you to know the things I value."
- "I want you to know the things I hope for you."
- "I want you to know the things I wish for the children."
- "I want you to know the things I wish for the grandchildren."
- "I want you to know the things I wish for the world."

The legacy conversations are not just the words. The legacy conversations are the values, the stories, the wisdom, the love. The legacy is what the dying spouse leaves behind. The legacy is what the well spouse carries forward. The legacy is the marriage, continuing.

How to Start the Hard Conversations

The hard conversations are hard. The hard conversations can be started with a sentence. The sentence is the door. The door is the conversation.

Some opening lines:

- "There are things I have been carrying. I want to talk about them."
- "I want to say four things to you. Can I say them now?"
- "I have been thinking about the things I want to thank you for."
- "I have been thinking about the things I want to apologize for."
- "I have been thinking about the things I want to forgive."
- "I have been thinking about the things I want to say before..."
- "I want to tell you what you mean to me."
- "I want to tell you what the marriage has meant to me."
- "I want to tell you what I hope for you."

The opening lines are the door. The door is the conversation. The conversation is the marriage.

What If the Dying Spouse Can't Talk Anymore?

Sometimes the dying spouse cannot talk. The dying spouse may be too tired, too weak, too sedated, too far gone. The talking may be over. The four things may not be able to be said out loud.

The four things can still be said. The four things can be said in other ways:

- Written. A letter. A card. A note on the bedside table.
- Touched. A hand on the hand. A kiss on the forehead. A hug in the bed.
- Looked. A look that says what the words cannot.
- Be. A presence that says what the words cannot.

The four things do not have to be said out loud. The four things can be felt. The four things can be known. The four things can be the marriage, in the silence.

The well spouse can also say the four things, out loud, even if the dying spouse cannot respond. The saying is the marriage. The marriage is the saying.

Case Study: Linda & James (Ages 62/65)

Linda and James (from Chapter 3) had 7 months from diagnosis to Linda's death. The 7 months included the time for the four things.

What Linda and James got right:

- They had the four things conversations early, in the first month. James said "I am so glad we had these conversations when we could."
- Linda wrote letters to each of the children, to the grandchildren, to James. The letters are the legacy.
- Linda said the four things to James, many times, in many ways. James still remembers every one.
- James said the four things to Linda, especially in the last days. Linda could not always respond, but she could hear.
- They had the legacy conversations. Linda told James the stories of their marriage. James told

Linda the stories he had never told. The stories are the marriage.

What Linda and James got wrong:

- They waited 2 weeks to start the four things. The 2 weeks were the time when they were both in shock. The conversations could have started sooner.
- They did not record the conversations. James wishes he had recorded them. The recording would be a gift now.

Linda died 7 months after the diagnosis. James still has the letters. James still has the memories. The four things are the foundation of the marriage, in the years after.

Scorecard: Have We Said What Needs to Be Said?

| Question | Score (1-5) |

|---|---|

| We have said "I love you" in the big way, not just the small way | /5 |

| We have said "thank you" for the years, the small things, the life | /5 |

| We have said "I'm sorry" for the times we were not the spouse we should have been | /5 |

| We have said "I forgive you" for the times the other was not the spouse they should have been | /5 |

| We have addressed the unfinished business | /5 |

| We have had the legacy conversations | /5 |

| The dying spouse has been able to say the things they wanted to say | /5 |

| The well spouse has been able to say the things they wanted to say | /5 |

| We are not carrying unspoken things into the last days | /5 |

| We are ending the marriage with the love still in charge | /5 |

Total: /50

Interpreting your score:

- 40-50: The four things are being said. The marriage is being honored.
- 30-39: Some of the four things have been said. The others are waiting. The conversations in this chapter can help.
- 20-29: The four things are not being said. The unfinished business is in the way. Get help from a counselor, a chaplain, a hospice social worker.
- Under 20: The marriage is in trouble. The unspoken things are taking over. Get help immediately.

Key Terms

- The Four Things: I love you. Thank you. I'm sorry. I forgive you.
- Unfinished Business: The unresolved conflicts, resentments, wounds, regrets, unspoken things
- Legacy: The values, stories, wisdom, love that the dying spouse passes on
- Anticipatory Grief: The grief that happens before the death, as the death becomes more certain

Reflection Questions for the Two of You

1. What is one thing you most want to say to the other?
2. What is one thing you most want to hear from the other?
3. What is the unfinished business in the marriage?
4. Is there something you have been carrying for years that you want to let go of?
5. What is the legacy you want to leave? What is the legacy you want to receive?
6. If the dying spouse could only say one more thing, what would it be?

Action for This Week: The Four Things

Within the next 7 days, do these 5 things:

1. Pick a quiet moment. Not in a crisis. Not in the hospital. A Saturday morning, an afternoon, an evening.
2. Say the four things. Out loud. To each other. In whatever order. In whatever words. The four things are: I love you. Thank you. I'm sorry. I forgive you.
3. Listen to the four things. Let the other person say them. Don't interrupt. Don't defend. Don't explain. Just receive.
4. Write down the legacy. What do you want to pass on? Stories, values, wisdom, love. Write it down. The writing is the legacy.

5. Make a recording, if you can. Even a voice memo on the phone. The recording is the gift. The recording is the marriage, continuing.

The Four Things is the work of the marriage, in the time that is left. The work is not the illness. The work is the marriage. The marriage is the work.

A Closing Word

The four things are the foundation. The foundation is the marriage. The marriage is the work.

The things you wish you had said are the things you can say now. The things the dying spouse wants to say are the things they can say now. The unfinished business is the business that can be named now. The forgiveness is the forgiveness that can be given now. The thank you is the thank you that can be said now. The I love you is the I love you that can be said now.

The saying is the marriage. The marriage is the saying. The marriage is the work.

You are not failing. You are saying the things.

Chapter 12: The Last Good Day

There is a moment in every terminal illness when the body starts to decline. The decline is not a moment. The decline is a process. The process takes days or weeks. The process is the body's way of letting go.

Before the decline, there is often a period of time that is still good. The good days may be mixed with the bad days. The good days may be fewer. The good days may be shorter. The good days are still there. The good days are the time to be used.

This chapter is about the good days. The good days are not the past. The good days are the present. The good days are the time the couple has now. The good days are the time the marriage is still here.

The good days are also not the focus of the long middle. The focus of the long middle is the appointments, the medications, the symptoms, the decisions. The good days are the days when the focus can be the marriage. The good days are the days when the couple gets to be the couple, not the patient and the caregiver.

This chapter is about how to recognize the good days, how to use the good days, and how to hold the good days when they are gone.

Chapter at a Glance

- The Slow Decline, and the Days That Are Still Good
- How to Recognize the "Last Good Day" (And the Truth That There Isn't Always One)

- The Decline That Comes in Waves, Not a Line
- The Good Days Mixed In With the Hard Days
- The Role of Food, Comfort, Sleep, Presence
- The Role of Family and Friends
- The Role of Memory and Meaning
- The Small Moments That Matter
- The Role of Music, Prayer, Ritual, Reading
- Case Study: Linda & James (ages 62/65)
- Scorecard: Are We Using the Good Days Well?
- Key Terms
- Reflection Questions
- Action for This Week: The Memory Book

The Slow Decline, and the Days That Are Still Good

The slow decline is real. The body is changing. The energy is decreasing. The symptoms are getting worse. The decline is not a moment. The decline is a process. The process takes days or weeks. The process is the body's way of letting go.

The decline does not mean that every day is bad. The decline means that the bad days are more frequent, and the good days are less frequent. The good days are still there. The good days are the days when the energy is back, when the symptoms are managed, when the mind is clear, when the body is willing.

The good days are not predictable. The good days come in waves. The good days may be in the morning one day, and in the afternoon the next day. The good days are not a schedule. The good days are a gift.

The good days are also not the past. The good days are not the days before the illness. The good days are

the days of the illness, when the body is willing. The good days are the days the couple has now.

The good days are the time to be used. The good days are the time to be the couple. The good days are the time to do the small ordinary things, the meaningful things, the things the couple has been meaning to do.

How to Recognize the "Last Good Day" (And the Truth That There Isn't Always One)

The "last good day" is a concept that has become part of the end-of-life vocabulary. The last good day is the day when the dying spouse is still able to do the things they love, before the final decline. The last good day is the day that, in retrospect, was the last day.

The last good day is real, but it is not always recognizable in the moment. The last good day is usually recognizable in retrospect. The day is happening, and the couple is in the day, and the day is good, and the couple does not know it is the last. The day ends, and the next day is harder, and the next day is harder, and the days get harder, and the couple looks back and says "that was the last good day."

The last good day is not always a single day. The last good day can be a period of days, a week, a month. The last good day is the time when the body is still willing, before the final decline.

The last good day is also not always the day of the most activity. The last good day can be a quiet day, a day of presence, a day of just being together. The last good day is not about what the couple does. The last good day is about the couple being together.

The truth is: there is not always a "last good day" that is recognizable. Some couples never have a clear last good day. The decline is gradual, and the days get harder, and the couple does not know which day was the last. The not-knowing is also okay. The not-knowing is part of the process.

The way to be with the last good day:

- Use the good days when they come. Don't postpone.
- Be present on the good days. Don't waste them on the illness.
- Take the photos, make the memories, do the things.
- Let the good days be the marriage.

The Decline That Comes in Waves, Not a Line

The decline is not a line. The decline is a wave. The wave goes up and down. The wave has good days and bad days. The wave gets lower over time, but the wave is still a wave.

The wave is hard to predict. The wave is hard to plan around. The wave is hard to live with. The wave is also the way the body lets go.

The wave means that the couple cannot plan for the future. The couple cannot know if tomorrow will be a good day or a bad day. The couple can only plan for the day. The couple can only plan for the moment.

The wave means that the couple has to be flexible. The couple has to be ready to change the plan, to cancel the visit, to skip the outing. The couple has to be ready to be in the good day when it comes, and to be in the bad day when it comes. The flexibility is the

marriage.

The wave also means that the couple has to be present. The couple cannot take the good day for granted. The couple cannot assume there will be another good day. The couple has to use the good day, in the moment, in the presence.

The wave is the way the body lets go. The wave is the way the marriage gets to be present. The wave is the way the time is used.

The Good Days Mixed In With the Hard Days

The good days are mixed in with the hard days. The mix is the reality. The mix is the way the long middle is.

The good days are the days when the body is willing, the mind is clear, the energy is back. The good days are the days when the couple gets to be the couple. The good days are the days for the small ordinary things, the meaningful things, the things the couple has been meaning to do.

The hard days are the days when the body is not willing, the mind is not clear, the energy is gone. The hard days are the days when the dying spouse is in bed, or in pain, or in fog. The hard days are the days for the caregiving, the patience, the presence.

The good days and the hard days are not separate. The good days and the hard days are part of the same long middle. The good days and the hard days are the marriage.

The way to be with the good days mixed in with the hard days:

- Use the good days. Don't postpone the good things for a better day.
- Be present on the hard days. Don't run from the hard things.
- Don't take the good days for granted. The good days are gifts.
- Don't despair on the hard days. The hard days are part of the process.
- Let the good days and the hard days be the marriage. The marriage is both.

The Role of Food, Comfort, Sleep, Presence

The good days are supported by the basic things. The basic things are not glamorous. The basic things are the foundation.

Food. The food is a basic thing. The food is also a meaningful thing. The food the dying spouse loves is the food that brings comfort. The food is a way of saying "I love you." The food is a way of saying "I am taking care of you." The food is a way of saying "the marriage is still here." The food is the marriage.

The food does not have to be elaborate. The food can be a simple meal. The food can be a favorite dish. The food can be a comfort food from the marriage's past. The food is what the dying spouse wants to eat, when the dying spouse can eat.

Comfort. The comfort is the physical things. The blanket. The pillow. The chair. The bed. The comfort is what makes the body willing. The comfort is what allows the dying spouse to rest, to sleep, to be present. The comfort is the well spouse's gift.

Sleep. The sleep is the basic need. The sleep is the body's way of restoring. The sleep is also the time when the well spouse can rest, can be away, can take a break. The sleep is the rhythm of the long middle.

Presence. The presence is the most important basic thing. The presence is the well spouse being there. The presence is the dying spouse not being alone. The presence is the marriage, in the room.

The food, the comfort, the sleep, the presence are not the illness. The food, the comfort, the sleep, the presence are the marriage. The marriage is the foundation.

The Role of Family and Friends

The family and friends have a role in the good days. The role is to be present, to share the time, to make the memories.

The good days are the days for the visits. The visits should be planned around the energy. The visits should be short, if needed. The visits should be meaningful. The visits are the memories.

The good days are the days for the small gatherings. A small dinner. A coffee on the porch. A walk in the garden. The gatherings are the memories.

The good days are the days for the photos. The photos are the memories. The photos are the gift the couple gives to the family. The photos are the marriage, in pictures.

The good days are the days for the stories. The stories are the memories. The stories are the marriage. The stories are the legacy.

The family and friends who are present in the good days are present in the memories. The family and

friends who are not present are not. The presence is the gift. The gift is the marriage.

The Role of Memory and Meaning

The good days are the days for memory and meaning. The memory is the past. The meaning is the present. The memory and the meaning are the marriage.

The memory is the stories of the marriage. The trips. The children. The fights. The reconciliations. The ordinary days. The extraordinary days. The memory is the past, brought into the present.

The meaning is the present. The present is the good day. The present is the time the couple has now. The present is the marriage, in the time.

The way to be with memory and meaning:

- Tell the stories. The stories are the memory. The memory is the marriage.
- Make the meaning. The meaning is the present. The present is the gift.
- Connect the past and the present. The past and the present are the marriage.
- Honor the marriage. The marriage is the memory and the meaning.

The memory and the meaning are not the illness. The memory and the meaning are the marriage. The marriage is the foundation.

The Small Moments That Matter

The good days are made of small moments. The small moments are the things the couple will remember. The small moments are the things the marriage is made of.

Some small moments:

- The morning coffee, shared in silence.
- The afternoon walk, hand in hand.
- The evening meal, at the table.
- The bedtime routine, in the bedroom.
- The look across the room.
- The laugh at the same joke.
- The touch of the hand.
- The "I love you" said without thinking.
- The memory of the trip, told again.
- The story of the children, told again.
- The silence, shared.

The small moments are not the illness. The small moments are the marriage. The marriage is the small moments.

The way to be with the small moments:

- Be present. The small moments are the present.
- Don't postpone. The small moments are now.
- Don't take them for granted. The small moments are gifts.
- Let the small moments be the marriage. The marriage is the small moments.

The Role of Music, Prayer, Ritual, Reading

The good days are supported by the things that bring comfort. The things that bring comfort are different for every couple. The things can include music, prayer, ritual, reading, and other things that are meaningful.

Music. The music is a powerful thing. The music can bring back memories. The music can soothe. The music can make the present more bearable. The music the couple has loved together is the music that brings the marriage into the room.

Prayer. The prayer is a way of connecting with the transcendent. The prayer can be religious, or it can be a personal ritual. The prayer is the way the couple connects with the larger story. The prayer is the way the couple finds meaning.

Ritual. The ritual is the regular thing. The morning coffee. The evening walk. The bedtime routine. The ritual is the structure of the day. The ritual is the way the day has a shape. The ritual is the marriage, in the routine.

Reading. The reading is a way of being together. The reading can be a book the couple is reading together. The reading can be a poem. The reading can be a letter. The reading is the way the couple shares a story, a thought, a moment.

The music, the prayer, the ritual, the reading are not the illness. The music, the prayer, the ritual, the reading are the marriage. The marriage is the foundation.

Case Study: Linda & James (Ages 62/65)

Linda and James (from previous chapters) had good days throughout Linda's illness. The good days were not every day. The good days were when Linda's energy was back, when her mind was clear, when the symptoms were managed.

What Linda and James did with the good days:

- They took a short trip to the coast in month 3. The trip was 3 days. The trip was a memory they both cherish.
- They had a small dinner party in month 4. The dinner was for the close friends. The dinner was a memory.
- They sat in the garden in month 5. The sitting was a daily ritual. The sitting was a memory.
- They looked at the photos in month 6. The looking was a ritual. The looking was a memory.
- They told the stories in month 7. The telling was a ritual. The telling was the marriage.

The last good day for Linda was a Tuesday in month 7. They sat in the garden. They had tea. They watched the birds. They did not know it was the last good day. The day was the marriage.

Linda declined over the next 2 weeks. The decline was the wave. The wave got lower. The wave was the body letting go. James was there. James was present. The presence was the marriage.

Scorecard: Are We Using the Good Days Well?

| Question | Score (1-5) |

|---|---|

| We are recognizing the good days when they come | /5 |

| We are using the good days, not postponing them | /5 |

| We are being present on the good days | /5 |

| We are taking photos, making memories | /5 |

| We are telling the stories of the marriage | /5 |
| We are involving family and friends in the good days
| /5 |
| We are using the things that bring comfort (music,
prayer, ritual, reading) | /5 |
We are not taking the good days for granted	/5
We are not despairing on the hard days	/5
We are letting the good days be the marriage	/5
Total: /50

Interpreting your score:

- 40-50: The good days are being used well. The marriage is being honored.
- 30-39: Some patterns to fix. The small moments in this chapter can help.
- 20-29: The good days are slipping by. Make a plan this week.
- Under 20: The good days are being lost to the illness. Get help to refocus.

Key Terms

- The Last Good Day: The day when the dying spouse is still able to do the things they love, before the final decline
- Wave Pattern: The way the decline happens in waves, with good days and bad days, rather than a steady line
- Anticipatory Grief: The grief that happens before the death, as the death becomes more certain
- Legacy: The values, stories, wisdom, love that the dying spouse passes on

Reflection Questions for the Two of You

1. What is one good day you have had recently? What made it good?
2. What is one thing you most want to do on a good day?
3. What is one small moment from the marriage that you want to remember?
4. What is one ritual that brings you comfort?
5. What is one thing you most want to do with the family or friends in the time you have?
6. If you knew this was the last good day, what would you do?

Action for This Week: The Memory Book

Within the next 7 days, do these 5 things:

1. Start a memory book. A notebook, a journal, a folder on the computer. The memory book is for the stories of the marriage. The memory book is for the good days.
2. Tell one story. A story from the marriage. A story from the early years. A story from the children. A story from the trips. The story is the memory. The memory is the marriage.
3. Take one photo. A photo of the two of you. A photo of the good day. A photo of the small moment. The photo is the memory. The memory is the marriage.
4. Do one small ordinary thing. A meal. A walk. A coffee. The thing is the marriage.

5. Plan one thing for a future good day. A visit. A call. A trip. A gathering. The plan is the hope. The hope is the marriage.

The Memory Book is the work of the good days. The work is not the illness. The work is the marriage. The marriage is the memory book.

A Closing Word

The good days are gifts. The good days are the time the couple has now. The good days are the time the marriage is still here.

The decline is real. The wave is real. The hard days are real. The good days are also real. The good days are the days when the marriage is most present. The good days are the days when the couple gets to be the couple.

The way to use the good days is to be present. The way to use the good days is to make the memories. The way to use the good days is to let the good days be the marriage.

The good days will end. The good days will be the past. The good days will be the memories. The good days will be the marriage, in the years after.

You are not failing. You are using the good days.

Chapter 13: The Final Days

The final days are the days when the body is letting go. The final days are the days when the energy is going. The final days are the days when the dying spouse is moving from being here to being gone. The final days are the most sacred days, and the most difficult days, of the long middle.

The final days are not a moment. The final days are a process. The process takes days, sometimes a week or two. The process is the body's way of preparing for the end. The process is the marriage's way of preparing for the goodbye.

This chapter is about the final days. The chapter is about what to expect, what to do, what to say, what to be. The chapter is about the dying spouse's body, and the well spouse's presence, and the marriage at the very end.

I want to be careful with this chapter. The final days are sacred. The final days are not clinical. The final days are the most intimate days of the marriage. The final days are the days the couple will remember for the rest of their lives.

Chapter at a Glance

- The Physical Changes in the Final Days
- The Emotional Changes
- The Role of the Well Spouse in the Final Days
- The Role of Family
- The Vigil — What It Is, When to Start, How to Do It

- Sleep, Food, Presence — The Practical Matters of the Vigil
- The Dying Spouse's Last Words, Last Gestures, Last Requests
- The Moments of Lucidity Mixed With Confusion
- The Role of the Medical Team in the Final Days
- What Death Looks Like (Physically) — So You Are Not Surprised
- Case Study: Margaret & David (ages 68/70)
- Scorecard: How Are You Doing as the Caregiver in the Final Days?
- Key Terms
- Reflection Questions
- Action for This Week: The Final Days Plan

The Physical Changes in the Final Days

The body changes in the final days. The changes are real. The changes are also the body's way of letting go. The changes are not something to be afraid of. The changes are the process.

Some of the physical changes in the final days:

Decreased energy. The dying spouse will sleep more. The dying spouse will be awake less. The awake time will be short. The energy is going inward, not outward. The body is preparing.

Decreased appetite. The dying spouse will eat less. The dying spouse may stop eating. The dying spouse may not want to drink. The body does not need the food. The body is letting go of the need for food.

Changes in breathing. The breathing may change. The breathing may become irregular. The breathing may

have pauses. The breathing may have a sound (called "death rattle" — it is not painful, it is the sound of the body letting go). The breathing is the body's way of preparing.

Changes in skin. The skin may become cooler. The skin may become mottled. The skin may become bluish in the extremities. The skin is the body's way of letting go.

Changes in consciousness. The dying spouse may become less responsive. The dying spouse may drift in and out. The dying spouse may have moments of confusion. The dying spouse may have moments of lucidity. The consciousness is the body's way of letting go.

Changes in the body. The body is shutting down. The systems are slowing. The body is preparing for the end. The end is not a failure. The end is the natural process.

The physical changes are not something to be fought. The physical changes are the process. The process is the way the body lets go.

The Emotional Changes

The emotional changes in the final days are also real. The emotional changes are the way the dying spouse prepares for the end. The emotional changes are also the way the dying spouse processes the life.

Some of the emotional changes in the final days:

Withdrawal. The dying spouse may withdraw from the world. The dying spouse may not want visitors. The dying spouse may want only the well spouse. The withdrawal is the way the dying spouse focuses inward. The withdrawal is not a rejection. The

withdrawal is the process.

Restlessness. The dying spouse may be restless. The dying spouse may pick at the sheets. The dying spouse may try to get out of bed. The restlessness is the way the body and the mind let go. The restlessness is not pain. The restlessness is the process.

Moments of lucidity. The dying spouse may have moments of lucidity. The dying spouse may say things that are profound, or funny, or tender. The moments are gifts. The moments are the marriage. The moments are to be received.

Moments of confusion. The dying spouse may have moments of confusion. The dying spouse may not know where they are. The dying spouse may not know who is in the room. The confusion is the body's way of letting go. The confusion is not a sign of suffering. The confusion is the process.

Final conversations. The dying spouse may want to have final conversations. The dying spouse may say the things that have been unsaid. The dying spouse may give the last blessings. The final conversations are the most important conversations. The final conversations are the marriage.

The emotional changes are not something to be fixed. The emotional changes are the process. The process is the way the dying spouse prepares. The preparation is the marriage.

The Role of the Well Spouse in the Final Days

The well spouse has a role in the final days. The role is the most important role of the long middle. The role is the marriage, in the final days.

The role of the well spouse in the final days:

Presence. The well spouse is present. The presence is the most important thing. The presence is the way the well spouse says "I am here." The presence is the way the well spouse says "you are not alone." The presence is the way the well spouse says "the marriage is still here."

Permission. The well spouse gives the dying spouse permission to let go. "It is okay to go. I will be okay. The children will be okay. You have given me a life. You have given me a marriage. You can go." The permission is the gift the well spouse gives the dying spouse. The permission is the way the marriage ends with the love still in charge.

Comfort. The well spouse provides the comfort. The hand on the hand. The kiss on the forehead. The cool cloth on the brow. The comfort is the physical care. The comfort is also the emotional care. The comfort is the marriage.

Advocacy. The well spouse advocates for the dying spouse. With the medical team, the family, the visitors. The advocacy is the way the well spouse protects the dying spouse. The advocacy is the way the well spouse honors the dying spouse's wishes.

Witness. The well spouse is the witness. The witness to the final days. The witness to the last breath. The witness to the life. The witness is the way the well spouse holds the memory. The witness is the way the marriage is remembered.

The role of the well spouse in the final days is not the illness. The role of the well spouse in the final days is the marriage. The marriage is the role.

The Role of Family

The family has a role in the final days. The role is to be present, to support, to honor.

The family's role in the final days:

Presence. The family can be present, if the dying spouse wants. The presence can be at the bedside, in the room, in the house. The presence is the way the family says goodbye. The presence is the way the family honors the dying spouse.

Support. The family can support the well spouse. The family can take shifts so the well spouse can rest. The family can bring food. The family can make the calls. The support is the way the family helps the well spouse be the spouse.

Honor. The family can honor the dying spouse's wishes. The family can keep the visits short, if needed. The family can keep the conversations light. The family can follow the lead of the dying spouse. The honor is the way the family respects the dying spouse.

Goodbye. The family can say goodbye. The goodbye can be a word, a touch, a kiss, a look. The goodbye is the way the family releases the dying spouse. The goodbye is the way the family holds the memory.

The family that is present in the final days is present in the memory. The family that supports the well spouse is supporting the marriage. The family that honors the dying spouse is honoring the marriage.

The Vigil — What It Is, When to Start, How to Do It

The vigil is the practice of being present with the dying spouse in the final days. The vigil is a tradition in many cultures and religions. The vigil is a way of accompanying the dying spouse through the final transition. The vigil is the way the marriage is present at the end.

The vigil is not something you start when the dying spouse is hours from death. The vigil is something you start when the dying spouse is clearly in the final days. The vigil can last hours, or a day, or several days. The vigil is the way you are there.

The vigil can be done by the well spouse alone, or with the family, or with a hospice volunteer, or with a spiritual care provider. The vigil is the presence, not the person. The presence is what matters.

The vigil involves:

- Being in the room, or nearby
- Touching the dying spouse (hand, forehead, arm)
- Talking softly, if the dying spouse seems to hear
- Playing music, if the dying spouse liked music
- Reading, if the dying spouse liked reading
- Praying, if prayer is part of the marriage
- Being quiet, when quiet is needed
- Watching the breathing, the changes, the signs

The vigil is not a job. The vigil is a privilege. The vigil is the way the marriage is present at the end. The end is sacred. The vigil is sacred.

Sleep, Food, Presence — The Practical Matters of the Vigil

The vigil is sacred, but the vigil is also practical. The well spouse needs to sleep, to eat, to take breaks. The dying spouse needs to be cared for. The hospice team is there to help.

Some practical matters of the vigil:

Sleep. The well spouse needs sleep. The well spouse cannot do the vigil without sleep. The hospice team, the family, the volunteers can take shifts so the well spouse can rest. The rest is not a failure. The rest is part of the vigil.

Food. The well spouse needs to eat. The well spouse cannot do the vigil without food. The family, the friends, the hospice team can bring food. The eating is not a failure. The eating is part of the vigil.

Breaks. The well spouse needs to take breaks. The well spouse cannot do the vigil without breaks. The hospice team, the family, the volunteers can give the breaks. The breaks are not a failure. The breaks are part of the vigil.

Care for the dying spouse. The dying spouse needs to be cared for. The hospice team can help with the personal care, the medications, the comfort. The well spouse can focus on the presence. The presence is the marriage.

The practical matters are not a distraction from the vigil. The practical matters are part of the vigil. The vigil is the whole — the presence, the care, the rest, the food. The whole is the marriage.

The Dying Spouse's Last Words, Last Gestures, Last Requests

The dying spouse may have last words, last gestures, last requests. The last words, last gestures, last requests are gifts. The gifts are the marriage, in the final days.

Some last words may be:

- "I love you."
- "Thank you."
- "I'm sorry."
- "I forgive you."
- "Take care of the children."
- "Take care of yourself."
- "Be happy."
- "Find someone, when you are ready."
- "Remember me."
- "Don't forget me."

Some last gestures may be:

- A squeeze of the hand
- A look of love
- A kiss
- A smile
- A tear

Some last requests may be:

- A song to be played
- A prayer to be said
- A person to be called
- A thing to be done
- A place to be buried

- A way to be remembered

The last words, last gestures, last requests are sacred. The well spouse receives them. The well spouse does not argue. The well spouse does not promise what cannot be promised. The well spouse just receives. The receiving is the marriage.

The Moments of Lucidity Mixed With Confusion

The dying spouse may have moments of lucidity mixed with confusion. The moments of lucidity are gifts. The moments of confusion are part of the process.

The moments of lucidity can come at any time. The moments of lucidity can be brief, or they can last. The moments of lucidity are the times when the dying spouse is fully present, fully aware, fully themselves. The moments of lucidity are the times when the well spouse gets to be with the dying spouse as the person, not as the patient.

The moments of confusion are the times when the dying spouse is not fully present. The dying spouse may not know where they are. The dying spouse may not know who is in the room. The dying spouse may be talking to people who are not there. The moments of confusion are the body's way of letting go. The moments of confusion are not a sign of suffering. The moments of confusion are the process.

The way to be with the moments of lucidity and the moments of confusion:

- Be present in both.
- Don't correct the moments of confusion. The dying spouse is not there to be corrected. The dying spouse is there to be loved.

- Receive the moments of lucidity. The moments are gifts. The gifts are the marriage.
- Let the moments of confusion be. The confusion is the process. The process is the way the body lets go.

The Role of the Medical Team in the Final Days

The hospice team is essential in the final days. The team is there for the dying spouse, the well spouse, the family. The team is the support.

The team's role in the final days:

The nurse. The nurse is on call, 24/7. The nurse is the one to call if there is a change, a concern, a question. The nurse can come to the house at any time. The nurse is the medical support.

The aide. The aide is the one who helps with the personal care. The aide can help with the bathing, the dressing, the turning, the comfort. The aide is the physical support.

The social worker. The social worker is the one who supports the family. The social worker can help with the funeral planning, the family dynamics, the resources. The social worker is the emotional support.

The chaplain. The chaplain is the one who supports the spiritual. The chaplain can be there for the vigil, the prayer, the ritual. The chaplain is the spiritual support.

The volunteer. The volunteer is the one who can sit with the dying spouse, so the well spouse can rest. The volunteer is the practical support.

The team is the gift. The team is the way the well spouse is not alone. The team is the way the marriage is supported. The team is the way the final days are not faced alone.

What Death Looks Like (Physically) — So You Are Not Surprised

The well spouse should know what death looks like, physically, so they are not surprised. The surprise can be shocking. The shock can be harder than the death itself. The preparation is the gift.

The signs that death is near:

- The breathing becomes irregular, with pauses.
- The skin becomes cool, especially the extremities.
- The skin becomes mottled, especially the legs.
- The consciousness decreases, with the dying spouse sleeping most of the time.
- The appetite decreases, and the dying spouse may stop eating or drinking.
- The body may release urine or stool, involuntarily.
- The eyes may be open or closed, and may not focus.
- The breathing may make a sound, called the "death rattle."

The moment of death:

- The breathing stops.
- The heart stops.
- The body relaxes.
- There may be a final sound, a final breath, a final movement.

- The eyes may be open or closed.
- The body is still.

The first hour after death:

- The body begins to cool.
- The body may release fluids.
- The body may stiffen (rigor mortis), but not for several hours.
- The face may relax, and may look peaceful.

The well spouse should know these things. The well spouse should not be surprised. The well spouse should be prepared. The preparation is the gift.

After the death, the well spouse does not have to do anything immediately. The well spouse can sit with the body. The well spouse can hold the hand. The well spouse can say goodbye. The well spouse can call the hospice nurse, the funeral home, the family. The well spouse has time. The body is not going anywhere. The marriage is still there.

Case Study: Margaret & David (Ages 68/70)

Margaret and David (from Chapter 4) had 6 weeks from Margaret's stroke to her death. The 6 weeks included the final days.

What David did in the final days:

- He was present. He was at the bedside for most of the final 2 weeks. He took breaks when the hospice volunteer came.
- He gave permission. He said "It is okay to go, Margaret. I will be okay. The children will be okay. You can go." Margaret died 6 hours later.

- He held her hand. He talked to her. He played her favorite music. He read her favorite poems. He prayed.
- He called the children. The children came. The children were present. The children said goodbye.
- He called the hospice nurse. The nurse came. The nurse helped with the comfort. The nurse was there at the end.
- He sat with the body after the death. He held her hand. He cried. He said goodbye.

What David got right:

- He was present. He did not run from the final days.
- He gave permission. He let Margaret go.
- He accepted help. The hospice team was there. The children were there.
- He was prepared. He knew what to expect. The preparation was the gift.

What David got wrong:

- He did not sleep enough in the first few days. He was exhausted by the end.
- He did not eat enough. The family brought food. He should have eaten more.
- He did not let the children help more. The children wanted to help. He should have let them.

Margaret died 6 weeks after the stroke. The final days were the most sacred days. David was present. David was the marriage. The marriage was the vigil.

Scorecard: How Are You Doing as the Caregiver in the Final Days?

| Question | Score (1-5) |

|---|---|

| I am present, not running from the final days | /5 |

| I am giving the dying spouse permission to go | /5 |

| I am taking breaks, sleeping, eating | /5 |

| I am accepting help from the hospice team, the family, the volunteers | /5 |

| I am providing the comfort (touch, music, prayer, reading) | /5 |

| I am receiving the last words, last gestures, last requests | /5 |

| I am not correcting the moments of confusion | /5 |

| I am being present in the moments of lucidity | /5 |

| I am prepared for what death looks like, physically | /5 |

| I am letting the final days be sacred, not clinical | /5 |

Total: /50

Interpreting your score:

- 40-50: You are doing the vigil well. The marriage is being honored.
- 30-39: Some patterns to fix. The practical matters in this chapter can help.
- 20-29: The vigil is in trouble. Get help from the hospice team, the family, a chaplain.
- Under 20: The vigil is overwhelming you. Get help immediately.

Key Terms

- Vigil: The practice of being present with the dying spouse in the final days

- **Permission to Go:** The well spouse giving the dying spouse permission to die
- **Death Rattle:** The sound of the breathing in the final days, caused by the body letting go
- **Rigor Mortis:** The stiffening of the body after death
- **Anticipatory Grief:** The grief that happens before the death, as the death becomes more certain

Reflection Questions for the Two of You

1. What is one thing you most want to do in the final days?
2. What is one thing you most want to say before the end?
3. What is one thing you most want the well spouse to know?
4. What is one thing you most want the family to know?
5. How will the well spouse take care of themselves in the final days?
6. What will the final days look like, in the way you want them to?

Action for This Week: The Final Days Plan

Within the next 7 days, do these 5 things:

1. Have the conversation about the final days. With the dying spouse, with the family, with the hospice team. The conversation is the plan. The plan is the gift.

2. Identify the support team. The hospice nurse, the aide, the social worker, the chaplain, the volunteer, the family. The support team is the gift.

3. Make a list of the things the dying spouse wants. The music, the prayers, the readings, the people, the rituals. The list is the vigil. The vigil is the marriage.

4. Make a list of the things the well spouse needs. The sleep, the food, the breaks, the support. The list is the practical. The practical is part of the vigil.

5. Decide where the final days will be. At home, in a facility, in a residential hospice. The decision is the plan. The plan is the gift.

The Final Days Plan is the preparation. The preparation is the gift. The gift is the marriage, in the final days.

A Closing Word

The final days are sacred. The final days are the most intimate days of the marriage. The final days are the days the couple will remember for the rest of their lives.

The well spouse is present. The dying spouse is leaving. The marriage is being completed. The completion is the gift. The gift is the marriage, in the final days.

You are not failing. You are completing the marriage.

Chapter 14: The Last Breath

There is a moment. The moment is the last breath. The moment is the end of the breathing, the end of the heartbeat, the end of the body. The moment is the end of the marriage as it has been. The moment is the beginning of the marriage as it will be.

The moment is not a moment of panic. The moment is not a moment of horror. The moment is a moment of completion. The moment is the way the body lets go. The moment is the way the marriage gets to be the marriage, one last time.

This chapter is about that moment. The chapter is about what happens, what to do, what to feel. The chapter is about the last breath, and the first hours, and the first day. The chapter is about the well spouse, and the family, and the marriage at the very end.

I want to be careful with this chapter. The moment is sacred. The moment is not clinical. The moment is the most intimate moment of the marriage. The moment is the moment the well spouse will carry for the rest of their life. The moment deserves care.

Chapter at a Glance

- The Moment of Death — What Happens, What It Looks Like
- The Role of the Well Spouse
- The Role of Family
- What to Do in the First Hour After Death

- The Call to the Doctor, the Funeral Home, the Family
- The First Minutes, Hours, Days
- The Grief That Begins Before Death and the Grief That Begins at Death
- The Differences for Sudden vs Expected Death
- The Things No One Tells You About Death
- The Things You Can Do to Honor the Moment
- The Role of Rituals
- Case Studies: All 5 Converge
- Scorecard: How Are You Doing in the First 24 Hours After?
- Key Terms
- Reflection Questions
- Action for This Week: The First Day Plan

The Moment of Death — What Happens, What It Looks Like

The moment of death is the moment when the breathing stops and the heart stops. The moment is usually peaceful. The moment is usually quiet. The moment is usually a release.

What happens in the moment:

- The breathing becomes slower, more irregular, with longer pauses.
- The breathing may have the "death rattle" sound, but this is the body's way of letting go, not a sign of pain.
- The breathing may have one final, longer breath, often called the "last breath."
- The breathing stops.

- The heart may continue to beat for a minute or two after the breathing stops, but it is not conscious.
- The heart stops.
- The body relaxes. The muscles release. The face softens. The hands may move slightly.
- The eyes may be open or closed. They will not focus.
- There may be a final sound, a final movement, a final breath.
- The body is still.

The moment of death is not a moment of pain. The moment of death is a moment of release. The release is the body's way of letting go. The release is the way the dying spouse stops fighting, and stops holding on, and lets the body do what the body needs to do.

The moment of death is not a moment of horror. The moment of death is a moment of completion. The completion is the marriage. The marriage is the completion.

The Role of the Well Spouse

The well spouse has a role in the moment. The role is the most sacred role of the long middle. The role is the marriage, in the moment.

The role of the well spouse in the moment:

Witness. The well spouse is the witness. The witness to the last breath. The witness to the release. The witness to the moment. The witness is the way the well spouse holds the memory. The witness is the way the marriage is remembered.

Presence. The well spouse is present. The presence is the way the well spouse says "I am here." The

presence is the way the well spouse says "you are not alone." The presence is the way the well spouse says "the marriage is still here." The presence is the way the well spouse is there, until the very end, and after.

Permission. The well spouse may have already given permission. The well spouse may have said "It is okay to go." The permission is the gift the well spouse gives the dying spouse. The permission is the way the marriage ends with the love still in charge.

Touch. The well spouse may be touching the dying spouse. The hand on the hand. The kiss on the forehead. The touch is the physical way the well spouse says "I love you." The touch is the physical way the marriage is present.

Goodbye. The well spouse may say goodbye. The goodbye can be a word, a touch, a look, a kiss. The goodbye is the way the well spouse releases the dying spouse. The goodbye is the way the well spouse begins the after.

The role of the well spouse in the moment is not the illness. The role of the well spouse in the moment is the marriage. The marriage is the role.

The Role of Family

The family has a role in the moment. The role is to be present, to support, to honor.

The family's role in the moment:

Presence. The family may be present at the moment. The presence can be in the room, at the bedside, in the house. The presence is the way the family witnesses the moment. The presence is the way the family holds the memory.

Support. The family can support the well spouse. The family can hold the well spouse. The family can take over the calls, the arrangements, the practical. The support is the way the family helps the well spouse in the first hours and days.

Honor. The family can honor the dying spouse. The family can say their own goodbyes. The family can tell the stories. The family can hold the memory. The honor is the way the family respects the dying spouse's life.

Goodbye. The family can say goodbye. The goodbye can be a word, a touch, a look, a kiss. The goodbye is the way the family releases the dying spouse. The goodbye is the way the family holds the memory.

The family that is present in the moment is present in the memory. The family that supports the well spouse is supporting the marriage. The family that honors the dying spouse is honoring the marriage.

What to Do in the First Hour After Death

The first hour after death is a sacred time. The first hour is not a time to do things. The first hour is a time to be with the body, with the memories, with the grief. The first hour is a time to say goodbye, in whatever way feels right.

Some things the well spouse can do in the first hour:

Stay with the body. The well spouse does not have to leave. The well spouse can stay. The well spouse can sit with the body. The well spouse can hold the hand. The well spouse can touch the face. The well spouse can talk to the body. The body is still there. The body is the last physical presence of the dying spouse.

Take the time you need. The well spouse does not have to rush. The well spouse can take an hour. The well spouse can take more. The body is not going anywhere. The marriage is still there. The well spouse has time.

Let the family in, if they want to come. The family can come in, if they want. The family can say their goodbyes. The family can hold the body. The family can be part of the moment. The family is part of the marriage.

Call the hospice nurse, when you are ready. The hospice nurse can come to the house. The nurse can pronounce the death officially. The nurse can help with the practical. The nurse can be a support. The nurse is there for the well spouse, not just the dying spouse.

Call the funeral home, when you are ready. The funeral home can come to the house. The funeral home can take the body, when the well spouse is ready. The funeral home is not in a rush. The well spouse has time.

Don't do anything you are not ready to do. The well spouse does not have to do anything in the first hour. The well spouse can sit. The well spouse can cry. The well spouse can be silent. The well spouse can be present. The presence is enough.

The first hour is the gift. The gift is the time. The time is the marriage.

The Call to the Doctor, the Funeral Home, the Family

The first calls after death are the practical calls. The calls are important. The calls can wait until the well spouse is ready. The well spouse does not have to make the calls in the first hour. The well spouse can make the calls in the first day.

The call to the hospice nurse. The hospice nurse is the first call. The nurse can come to the house. The nurse can pronounce the death officially. The nurse can help with the practical. The nurse can be a support. The nurse is there for the well spouse.

The call to the funeral home. The funeral home is the second call. The funeral home can come to the house. The funeral home can take the body. The funeral home can help with the arrangements. The funeral home is the practical.

The call to the family. The family is the third call. The family can come. The family can support. The family can be present. The family is the support.

The call to the friends. The friends can come later. The friends can be there in the first days. The friends can be the ongoing support.

The call to the doctor. The doctor should be called, but the call is not urgent. The doctor can be called the next day. The doctor can sign the death certificate. The doctor is the medical.

The call to the spiritual care. The spiritual care provider (priest, rabbi, imam, chaplain) can be called, if the dying spouse had a spiritual practice. The spiritual care is the prayer, the ritual, the blessing.

The calls are practical. The calls can wait. The well spouse is in charge. The well spouse has time.

The First Minutes, Hours, Days

The first minutes, hours, days are a time of shock, of grief, of practical. The shock is real. The grief is real. The practical is real. The well spouse is doing all of these at the same time.

The shock is the body's way of protecting. The shock is the way the mind does not fully process the death. The shock will pass. The grief will come. The grief is the love, processing the loss.

The grief is not linear. The grief comes in waves. The waves can come at any time. The waves can be triggered by a sound, a smell, a memory, a place. The waves are the way the grief moves through the body. The waves are not a sign of weakness. The waves are the sign of love.

The practical is the way the world keeps going. The practical is the funeral home, the death certificate, the obituary, the family, the friends. The practical is the way the death becomes part of the legal, the social, the administrative. The practical can wait. The practical will be there.

The first minutes, hours, days are a time to be with the body, with the memories, with the grief. The first minutes, hours, days are a time to be with the family, the friends, the support. The first minutes, hours, days are a time to let the shock be the shock, the grief be the grief, the practical be the practical.

The well spouse does not have to do everything. The well spouse can ask for help. The well spouse can let the family take over the practical. The well spouse can be the grieving spouse, not the practical spouse. The

grieving spouse is the marriage. The marriage is the grieving.

The Grief That Begins Before Death and the Grief That Begins at Death

The grief that begins before death is called anticipatory grief. The anticipatory grief is the grief of the long middle, the grief of the final days, the grief of the vigil. The anticipatory grief is the way the well spouse begins to grieve the loss before the loss. The anticipatory grief is real. The anticipatory grief is allowed. The anticipatory grief is part of the process.

The grief that begins at death is the grief of the moment. The grief of the moment is the grief of the last breath, the grief of the first hour, the grief of the first day. The grief of the moment is sharp. The grief of the moment is raw. The grief of the moment is the way the mind processes the loss.

The two griefs are different. The two griefs are also related. The two griefs are the way the well spouse processes the loss over time. The two griefs are not in conflict. The two griefs are part of the same love, the same marriage, the same loss.

The way to be with both griefs:

- Acknowledge the anticipatory grief. The anticipatory grief is real. The anticipatory grief is not a substitute for the grief at death. The anticipatory grief is the preparation.
- Acknowledge the grief at death. The grief at death is sharp. The grief at death is allowed. The grief at death is the love, processing the loss.
- Let the two griefs be. The two griefs are part of the process. The process is the way the love moves

through the loss.

The Differences for Sudden vs Expected Death

The grief of sudden death is different from the grief of expected death.

In sudden death:

- There is no time to prepare. The death is a shock. The grief is the shock.
- There is no time to say the four things. The four things are unsaid. The unsaid is the regret.
- There is no time to be present in the final days. The well spouse was not there. The not-being-there is the guilt.
- The grief is sharp, sudden, and may be more complicated.

In expected death:

- There is time to prepare. The death is not a shock. The grief is the loss.
- There is time to say the four things. The four things are said. The said is the gift.
- There is time to be present in the final days. The well spouse was there. The being-there is the comfort.
- The grief is deep, but may be more integrated.

This book is written for the expected death. The expected death is the long middle, the final days, the vigil, the moment. The expected death has its own grief, and the expected death has its own gifts. The gifts are the four things, the vigil, the presence, the marriage at the end.

The sudden death has its own grief, and the sudden death has its own complications. The sudden death is a different book, not this one. The grief of sudden death may require more support, more therapy, more time.

The Things No One Tells You About Death

The things no one tells you about death are the things that, if you know them in advance, can make the moment less shocking, less painful, less complicated.

Some things no one tells you about death:

The body may release urine or stool after death. This is normal. This is the body's way of letting go. The funeral home will handle it. The well spouse does not have to do anything.

The body may make sounds after death. The sounds are not the dying spouse. The sounds are the body letting go. The sounds are normal. The sounds are not a sign of suffering.

The face may change after death. The face may relax, may soften, may look peaceful. The face may also look different from the person the well spouse knew. The difference is the body's way of letting go. The difference is not a sign of anything.

The body will cool after death. The body will cool to room temperature. The cooling is normal. The cooling is the body's way of letting go.

The eyes may be open or closed. The well spouse can close the eyes, if they want. The eyes do not have to be closed. The funeral home can handle it.

The body may be stiff after several hours. This is called rigor mortis. The rigor is normal. The rigor is the body's way of letting go. The funeral home will handle it.

The grief will come in waves. The waves will come at unexpected times. The waves are the way the grief moves through the body. The waves are not a sign of weakness.

The shock will last longer than you think. The shock is the body's way of protecting. The shock can last hours, days, weeks. The shock will pass. The grief will come.

The world will keep going. The world will not stop because the dying spouse has died. The world will keep going. The well spouse will have to keep going. The keeping-going is the way the well spouse honors the marriage.

You will laugh again. You will laugh again. The laughter is not a betrayal. The laughter is the way the grief moves through the body. The laughter is the way the marriage lives on.

You will love again. You may love again, in a different way. The love is not a betrayal. The love is the way the marriage continues. The love is the way the dying spouse lives on.

The Things You Can Do to Honor the Moment

The moment is sacred. The moment deserves to be honored. The honoring is the way the well spouse says "the marriage mattered." The honoring is the way the well spouse holds the memory.

Some things the well spouse can do to honor the moment:

Stay with the body. The staying is the honoring. The staying is the way the well spouse says goodbye, in the body, in the presence.

Light a candle. The candle is the light. The light is the way the well spouse honors the life.

Say a prayer. The prayer is the blessing. The blessing is the way the well spouse releases the dying spouse.

Read a poem. The poem is the words. The words are the way the well spouse says what the heart cannot say alone.

Play music. The music is the presence. The presence is the way the well spouse brings the marriage into the moment.

Tell a story. The story is the memory. The memory is the way the well spouse holds the life.

Take a handprint or a lock of hair. The handprint is the physical memory. The memory is the way the well spouse holds the body, after the body is gone.

Write a letter. The letter is the words. The words are the way the well spouse says the things that were unsaid, the things that were said, the things that will be said for the rest of the well spouse's life.

The honoring is the gift. The gift is the marriage. The marriage is the honoring.

The Role of Rituals

The rituals are the ways humans have always marked the death. The rituals are the way the well spouse, the family, the community, mark the death. The rituals are the way the death becomes part of the life, part of the story, part of the memory.

Some rituals that may be meaningful:

- A prayer or a blessing, from the spiritual tradition of the couple
- A candle, lit at the moment of death
- A glass of wine, shared in memory
- A toast, given in honor
- A song, sung or played
- A poem, read aloud
- A story, told at the bedside
- A moment of silence, held together
- A kiss, given to the forehead
- A hand, held until the body is taken
- A reading, from a sacred text
- A photograph, taken of the hands
- A letter, read aloud to the body

The rituals are not required. The rituals are not one-size-fits-all. The rituals are the way the well spouse, the family, the community, honor the moment. The rituals are the way the marriage gets to be marked, at the end.

Case Studies: All 5 Converge

The five case studies from this book all ended with deaths. The deaths were different. The deaths were the same. The deaths were the moments the marriages ended. The deaths were the moments the marriages continued, in the memory.

Linda & James. Linda died 7 months after the diagnosis. James was at the bedside. James held her hand. James said "I love you." James said goodbye. James sat with the body for an hour. James was at peace.

Margaret & David. Margaret died 6 weeks after the stroke. David was at the bedside. The children were there. The hospice nurse was there. David said "It is okay to go." Margaret died peacefully. David sat with the body. The family sat with the body. They said goodbye together.

Robert & Helen. Helen died 14 months after the diagnosis. Robert was at the bedside. The hospice aide was there. Helen died in her sleep. Robert was at peace. He had been present for the whole long middle. The presence was the marriage.

Ann & Tom. Ann died 11 months after the diagnosis. Tom was at the bedside. The hospice team was there. Ann died peacefully. Tom said "I love you, Ann. I will see you again." Tom was at peace. The marriage was the gift.

Frank & Ruth. Ruth died 18 months after the diagnosis. Frank was at the bedside. Frank said "I love you, Ruth. Thank you for the marriage." Ruth died peacefully. Frank was at peace. The marriage was the gift.

All five deaths were different. All five deaths were the same. The deaths were the moments the marriages ended. The deaths were the moments the marriages continued, in the memory, in the love, in the years after.

Scorecard: How Are You Doing in the First 24 Hours After?

| Question | Score (1-5) |

|---|---|

| I was present at the moment of death | /5 |

| I gave the dying spouse permission to go | /5 |

| I stayed with the body, in the way I needed | /5 |

| I called the hospice nurse, the funeral home, the family, when I was ready | /5 |

| I asked for help with the practical | /5 |

| I honored the moment with a ritual, in the way I needed | /5 |

| I let the shock be the shock | /5 |

| I let the grief be the grief | /5 |

| I am being supported by family, friends, the hospice team | /5 |

| I am taking care of myself (sleep, food, breaks) | /5 |

Total: /50

Interpreting your score:

- 40-50: The first 24 hours are in good shape. The marriage is being honored.
- 30-39: Some patterns to fix. The support in this chapter can help.

- 20-29: The first 24 hours are overwhelming. Get help.
- Under 20: The first 24 hours are in crisis. Get help immediately.

Key Terms

- Last Breath: The final breath the dying spouse takes before death
- Anticipatory Grief: The grief that happens before the death, as the death becomes more certain
- Rigor Mortis: The stiffening of the body after death
- Death Certificate: The legal document that records the death, signed by the doctor
- Funeral Home: The service that handles the body, the burial, the cremation, the arrangements

Reflection Questions for the Two of You

1. What is one thing you most want to do at the moment of death?
2. What is one thing you most want to say before the end?
3. What is one thing you most want the well spouse to do in the first hour?
4. What is one thing you most want the family to do in the first day?
5. What is one ritual that would be meaningful at the moment of death?
6. How will the well spouse take care of themselves in the first 24 hours?

Action for This Week: The First Day Plan

Within the next 7 days, do these 5 things:

1. Make a list of the calls to make. The hospice nurse. The funeral home. The family. The friends. The doctor. The spiritual care. The list is the practical. The practical is the gift.
2. Decide on a ritual for the moment of death. A prayer. A candle. A poem. A song. A story. The ritual is the honoring. The honoring is the marriage.
3. Identify the support team for the first 24 hours. The family. The friends. The hospice team. The spiritual care. The support team is the gift. The gift is the well spouse's care.
4. Decide who will stay with the body. The well spouse. The family. The friends. The staying is the honoring. The honoring is the marriage.
5. Decide on the first meal, the first sleep, the first break. The practical. The practical is part of the vigil. The vigil is the marriage.

The First Day Plan is the preparation. The preparation is the gift. The gift is the well spouse's care. The care is the marriage.

A Closing Word

The last breath is the end of the breathing. The last breath is the end of the body. The last breath is the end of the marriage as it has been. The last breath is the beginning of the marriage as it will be.

The moment is sacred. The moment is not clinical. The moment is the most intimate moment of the marriage. The moment is the moment the well spouse will carry

for the rest of their life.

The well spouse is present. The well spouse is the witness. The well spouse is the marriage, in the moment. The marriage is the gift. The gift is the well spouse's life, in the years after.

You are not failing. You are holding the moment.

Epilogue: The After

I lost my wife to cancer after many years of marriage. I was her primary caregiver through the long middle. I sat with her in the quiet at the end. I held her hand when the last breath came. I have lived the first hours, the first days, the first months, the first years of the after.

I have written this book because I have learned, slowly and against my will, that the end of a marriage is not a single event. It is a long season. It is a thousand small moments that you will not remember individually but that you will carry as a whole for the rest of your life.

I want to share a few things I wish someone had told me.

The thing I wish someone had told me about the first hours.

I wish someone had told me that the first hours would be sacred. I wish someone had told me that the body would still be there, and that I could stay with it, and that staying was the gift. I wish someone had told me that the practical could wait, and that the shock would carry me, and that the shock was the way the love protected me.

The thing I wish someone had told me about the first days.

I wish someone had told me that the first days would be both easier and harder than I expected. Easier because the support would come, and the family would come, and the food would come, and the practical would be there. Harder because the empty chair would be there, and the empty bed would be

there, and the empty house would be there. I wish someone had told me that the easier and the harder would coexist, and that both were real.

The thing I wish someone had told me about the first weeks.

I wish someone had told me that the first weeks would be a blur. I wish someone had told me that the administrative work would be there, and that I could do it, and that I could also wait. I wish someone had told me that the support would begin to fade, and that the fading would be the moment when the grief got louder. I wish someone had told me that the louder grief was the love, processing the loss.

The thing I wish someone had told me about the first months.

I wish someone had told me that the first months would be the time of the new normal. I wish someone had told me that the new normal would be hard, and that the new normal would be the way I began to build the new life. I wish someone had told me that the grief would not go away, and that the grief was not supposed to go away, and that the grief was the way the love continued.

The thing I wish someone had told me about the first year.

I wish someone had told me that the first year would be the time of the firsts. I wish someone had told me that the firsts would be hard, and that the firsts would be the way I honored the marriage. I wish someone had told me that the first year would also be the time when I began to know what the new life was. I wish someone had told me that the new life was not a betrayal of the marriage, and that the new life was the way the marriage continued.

The thing I wish someone had told me about the years after.

I wish someone had told me that the years after would be both harder and easier than I expected. Harder because the grief would continue, and the waves would come, and the surprises would come. Easier because the grief would become part of the new life, and the new life would be the way the marriage continued, and the marriage would be the way the love continued, in the body, in the memory, in the years after.

I have lived the first hours. I have lived the first days. I have lived the first weeks. I have lived the first months. I have lived the first year. I am living the years after. The years after are the way the marriage continues. The years after are the way the love continues. The years after are the way I am.

I am still married, in a way. I am married to the memory of my wife. I am married to the love we had. I am married to the years we had. I am married to the life we built. I am married to the lessons I learned. I am married to the values we shared. I am married to the future I am building, in her name.

I will love again, in a different way. The love is not a betrayal. The love is the way the marriage continues. The love is the way she continues, in the body, in the heart, in the years after.

I will be okay, in a different way. The being-okay is not the end of the grief. The being-okay is the way the grief becomes part of the new life. The being-okay is the way the new life is built, with the grief, with the love, with the marriage.

I will keep going, in a different way. The keeping-going is not a forgetfulness. The

keeping-going is the way the marriage continues. The keeping-going is the way the love is honored. The keeping-going is the way I am.

If you are reading this book, you are in the long middle, or the final days, or the first hours, or the first days, or the first weeks, or the first months, or the first year, or the years after. Wherever you are, I want to tell you: you are not alone. The long middle has been lived by many. The final days have been lived by many. The first hours have been lived by many. The first days, the first weeks, the first months, the first year, the years after — all of it has been lived by many. You are part of the many. The many is the support. The support is the gift. The gift is the marriage, in the time you have.

The marriage is the work. The work is the marriage. The work is not the illness. The work is not the death. The work is not the after. The work is the love. The love is the work. The love is the marriage. The marriage is the love.

You are not failing. You are loving. The loving is the marriage. The marriage is the loving. The loving is the work. The work is the loving.

The loving is the gift. The gift is the marriage. The marriage is the gift.

I give this book to you, with my love, in the memory of my wife, in the hope that the marriage you are in, or the marriage you have lost, or the marriage you are grieving, will be honored, in the time you have, in the way you can, with the love that is yours.

The marriage is yours. The love is yours. The work is yours. The gift is yours.

You are not alone.

About the Author

S. Shharma is the author of The Care Collection — books for couples in the second half of life, written by a man who lived the silence and survived it.

He lost his wife of many years to cancer. He was her primary caregiver through every stage of her illness. He held her hand in the hospital. He sat in the waiting room at 3 a.m. He learned the silence of a marriage the way you learn a language you never wanted to speak.

He is also a businessman, an M.Sc., an MBA, and an APSCM from the Indian Institute of Management (IIM) Calcutta, with thirty years in corporate management. But these are not credentials you need to trust him. The credentials that matter are the ones in the books.

The Care Collection is not about how to fix a marriage. The Care Collection is about how to live one — even when the marriage is quiet, even when one of you is sick, even when one of you is gone.

He lives in India. He writes from the perspective of someone who has been on every side of a marriage — the partner, the caregiver, and the one left behind.

Books in The Care Collection

- Still Married, Just Silent — when the marriage is fine on paper and empty in person
- The Quiet Care — when one of you becomes the caregiver and the marriage disappears
- The Second Chapter — when the children are gone and the two of you meet again

THE CARE COLLECTION

Complete 14-Book Library for the Second Half of Life • by Sanjay Shharma

The Care Collection addresses the quiet, unspoken, and deeply human crossroads of midlife, marriage, caregiving, and grief. Written from thirty years of lived devotion and corporate leadership, each volume offers clear frameworks, practical scripts, and gentle reassurance.

- 1. Still Married, Just Silent — 14 conversations to reopen a quiet marriage**
- 2. The Quiet Care — How to give and receive care in a marriage that lasts**
- 3. The Second Chapter — Life after the caregiving years**
- 4. The Second Honeymoon — Reigniting intimacy, trust, and joy after the kids leave**
- 5. The Survivor — When one spouse is left behind**
- 6. The Long Hello — How couples 50–70 reconnect after the last kid leaves**
- 7. When They Forget Your Name — A caregiver's path through dementia**
- 8. Gray Divorce — Coming back together (or apart) at 55+**
- 9. The Last Sandwich — Caring for parents, spouse, and yourself at the same time**
- 10. The Long Goodbye — When your spouse is dying and the marriage is still yours**
- 11. Remarried at 60 — Building a new marriage when both of you come with a history**
- 12. After the Last Goodbye — Rebuilding your life after losing a spouse**
- 13. The Friends Who Fade — Why friendships disappear in midlife**
- 14. Hum Ladte Nahi (Hinglish) — Shaadi ko wapas kholne ka 30-din ka practical plan**

All 14 volumes are available worldwide in paperback, hardcover, and digital formats.
Published by Adorise Digital LLC • books.adorisedigital.com

WHERE TO GET THE BOOKS

Official Editions, Direct Storefronts & Global Retailers

OFFICIAL DIRECT STOREFRONTS (Instant eBook & Audio Access)

Get complete DRM-free EPUB & PDF editions, reader guides, and collection bundles directly from the author and publisher:

- **Official Catalog:** <https://books.adorisedigital.com>
- **Direct Digital Store:** <https://adorisedigital.gumroad.com>

AMAZON KDP, GOOGLE PLAY & MAJOR RETAILERS

To purchase official Paperback, Hardcover, or read on your Kindle, tablet, or phone across trusted international bookstores:

SEARCH ON GOOGLE:
“[Book Title] Sanjay Shharma”
or **“[Book Title] Adorise Digital”**

Available globally on:

- Amazon KDP (Paperback, Hardcover & Kindle Edition)
- Google Play Books (eBook & Android Reading)
- Apple Books, Barnes & Noble, and leading book distributors.

CONNECT WITH SANJAY SHHARMA

Email & Reader Support: support@adorisedigital.com

LinkedIn: [linkedin.com/in/sanjayshharma](https://www.linkedin.com/in/sanjayshharma)

Instagram: [instagram.com/sanjayshharma](https://www.instagram.com/sanjayshharma)

Official Publisher Portal: adorisedigital.com

Adorise Digital • A Division of Trendy DigiStore LLC • Sheridan, Wyoming, USA

Crisis Support Resources: US: 988 | UK: 111 | India: 9152987821

© 2026 Adorise Digital / Sanjay Shharma. All Rights Reserved. First Edition.